M.PHIL. CLINICAL PSYCHOLOGY · STUDY NOTES

# Psychotherapy — Unit I: Introduction to Psychotherapy

Definitions · Objectives · Training of therapists · Ethical & legal issues · Rights & responsibilities · Consent & assent · Planning & recording · Structure & goals · Pre/post-assessment · Evidence-based practice

**Syllabus (Unit I):** Introduction to Psychotherapy: Definitions, objectives, issues related to training professional therapists; ethical and legal issues involved in therapy work; rights and responsibilities in psychotherapy; issues related to consent (assent in case of minors); planning and recording of therapy sessions; structuring and setting goals; pre- and post-assessment; practice of evidence-based therapies.

Built from the 34+ psychotherapy books and the uploaded *notes_of_psychotherapies[1].pdf* in the repository, with primary documents of the RCI, IACP (as far as accessible), APA, BPS, WHO and Indian statutes. Page numbers are PDF page indices. See “How to read these notes” and the Source Audit.

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# How to read these notes

These notes are written so that you can produce a full examination answer (roughly **100–150 lines, i.e. a 5-mark long answer in your programme**) on each of the ten syllabus topics. Every topic therefore contains: a definition set, theory, classification tables, procedures, worked examples, ethical–legal points, a flowchart or diagram, a mnemonic, a ready answer skeleton with line budget, and practice questions.

## 1. Evidence labels used throughout

Not every statement in a psychotherapy text has the same authority. These coloured labels tell you what kind of statement you are reading, so that in an examination you never present an opinion as law.

| Label | Meaning | Example in these notes |
|---|---|---|
| BINDING LAW | A statute or court order that binds by force of law. Breach can mean penalty, loss of registration or liability. | RCI Act 1992 s.13; MHCA 2017 s.23; POCSO 2012 s.19 |
| PROFESSIONAL CODE | An enforceable professional or ethics code of a regulator or association. | APA Ethics Code Standard 4.02; RCI conduct regulations |
| GUIDELINE | Aspirational or best-practice guidance: recommended, not mandatory. | APA Record Keeping Guidelines (2007); telepsychology guidelines |
| THEORETICAL VIEW | A theoretical or author's opinion; schools differ. | Wolberg's three aims; Kleinke's six goals |
| ILLUSTRATIVE EXAMPLE | An invented teaching example. Names are fictitious; details are illustrative, not case records. | All vignettes, sample notes, sample scores |
| SECONDARY SOURCE | Information I could only obtain through a secondary summary (e.g., a legal-literacy website or a search-result excerpt), not the primary document. | RCI 1998 conduct regulation numbers (via Nyaaya); IACP code |

## 2. Citation key

Citations look like [Corey 2024, PDF p.75]. **PDF p.** is the page index inside the PDF file in the repository, which usually differs from the printed page number (e.g., Corey's “Right of Informed Consent” is PDF p.75 = printed p.48). Where I verified the printed page I give it. [Notes p.N] refers to your uploaded file

*notes_of_psychotherapies[1].pdf* (112 pages). Quotation is kept short; everything else is paraphrased and explained in my own words.

| Short name | Full source (as found in the repository or web) |
|---|---|
| Notes | *notes_of_psychotherapies[1].pdf* — compilation of book chapters (Unit 1 Introduction pp.1–12; Unit 3 Interviewing from p.11; Counselling process pp.71–78 and later units). Some pages carry source markers such as "P.478" showing they are excerpts from larger books. |
| Wolberg 2013 | Wolberg, L. R. (2013). *The Technique of Psychotherapy* (4th ed.). 2,639 pages in the PDF. |
| Corey 2024 | Corey, G. (2024). *Theory and Practice of Counseling and Psychotherapy* (11th ed.). Cengage. |
| Corsini 2011 | Corsini, R. J., & Wedding, D. (2011). *Current Psychotherapies* (9th ed.). Includes Ch.16 "Contemporary Challenges and Controversies" (Pope & Wedding). |
| Sharf 2010 | Sharf, R. S. (2010). *Theories of Psychotherapy and Counseling: Concepts and Cases* (5th ed.). Cengage. (Three identical copies in the repository.) |
| Murdock 2017 | Murdock, N. L. (2017). *Theories of Counseling and Psychotherapy: A Case Approach* (4th ed.). Pearson. |
| Jongsma 2014 | Jongsma, A. E., Peterson, L. M., & Bruce, T. J. (2014). *The Complete Adult Psychotherapy Treatment Planner* (5th ed.). Wiley. |
| Beck 2011 | Beck, J. S. (2011). *Cognitive Behavior Therapy: Basics and Beyond* (2nd ed.). Guilford. |
| Hofmann 2007 | Hofmann, S. G., & Weinberger, J. (Eds.) (2007). *The Art and Science of Psychotherapy*. Routledge. |
| Lister-Ford 2007 | Lister-Ford, C. (2007). *A Short Introduction to Psychotherapy*. Sage. |
| Gabbard 2004 | Gabbard, G. O. (2004). *Long-Term Psychodynamic Psychotherapy: A Basic Text*. American Psychiatric Publishing. |
| Polnay 2023 | Polnay, A. et al. (2023). *Cambridge Guide to Psychodynamic Psychotherapy*. Cambridge University Press. |
| APA Ethics Code | American Psychological Association (2017). *Ethical Principles of Psychologists and Code of Conduct* (effective 1 Jan 2017). Read in full from the primary PDF. |
| RCI Act | *The Rehabilitation Council of India Act, 1992* (No. 34 of 1992) with the 2000 Amendment. Read in full from the Gazette text. |
| MHCA 2017 | *The Mental Healthcare Act, 2017*, Gazette of India Extraordinary, 7 April 2017. Read from the official Gazette text. |
| POCSO | *Protection of Children from Sexual Offences Act, 2012*. ss.2(d), 19 and 21 read from an Act text. |

| | |
|---|---|
| ICMR 2017 | Indian Council of Medical Research (2017). *National Ethical Guidelines for Biomedical and Health Research Involving Human Participants*. Read §6.5 (children) from the primary PDF. |

**– Why PDF page numbers and not printed page numbers?**

Printed page numbers change between editions and are sometimes missing from scanned or reflowed e-books, whereas the PDF page index is exactly reproducible by you: open the file and go to that page. Where a printed page was visible I note it, e.g., Corey 2024 PDF p.75 = printed p.48.

## 3. Honest limits of these notes

- **IACP:** the Indian Association of Clinical Psychologists' website could not be reached from the environment in which these notes were prepared, so the text of the IACP code of conduct was **not read**. I describe only what secondary summaries report and I mark it SECONDARY SOURCE . Obtain the code from IACP or your department and compare it with the APA headings given here.
- **RCI 1998 conduct regulations:** the primary regulation text could not be downloaded. Regulation numbers are given as reported by Nyaaya (2022) and marked SECONDARY SOURCE ; the parent power to make them (RCI Act s.21) is verified from the Act.
- **DPDP Act 2023 and Rules 2025:** the primary text could not be downloaded; points are from published legal commentary and marked SECONDARY SOURCE . Commencement is phased, so check current status.
- **The RCI M.Phil. Clinical Psychology curriculum** was not accessible; I use RCI's 2011 Professional Diploma in Clinical Psychology document to illustrate RCI's thinking on supervised work and records, and flag it as such.
- **About 18 repository files** (e.g., Rorschach, TAT, MCMI-III, MISIC, VSMS, NIMHANS SLD, PGI, CBT by Hawton) are scanned images with no text layer and could not be read. They are not cited.
- Laws change. Statements about Indian law are for study. In practice, read the current text and seek advice.

# Professional and regulatory bodies at a glance

Your examiners may ask “What does the RCI / IACP / APA say about this?”. This map shows who each body is, whether it can compel you, what I could verify, and where it appears in the notes.

**Who regulates or guides psychotherapy practice?**

| Body | Nature and force | Key content relevant to Unit I | Verification status |
|---|---|---|---|

| | | | |
|---|---|---|---|
| **RCI**<br>Rehabilitation Council of India | BINDING LAW Statutory body created by the RCI Act 1992. Regulates training of rehabilitation professionals (clinical psychologists are named as such), keeps the Central Rehabilitation Register, and may prescribe standards of professional conduct, etiquette and a code of ethics (s.21). | Only enrolled professionals may practise or hold the post (s.13). Misconduct can lead to removal from the Register (s.21(3)). Prescribes minimum standards of education (s.18) and approves courses; its diploma document sets supervised-work and record requirements. | Act read in full (primary). Government notice on RCI functions read. 1998 conduct regulations only via Nyaaya SECONDARY SOURCE . M.Phil. curriculum not accessed. |
| **IACP**<br>Indian Association of Clinical Psychologists | PROFESSIONAL CODE A professional association (its notices say it is registered under the Societies Registration Act, 1860). Binds its members through its code of conduct, not by statute. | Reported to have adopted a code of conduct for professional members stressing competence, responsibility, client welfare and integrity; Nyaaya states that psychologists follow the IACP code. IACP also advocates on training and professional identity. | SECONDARY SOURCE Website unreachable; code text **not read**. Do not quote clause numbers. |

| | | | |
|---|---|---|---|
| **APA**<br>American Psychological Association | PROFESSIONAL CODE<br>Enforceable for APA members; widely used worldwide as a reference. Its guidelines are GUIDELINE aspirational. | Ethics Code 2017 (5 General Principles, enforceable Standards: e.g., 2.01 competence, 3.04 avoiding harm, 3.05 multiple relationships, 3.10 informed consent incl. assent, 4.01–4.05 confidentiality, 6.01–6.02 records, 10.01 consent to therapy, 10.10 termination). Record Keeping Guidelines (2007). Telepsychology Guidelines (2013; revised later). EBPP policy (2006). Resolution on Recognition of Psychotherapy Effectiveness (2012). | Ethics Code read in full (primary). Record Keeping and Telepsychology guidelines read (primary). 2006 EBPP wording from three secondary quotations (consistent). 2012 Resolution conclusion read. |
| **BPS**<br>British Psychological Society | PROFESSIONAL CODE UK professional body; its Code of Ethics and Conduct (2021) sets four ethical principles. | Respect, Competence, Responsibility, Integrity (as stated on the BPS site). | Principles confirmed from BPS web pages; full code not read. |
| **WHO**<br>World Health Organization | GUIDELINE Technical guidance, not binding on practitioners. | mhGAP Intervention Guide for non-specialist settings; Problem Management Plus (PM+), a brief manualised psychological intervention for non-specialist delivery. Relevant to task-sharing, training and evidence-based practice. | Existence and purpose confirmed from WHO and PubMed Central pages. |

| | | | |
|---|---|---|---|
| **Mental Healthcare Act 2017** | BINDING LAW Parliament's mental-health statute. Defines "clinical psychologist" by RCI-recognised qualification; State Authority registers clinical psychologists as mental health professionals (s.55(1)(d)). | Capacity (s.4), informed consent definition (s.2), minors and nominated representative (s.15), information rights (s. 22), confidentiality and exceptions (s.23), digital information (s. 24), access to records (s.25). | Read from the Gazette text (primary). |
| **POCSO Act 2012** | BINDING LAW Child-protection statute. | "Child" = person below 18 (s.2(d)); any person with knowledge or apprehension of a sexual offence must report (s.19); failure to report is an offence (s.21). | Sections read from an Act text. |
| **DPDP Act 2023 / Rules 2025** | BINDING LAW Data-protection statute. | Verifiable parental consent before processing a child's personal data (s.9), with exemptions for some healthcare uses under the Rules. | SECONDARY SOURCE Commentary only; check current text and commencement. |
| **ICMR** Indian Council of Medical Research | GUIDELINE National ethical guidelines for *research* with human participants (2017). | §6.5: parental/LAR consent plus assent for children aged 7–18; a child's objection must be respected. | Read from the primary PDF (research context only). |

## Which body speaks to which syllabus topic?

| Topic | RCI | IACP | APA | Indian statutes | Others |
|---|---|---|---|---|---|
| 1–2 Definitions, objectives | Diploma doc lists psychoeducation, counselling, psychotherapy and behaviour therapy as services; Paper II Unit I names definitions, objectives, planning/ recording | (not read) | 2012 Resolution: psychotherapy is effective, cost-effective, an established evidence-based practice | MHCA defines "mental healthcare" (s.2(1)(o)); never defines psychotherapy | Wolberg, Norcross definitions (Notes) |

| | | | | | |
|---|---|---|---|---|---|
| 3 Training | Recognition of courses, minimum standards (s. 18), register (ss.19, 23), supervised-work rules | (not read; advocacy on training) | Std 2.01, 2.03, 2.06; 7.01–7.07; 10.01(c) trainee status | MHCA s.2(1)(g), s.55 | Gabbard on supervision; WHO PM+ |
| 4 Ethics, law | Act s.21; 1998 conduct regulations SECONDARY SOURCE | Code of conduct (not read) | Principles A–E; Std 1.02, 3.04–3.08, 4.01–4.07 | MHCA ss.23–25; POCSO ss.19, 21; DPDP | BPS 4 principles; Corey; Corsini Ch. 16 |
| 5 Rights, responsibilities | Misconduct, fees, advertising, records SECONDARY SOURCE | (not read) | Std 3.10, 4.02, 6.03–6.04, 10.01, 10.10 | MHCA ss.18–25 | Murdock; Corey |
| 6 Consent, assent | (not found) | (not read) | Std 3.10(b) four duties for persons unable to consent | MHCA ss.2, 4, 15, 87; POCSO; DPDP s.9 | ICMR §6.5 (research) |
| 7 Planning, recording | Diploma: eight elements of a psychotherapy record; logbook | (not read) | Std 6.01–6.02, 4.03; Record Keeping Guidelines | RCI register of patients SECONDARY SOURCE ; MHCA s.25 | Beck notes; Jongsma |
| 8 Structure, goals | Practical II: set short- and long-term treatment goals | (not read) | Std 10.01 | — | Beck; Jongsma |
| 9 Pre/post assessment | Practical I: assessment competency | (not read) | Std 9.01–9.03 (assessment) | — | Lister-Ford (CORE) |
| 10 Evidence-based practice | (not found) | (not read) | 2006 EBPP; 2012 Resolution; Std 2.04 | — | WHO mhGAP/ PM+; Lister-Ford; Hofmann |

# 1 Definitions of Psychotherapy

Topic 1 asks you to say precisely what psychotherapy *is*, who may do it, how it differs from neighbouring activities, and why definitions differ between schools. Examiners reward answers that (a) quote two or three recognised definitions with authors, (b) extract their common elements, and (c) separate psychotherapy from counselling, assessment and psychiatric treatment.

**AT A GLANCE**

- **Core idea:** psychotherapy is the planned, theory-based use of psychological methods, within a deliberate professional relationship, by a trained person, to relieve distress, change behaviour/thinking/emotion and promote growth.
- **Must-know definitions:** Wolberg (treatment by psychological means; three aims); Norcross (1990; informed, intentional application of methods from established psychological principles); Korchin, Bruch, Watkins (short classic definitions); Corey (collaborative process in which both persons change).
- **Must-know contrasts:** counselling, psychological assessment, psychiatric treatment, education, casework, friendship.
- **Mnemonic: TRAPS** (Trained person · Relationship · Approach · Purpose · Shared direction).

## 1. Introduction: why a definition matters

Wolberg opens his chapter "What Is Psychotherapy?" by observing that few words in the mental-health lexicon are as ambiguous: the term is loosely used to mean helping, treating, advising, guiding, educating and even influencing, and this "diffuseness" has fostered many divergent theories and techniques [Wolberg 2013, PDF p.29]. He still insists that a brief, precise description is needed *to mark the boundaries of operation* and to permit theory-building and empirical study.

Definitions matter in at least five practical ways:

1. **Professional boundaries** (what a clinical psychologist may and may not do, and how this differs from advice-giving or friendship).
2. **Regulation and training** (a regulator such as the RCI needs to say who is competent to deliver a defined service).
3. **Research** (outcome studies need an agreed independent variable; otherwise "therapy" means anything).
4. **Consent and rights** (clients can only consent to something that can be described to them; see Topics 5 and 6).
5. **Payment and accountability** (insurers, employers and courts ask "what was the treatment?").

## 2. Origin of the word and the "talking cure"

**Etymology** word origin (where the word comes from): *psyche* (Greek, soul or being) + *therapeutikos* (caring for another), so psychotherapy literally means "caring for another person's soul or being" (Kleinke, 1994, cited in [Notes p.1]). Anna O., an early psychoanalytic patient of Breuer, called her

treatment **"the talking cure"** [Notes p.1]. The Notes comment that this is elegant but vague: talking, expressing and sharing pain may be healing in itself, yet that does not satisfy contemporary researchers who need operational definitions.

Historically, psychoanalytic psychotherapy was the exclusive form in the early twentieth century; behavioural therapy emerged as an alternative in the 1950s; humanistic therapy moved to the forefront in the 1960s; and integrative approaches have gathered strength in recent decades [Notes p. 3–4]. Today there are said to be more than 400 named approaches (Karasu, 1986, cited in [Notes p.72]) and over 450 schools (Simon, 2001, cited in [Notes p.8]). This proliferation is exactly why a common definition is difficult.

## 3. Authoritative definitions

Learn the *author, year and key phrase* of each definition. The wording below is paraphrased except for the short quotations.

**Table 1.1 Major definitions of psychotherapy**

| Author / source | Definition (short quotation or paraphrase) | What it emphasises | Evaluation |
|---|---|---|---|
| **Wolberg** (working definition; 1995 as cited in Notes; 4th ed. 2013) | "Psychotherapy is the treatment, by psychological means, of problems of an emotional nature in which a trained person deliberately establishes a professional relationship with the patient" with the object of (1) removing, modifying or retarding existing symptoms, (2) mediating disturbed patterns of behaviour, and (3) promoting positive personality growth and development. [Wolberg 2013, PDF p.29] | Medical-psychological: *treatment*, *trained person*, *deliberate relationship*, three aims. | Most useful for exams because it lists aims. Uses "patient" and a treatment model; humanistic writers may prefer "client" and growth language. |
| **Norcross** (1990) | The "informed and intentional application of clinical methods and interpersonal stances derived from established psychological principles" to help people modify behaviours, cognitions, emotions and other personal characteristics "in directions that the participants deem desirable". [Notes p.1–2] | Intentional, evidence-informed methods; *participants decide* what is desirable. | Modern, school-neutral and client-centred. Does not mention symptoms or the profession of the helper. |
| **Contemporary definition** (as given in Notes) | The treatment of mental and emotional disorders through psychological techniques, with relief of symptoms, behaviour change that improves social functioning, and personality growth as goals. [Notes p.1] | Disorder-focused; three goals similar to Wolberg. | Convenient summary; "disorders" may be too narrow for developmental or growth work. |

| | | | |
|---|---|---|---|
| **Korchin** (1976) | "A conversation with a therapeutic purpose." [Notes p.1] | Talk plus purpose. | Memorable but too broad: a wise friend might also hold such a conversation. |
| **Schofield** (1964) | "The purchase of friendship." [Notes p.1] | Ironic critique: paid relationship. | Highlights the *asymmetry* and fee; misses method and training. Use only as a critical contrast. |
| **Bruch** (1981) | A situation in which two people interact and try to understand one another, with the specific goal of accomplishing something beneficial for the complaining person. [Notes p.1] | Mutual understanding; benefit for the complainant. | Shows interaction but not professional training or method. |
| **Watkins** (1983) | One person with an emotional disorder gets help from another with less emotional disorder. [Notes p.1] | Relative degree of disturbance. | Informal and arguably stigmatising; ignores training and method. |
| **Corey** (2024) | A process of engagement between two people, both of whom are bound to change; at best a collaborative process of co-constructing solutions to life's tasks. [Corey 2024, PDF p.31] | Collaboration and mutual change. | Good for humanistic/ integrative answers; less precise about professional boundaries. |
| **Notes' "professional relationship" definition** | A professional relationship bounded by limits on time, frequency, content and intimacy, in which a person talks over problems with a specialist in human behaviour; confidentiality has ethical and legal limits and dual relationships must be avoided. [Notes p. 2] | The *frame* of therapy. | Useful for linking Topic 1 to Topics 4–5. |

**HOW TO USE THESE IN THE EXAM**

For a 5-mark answer quote **Wolberg** and **Norcross** in full (they are the most scholarly), mention **Korchin/Schofield/Watkins** as short historical definitions, and then analyse their common elements. Writing "Norcross (1990), cited in the course notes" is acceptable and honest when you have not read the original article.

## 4. Common elements and differences across definitions

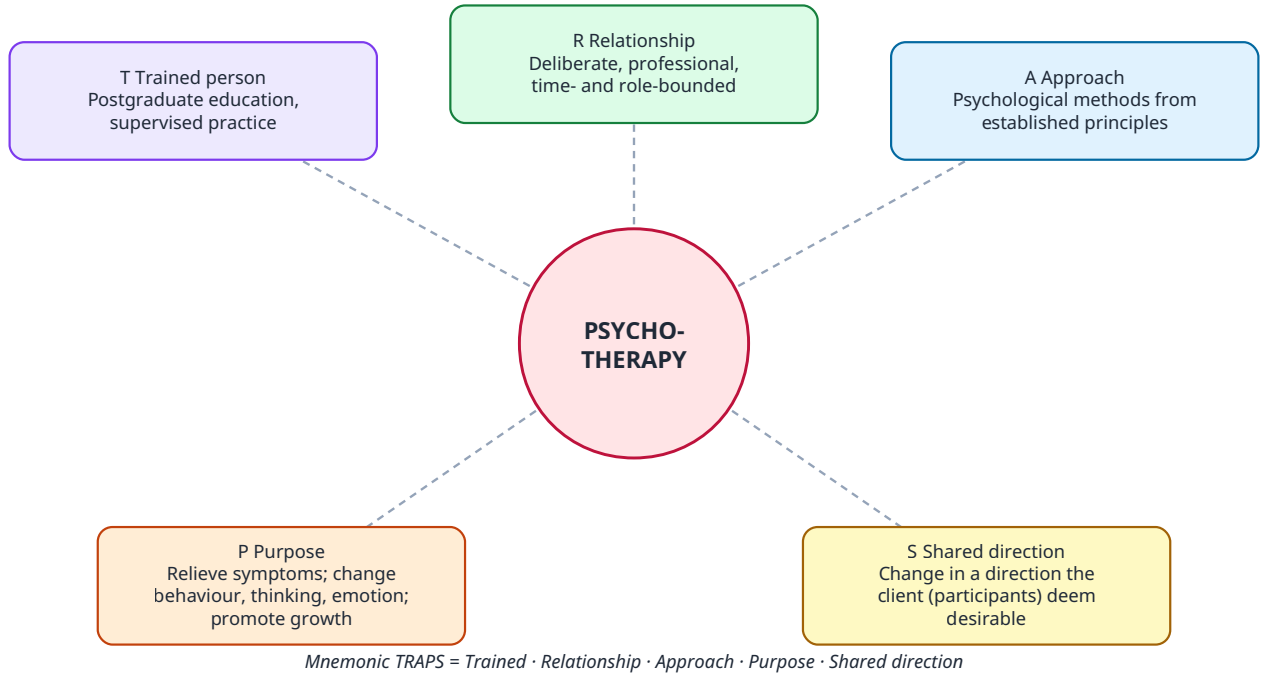



**Figure 1.1** Elements common to modern definitions (Wolberg; Norcross 1990 as cited in Notes p.1–2).
Source/basis: Wolberg 2013 PDF p.29; Notes-Psychotherapies PDF p.1–2

**Table 1.2 Elements that recur in definitions**

| Element | What the definitions say | Why it matters clinically |
|---|---|---|
| **T** Trained person | Wolberg: a "trained person"; dealing with emotional problems "requires a high degree of skill" acquired through extensive postgraduate training. [Wolberg 2013, PDF p.30] | Untrained helping can harm (Wolberg notes the relationship can become "disastrous to both participants" when emotional turmoil is handled in depth without skill). |
| **R** Relationship | "Deliberately established" professional relationship; the core of the therapeutic process, planned and nurtured, collaborative, unlike ordinary social relationships. [Wolberg 2013, PDF p.30] | The alliance is among the strongest predictors of outcome (Topic 10). |
| **A** Approach | Psychological means (communication, relationship manoeuvres, conditioning, cognitive and other methods) derived from established principles (Norcross). Excludes drugs, ECT and surgery. | Distinguishes psychotherapy from somatic treatment and from folk advice. |
| **P** Purpose | Symptom relief; modification of disturbed behaviour patterns; personality growth; better functioning. | Determines goals and outcome measures (Topics 2, 8, 9). |

| | | |
|---|---|---|
| **S** Shared direction | Change “in directions that the participants deem desirable” (Norcross). | Informed consent and collaborative goal-setting (Topics 5, 6, 8). |

### Points of difference between definitions

- **Medical versus psychological model.** Wolberg speaks of “treatment” of “problems of an emotional nature”; Norcross speaks of “assisting people” and leaves the goal to the participants. Wolberg even devotes a section to whether the medical model is outmoded [Wolberg 2013, PDF p.32].
- **Who defines success.** Wolberg's three aims are therapist-defined; Norcross's “directions participants deem desirable” is participant-defined.
- **Symptoms versus growth.** Behavioural/CBT definitions stress specific, measurable change; humanistic definitions stress actualisation and growth [Notes p.4].
- **Role of insight.** Psychodynamic definitions treat insight as central; behavioural ones treat it as optional.
- **Informal versus professional.** Korchin, Schofield and Watkins are informal; Wolberg and Norcross are professional.

## 5. The healing frame: four features and four theory families

### 5.1 Four features of psychotherapy common to all schools

The Notes summarise psychotherapy as including four major factors [Notes p.2]:

1. A **relationship** in which the client has confidence that the therapist is competent and cares about his or her welfare.
2. **Expectancy or hope** shared by therapist and client that benefit will result.
3. An underlying **conceptual framework** giving a rationale and a set of procedures that need the active participation of both.
4. A **practice setting** socially defined and accepted as a place of healing.

Additional influences are client factors (nature of the problem, ability to engage, motivation, outside resources such as family support) and therapist factors (competence, personality) [Notes p.2–3]. Gabbard adds that the therapeutic alliance is applicable to all forms of psychotherapy and that, in one large depression study, the alliance accounted for more variance in outcome (21%) than the interventions themselves [Gabbard 2004, PDF p.58].

**MNEMONIC — THE FOUR COMMON FEATURES**

## H · E · R · S

| | |
|---|---|
| **H** | Hope (expectancy of benefit) |
| **E** | Environment (a setting socially accepted as healing) |
| **R** | Relationship (confidence in a caring, competent therapist) |
| **S** | Scheme (a conceptual rationale with procedures requiring active participation) |

**Use:** "Why can very different schools all help?" Answer: each supplies **H**ope in a healing **E**nvironment, through a **R**elationship, using a **S**cheme; the Notes describe these as shared factors.

## 5.2 Four major theory families

**Table 1.3 Theory families and how each would define the task** [Notes p.3–4]

| Family | Typical definition of the task | Key concepts |
|---|---|---|
| Psychoanalytic / psychodynamic | Make unconscious conflicts and the effects of early relationships conscious; use the relationship to provide a *corrective emotional experience*. | Transference, countertransference, insight |
| Behavioural / CBT | Present-focused, problem-solving; change dysfunctional thoughts, emotions and behaviour through concrete, measurable goals. | Learning principles, cognitions, skills practice |
| Humanistic / experiential | Facilitate actualisation and growth through the client's own perception and awareness; phenomenological stance. | Self-awareness, growth, authenticity |
| Family systems | Change the relational system that maintains the problem. | Roles, patterns, boundaries |
| Integrative / eclectic | Draw on several traditions; supported by research suggesting that major approaches have broadly comparable effects for many problems, and that shared factors may explain this. | Common factors, technical eclecticism |

**CAUTION: "ALL THERAPIES ARE EQUALLY EFFECTIVE"**

The Notes state that research indicates different major perspectives are equally effective [Notes p.4]. This is a *summary of a contested view*, not a universal law. Evidence-based practice (Topic 10) holds that some treatments have better evidence for specific problems (e.g., exposure-based treatment for OCD). In an exam, write: "broadly comparable overall, but differences exist for particular disorders".

## 6. Psychotherapy versus related activities

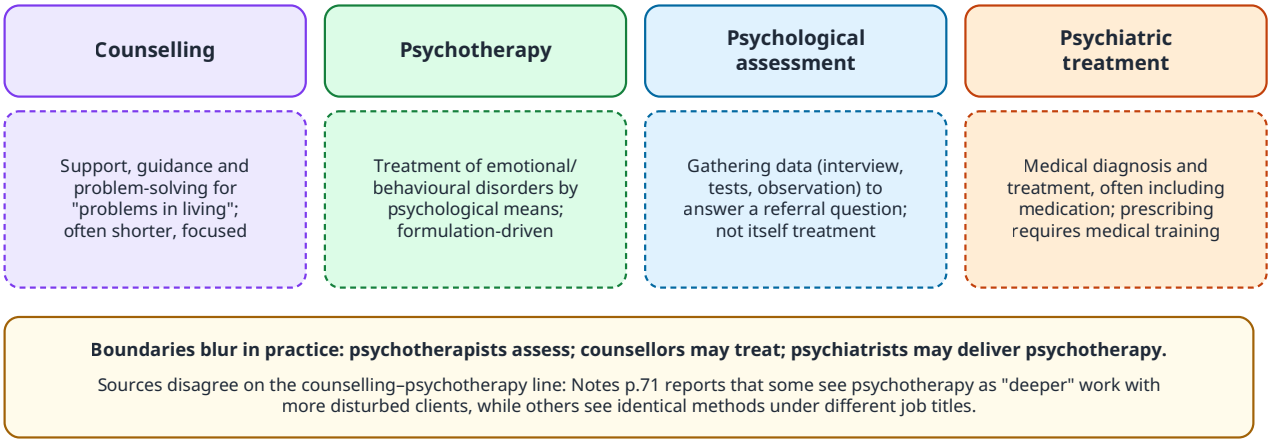



**Figure 1.2** Related but distinct activities. The distinctions are of emphasis, not rigid walls.
Source/basis: Wolberg 2013 PDF p.29–30, p.125; Notes-Psychotherapies PDF p.2, 71; Sharf 2010 PDF p.158–159

**Table 1.4 Psychotherapy, counselling, psychological assessment and psychiatric treatment**

| Feature | Psychotherapy | Counselling | Psychological assessment | Psychiatric treatment |
|---|---|---|---|---|
| **Main aim** | Treat emotional/ behavioural disorder; change patterns; promote growth | Help people understand themselves and solve problems of adjustment or decision-making [Wolberg 2013, PDF p. 133] | Collect and interpret data to answer a referral question (diagnosis, ability, personality, risk) | Diagnose and treat mental illness, often including medication |
| **Typical clients** | Persons with disorders, entrenched patterns, or seeking deeper change | Persons with problems in living, developmental, educational or vocational concerns | Anyone referred for evaluation | Persons with mental illness needing medical management |
| **Methods** | Systematic, theory-based psychological methods (CBT, psychodynamic, humanistic, systemic) | Listening, guidance, problem-solving, information, decision support; directive or non-directive [Wolberg 2013, PDF p.134] | Interview, tests, rating scales, observation, record review | Medical assessment, pharmacotherapy, ECT, and psychotherapy where trained |

| | | | | |
|---|---|---|---|---|
| **Depth/ duration** | Often longer and deeper (but brief therapies exist) | Often shorter, focused; much counselling is delivered by volunteers [Notes p. 71] | Time-limited, ends with a report | Ongoing medical follow-up |
| **Practitioner** | Trained clinician (clinical psychologist, psychiatrist, trained social worker, etc.) | Counsellors of various training levels | Clinical or other qualified psychologist | Psychiatrist (medical practitioner with postgraduate qualification) [MHCA 2017 s.2(1)(y)] |
| **Output** | Change in symptoms/ functioning; treatment record | Clarity, decision, coping | Written report and recommendations | Diagnosis, prescription, treatment plan |

## 6.1 Is the counselling–psychotherapy line real?

Authorities disagree. Some writers claim that psychotherapy is a deeper, more involved process for more disturbed clients; others hold that counsellors and psychotherapists do the same work under different titles dictated by employers [Notes p.71]. Wolberg concludes in his legal discussion (US context) that "a sharp distinction between counseling and psychotherapy cannot be drawn" and that no law can prevent people doing psychotherapy under the name of guidance or counselling [Wolberg 2013, PDF p.676]. From the **Adlerian** viewpoint, Dreikurs argued that psychotherapy is needed if the *lifestyle* must change, whereas counselling suffices when change can occur *within* the lifestyle [Sharf 2010, PDF p.159].

The Notes also give a simplified contrast: psychotherapy implies treating or curing problems, whereas counselling refers to advice and suggestions [Notes p.2]. Treat this as an **exam shorthand** only; many counselling models (e.g., person-centred counselling) are explicitly non-advisory.

## 6.2 Other neighbours

- **Education / psychoeducation:** transmits information and skills to a learner; psychotherapy additionally targets the person's emotional patterns. Wolberg compares educational, casework and counselling approaches and notes they overlap with psychotherapy in aiming at better adjustment but differ in emphasis [Wolberg 2013, PDF p.125].
- **Casework / social work:** classically focuses on the external problem or social situation rather than the individual's inner distress [Wolberg 2013, PDF p.130].
- **Friendship:** the professional relationship is primarily one-way (the client's concerns are discussed, not the therapist's), bounded in time, fee and intimacy, and a therapist cannot be a friend, colleague or lover to the client at the same time [Notes p.2].
- **Creative-arts, occupational and psychodrama methods:** Wolberg regards these "trial action" therapies as psychotherapeutic in effect but not, strictly, psychotherapy [Wolberg 2013, PDF p.30]; later textbooks include arts therapies as psychotherapies, so state your source's position.

## 7. The Indian legal and professional context

- BINDING LAW **RCI Act 1992, s.13:** only a rehabilitation professional who has a recognised qualification and is enrolled on the Register may practise as a rehabilitation professional in India, hold such an office in Government or sign certificates required by law; contravention is punishable (the Act text prescribes imprisonment up to one year, or fine up to ₹1,000, or both). Clinical psychologists are listed among "rehabilitation professionals" in the Act's definitions.
- BINDING LAW **MHCA 2017, s.2(1)(o):** "mental healthcare" *includes analysis and diagnosis of a person's mental condition and treatment as well as care and rehabilitation* for mental illness or suspected mental illness. The Act does **not** define "psychotherapy"; psychological treatment falls within the broader idea of mental healthcare.
- BINDING LAW **MHCA s.2(1)(g):** a "clinical psychologist" is a person with a recognised qualification in Clinical Psychology from an institution approved by the RCI, or a postgraduate degree in Psychology/Clinical/Applied Psychology plus a two-year full-time M.Phil. in Clinical Psychology (or Medical and Social Psychology) including supervised clinical training from a UGC-recognised university approved by the RCI.
- GUIDELINE **RCI Professional Diploma document (2011)** lists the services expected of diplomate clinical psychologists as interviewing and diagnosing, psychological assessment, *psychoeducation, counselling, psychotherapy and behaviour therapy*, always applying the code of conduct and ethics. This shows that, in RCI's own training language, counselling and psychotherapy are listed as separate but related services [RCI Diploma 2011, p.3]. Its Paper II Unit I ("Introduction to Psychotherapy and Counseling: Definitions, Objectives, Models of counseling, Schools of Psychotherapy, Planning and recording of counseling and therapy sessions") is almost identical to your Unit I [RCI Diploma 2011, p.15].
- SECONDARY SOURCE **Title protection.** Whether the bare word "psychotherapy" is a legally protected title in India was not established from the primary sources I read. What is clear is that practising as a clinical psychologist requires RCI enrolment (above). Check current rules before claiming otherwise.

## 8. Clinical application: explaining psychotherapy in plain language

Because consent depends on understanding (Topic 6), you must be able to define therapy simply. Good practice is to explain (i) what therapy is, (ii) what you and the client will each do, (iii) what it can and cannot promise, (iv) confidentiality limits, and (v) how progress will be checked.

ILLUSTRATIVE EXAMPLE **THERAPIST–CLIENT SITUATION (ILLUSTRATIVE)**

**Client (Meera, 27, panic attacks):** "Will you just tell me what to do?"

**Therapist:** "Therapy is a planned series of conversations and practice exercises. I bring training in methods that have helped many people with panic; you bring knowledge of your own life. Together we will understand what keeps the panic going, set goals you choose, and test new ways of responding. I won't be giving orders, and it won't always feel comfortable, but we will check your progress with simple questionnaires every week and change the plan if it isn't helping. What you tell me stays private, apart from a few safety exceptions I will explain now."

*Analysis:* this speech uses the TRAPS elements (training, relationship, approach, purpose, shared direction) and the HERS features (hope, environment, relationship, scheme); it prepares consent (Topic 6) and outcome monitoring (Topic 9).

## 9. Advantages, limitations and criticisms of definitions

| Advantages of a clear definition | Limitations / criticisms |
|---|---|
| • Clarifies roles and competence.<br>• Supports consent and rights.<br>• Makes research and training possible.<br>• Protects the public from untrained helpers. | • Over 400 approaches; no single definition fits all (Notes p.72).<br>• Medical-model wording can pathologise ordinary suffering.<br>• Definitions that stress "treatment" may exclude growth-oriented work; those that stress "conversation" are too broad.<br>• Cultural limits: many definitions presuppose Western ideas of individual autonomy (Corey 2024, PDF p.78 notes that individual choice is not a universal value).<br>• "Effectiveness" claims vary: the Notes cite 70–80% improved (Smith et al., 1980), while Lister-Ford reports that the average treated person was better off than about 80% of untreated persons in Smith & Glass's meta-analysis of 475 studies, and gives a more cautious ~70% improvement versus ~40% in untreated groups [Lister-Ford 2007, PDF p.62]. These are different statistics; do not mix them. |

The American Psychological Association's Council of Representatives adopted a 2012 resolution concluding that psychotherapy is effective and highly cost-effective, that its benefits markedly exceed those of not receiving it, and that it should be included in the health-care system as an established evidence-based practice [APA 2012 Resolution, conclusion] PROFESSIONAL CODE .

## 10. Mnemonics for Topic 1

**MNEMONIC 1 — THE FIVE ELEMENTS OF A DEFINITION**

### T · R · A · P · S

| | | |
|---|---|---|
| **T** | Trained person | Wolberg: "a trained person" |
| **R** | Relationship | Deliberate, professional, bounded |
| **A** | Approach | Psychological methods from established principles (Norcross) |
| **P** | Purpose | Relieve symptoms, change behaviour/cognition/emotion, promote growth |
| **S** | Shared direction | Change the participants deem desirable |

**Apply:** Test any statement: "A bank manager gives a worried customer friendly advice." T? No. R? Not professional-therapeutic. A? No method. Not psychotherapy.

**MNEMONIC 2 — WOLBERG'S THREE AIMS**

### R · M · P

**R**emove/modify/retard symptoms · **M**ediate disturbed behaviour patterns · **P**romote personality growth. Think: *"Relieve, Mend, Prosper."* The order reflects widening scope: symptoms » patterns » growth.

Exam-oriented summary (Topic 1)

- Psychotherapy = planned psychological treatment by a trained person in a deliberate professional relationship (Wolberg); informed and intentional use of methods from established psychological principles to help people change in directions they find desirable (Norcross 1990).
- Elements: **TRAPS**. Common healing features: **HERS**. Aims: **R-M-P**.
- Four theory families: psychoanalytic, behavioural/CBT, humanistic, family systems (+ integrative).
- Counselling = guidance/adjustment focus; assessment = data for a question; psychiatry = medical treatment; boundaries blur.
- India: RCI Act s.13 (only enrolled professionals); MHCA 2017 defines mental healthcare and clinical psychologist; the Act does not define psychotherapy.
- Evidence: APA 2012 Resolution; Smith & Glass meta-analysis.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (8 lines):** ambiguity of the term (Wolberg); etymology; talking cure; why define.
2. **Definitions (22 lines):** Wolberg (full), Norcross (full), Korchin, Bruch, Watkins, Corey; one-line comment on each.
3. **Common elements (16 lines):** TRAPS with a sentence each; draw the concept map (Figure 1.1).
4. **Features shared by all schools (12 lines):** HERS; four theory families.
5. **Psychotherapy vs counselling, assessment, psychiatric treatment (28 lines):** reproduce Table 1.4; mention that boundaries blur; Dreikurs.
6. **Indian context (12 lines):** RCI Act s.13; MHCA mental healthcare and clinical psychologist; RCI diploma services.
7. **Application (8 lines):** plain-language explanation to a client.
8. **Limitations and evidence (8 lines):** too many definitions; cultural limits; APA 2012 and meta-analysis.
9. **Conclusion (4 lines).**

## 11. Practice questions (revision practice, not predictions)

**Short 3m** State Wolberg's definition of psychotherapy and list its three aims.

**Short 3m** What are the four factors common to all schools of psychotherapy?

**Short 3m** Distinguish psychotherapy from counselling in one table.

**Long 5m** Define psychotherapy. Discuss the elements common to its definitions and distinguish it from counselling, psychological assessment and psychiatric treatment.

**Compare** Wolberg's and Norcross's definitions: which gives more voice to the client, and why does this matter?

**Application** A school teacher says, "I do therapy with my students every day because I listen to them." Using TRAPS, explain why this is or is not psychotherapy.

**– Model answer framework for the application question**

Teaching and listening show care, but check each element. **T**: teachers are trained in education, not in clinical formulation. **R**: the relationship is educational and role-bound; a therapeutic relationship is deliberately planned for change in emotion and behaviour. **A**: no systematic psychological method derived from established principles. **P**: purpose is learning, not treating a disorder. **S**: no explicit shared treatment goals or consent. Conclusion: supportive and valuable

counselling-type behaviour (and a reason to refer when serious distress appears), but not psychotherapy as defined by Wolberg/Norcross.

## 12. Sources used in Topic 1

Wolberg 2013 (PDF p.29–32, 125, 130, 133–134, 676); Notes (PDF p.1–4, 8, 71–72); Corey 2024 (PDF p.31, 78); Sharf 2010 (PDF p.159); Gabbard 2004 (PDF p.58); Lister-Ford 2007 (PDF p.62); MHCA 2017 Gazette text (ss. 2(1)(g),(o),(y)); RCI Act 1992 (s.13); RCI Professional Diploma document 2011 (p.3, 15); APA 2012 Resolution (conclusion). Full details are in the Reference section.

# 2 Objectives of Psychotherapy

Objectives answer the question “What is therapy *for*?” A strong answer distinguishes **aims** (broad purposes), **goals** (what this client wants) and **objectives** (measurable steps), lists the major objective types with reasons, and explains why they differ by client, diagnosis, approach and setting.

**AT A GLANCE**

- **Wolberg's three aims:** relieve (remove/modify/retard) symptoms · mediate disturbed patterns of behaviour · promote positive personality growth.
- **Kleinke's six common goals:** hope · mastery/self-efficacy · overcome avoidance · become aware of misconceptions · accept life's realities · insight.
- **Major objective domains:** symptom reduction · functioning · behaviour change · emotion regulation · insight · coping · relationships · relapse prevention. Mnemonic **SCRIBE-FM**.
- **Principle:** goals are negotiated, because patient, therapist and family may want different things.

## 1. Introduction and terminology

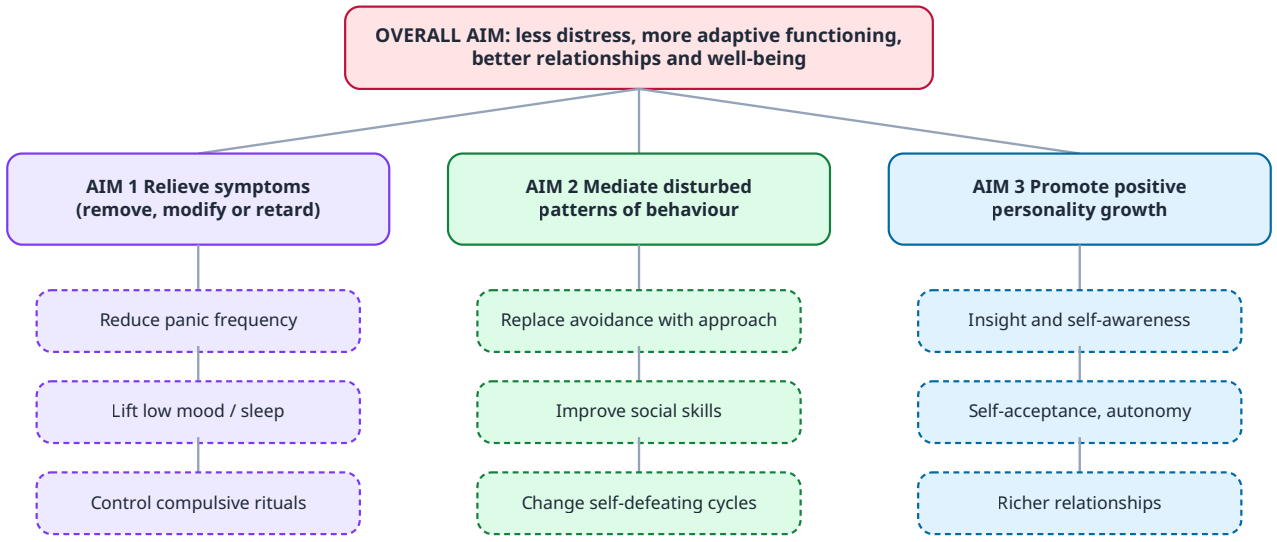
Writers use “aims”, “goals” and “objectives” loosely, but clinical documents (including treatment plans that insurers, institutions or the RCI-style records expect) use them in a stepwise sense. Jongsma and colleagues write long-term **goals** as broad, global statements of desired positive outcome that need not be measurable, whereas **objectives** must be stated in behaviourally measurable language so that it is clear when they have been achieved [Jongsma 2014, PDF p.25].

**Table 2.1 Aims, goals, objectives and interventions**

| Level | Meaning | Example (panic disorder) |
|---|---|---|
| **Aim** (purpose of therapy as a field) | General reason therapy exists (e.g., relieve symptoms, promote growth) | Reduce suffering and restore functioning |
| **Goal** (long-term, client-level) | Broad desired outcome for this person; need not be numeric | “Live without being governed by panic.” |
| **Objective** (short-term, measurable) | Behavioural, time-bound step towards the goal | “Travel by bus alone twice a week for four weeks with peak fear rated <= 4/10.” |
| **Intervention** | What the clinician does to help the client reach the objective | Interoceptive exposure; psychoeducation; breathing retraining |

## 2. Wolberg's three aims in detail

Wolberg's definition (Topic 1) names three objects of therapy. Understanding each, with its limits, lets you write far more than a list.




*Wolberg's three aims (remove symptoms · mediate behaviour patterns · promote growth); examples are illustrative.*
*Goals widen from symptom control to character and relationship change as therapy deepens (Wolberg PDF p.31–32).*

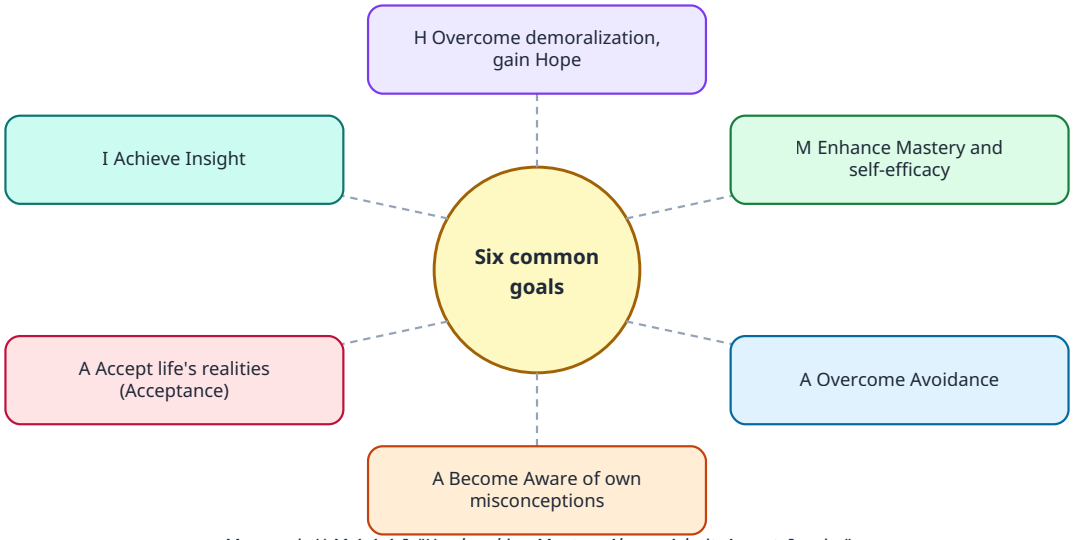
**Figure 2.1** From overall aim to specific objectives.
Source/basis: Wolberg 2013 PDF p.29, 31–32

1. **Removing, modifying or retarding existing symptoms.** A prime goal is to eliminate the patient's suffering and the handicaps imposed by symptoms. But complete relief is not always possible: inadequate motivation, limited ego strength ego strength (the positive personality assets that let a person overcome anxieties and acquire new, more adequate defences) and limits of time or money may restrict help to *modification* rather than cure. In some malignant conditions (Wolberg cites certain fulminating schizophrenic and organic brain disorders) psychotherapy can only *retard* a deteriorative process, but this palliative effect palliative (easing, not curing) is worthwhile because it may preserve contact with reality [Wolberg 2013, PDF p.31].
2. **Mediating disturbed patterns of behaviour.** Many occupational, educational, marital, interpersonal and social problems are emotionally rooted. Recognising that character structure is involved in all emotional illness has broadened the objectives from mere symptom relief to **correcting disturbed interpersonal patterns and relationships** [Wolberg 2013, PDF p.31].
3. **Promoting positive personality growth and development.** Therapy is also a vehicle for maturation, addressing blocks in psychosocial development so that a person can achieve more creative self-fulfilment, more productive attitudes towards life and more gratifying relationships [Wolberg 2013, PDF p.32]. Wolberg summarises that the goals extend “from the limited objective of helping to control symptoms to the liberation of the rich resources of the human mind” from neurotic obstructions.

**EXAM TRAP**

Do not write “Wolberg's aims are cure, adjustment and growth”. The three verbs are **remove/modify/retard**, **mediate** and **promote**. Also note that the terminology changes by author: Wolberg prefers “patient” (he argues the person receiving treatment is best called a patient) [Wolberg 2013, PDF p.31], while most counselling texts say “client”.

## 3. Kleinke's six common goals




*Mnemonic H-M-A-A-A-I: "Hardworking Mentors Always Admit, Accept, Inspire"*

**Figure 2.2** Six goals common to almost all psychotherapies (Kleinke 1994, cited in Notes p.4–5).
Source/basis: Notes-Psychotherapies PDF p.4–5

Kleinke (1994) described six fundamental therapeutic goals common to almost all psychotherapies [Notes p.4–5]:

1. **Overcoming demoralization and gaining hope.** People often arrive believing nothing can change. Restoring hope is a goal in itself and a precondition for effort.
2. **Enhancing mastery and self-efficacy** self-efficacy (belief in one's ability to carry out the actions needed to reach a goal). Success experiences (graded tasks, skill practice) convert hope into evidence.
3. **Overcoming avoidance.** Avoidance maintains anxiety and many disorders; exposure and behavioural experiments target it directly.
4. **Becoming aware of one's misconceptions** about self, others or the world (cognitive restructuring, interpretation).
5. **Accepting life's realities.** Some losses and limits cannot be undone; acceptance is a therapeutic outcome in acceptance-based and existential work.
6. **Achieving insight** insight (an understanding of the origins and development of one's emotional difficulties), leading to more rational control over feelings and actions.

## 4. A broader catalogue of counselling and therapy aims

The Notes list a wider set of aims espoused by counsellors and therapists [Notes p.72]. They are easiest to learn in clusters.

**Table 2.2 Fifteen aims grouped by emphasis (after Notes p.72)**

| Cluster | Aims in the list | Typical school |
|---|---|---|

| | | |
|---|---|---|
| Understanding the self | Insight; self-awareness; self-acceptance | Psychodynamic, humanistic |
| Growth and meaning | Self-actualisation / individuation; enlightenment (spiritual awakening); generativity and social action | Humanistic, transpersonal, existential |
| Skills and change in thinking/behaviour | Problem-solving; psychological education; social skills; cognitive change; behaviour change | CBT, behavioural, skills-based |
| Relationships and systems | Relating with others; systemic change; empowerment | Interpersonal, family, feminist |
| Repair | Restitution (making amends for previous destructive behaviour) | Restorative, addictions, forensic |

## 5. The major objectives explained (and measured)

For examination purposes, group objectives into eight domains. Each is stated, justified and linked to how it is assessed (Topic 9).

**Table 2.3 Major objective domains: meaning, reason and example measure**

| Domain | What it means | Why it is an objective | Example of how progress is shown |
|---|---|---|---|
| **S**ymptom reduction | Lower frequency, intensity or distress of symptoms | Relieves suffering; the first and most visible aim (Wolberg) | Fall in a depression or anxiety scale; fewer panic attacks per week |
| **C**oping skills | Teach strategies to manage stress, urges and setbacks | Skills outlast the sessions; CBT aims to make the patient "her own therapist" [Beck 2011, PDF p. 31] | Skill used independently between sessions (diary, behavioural test) |
| **R**elationships (interpersonal functioning) | Better quality, stability and boundaries in key relationships | Character and interpersonal patterns underlie much illness (Wolberg) | Reduced conflict; more social contact; partner/family report |
| **I**nsight | Understanding why one feels and acts as one does | Freud's "where id was, shall ego be" (Notes p.72): rational control over feelings | Client formulates own patterns; fewer repeated enactments |
| **B**ehaviour change | Replace maladaptive behaviour (avoidance, substance use, self-harm) with adaptive behaviour | Behaviour maintains disorder and is observable | Days abstinent; approach tasks completed; attendance at work |
| **E**motion regulation | Notice, tolerate and modulate emotions without avoidance or dysregulation | Both CBT and psychodynamic therapy seek to regulate negative feelings and modify distorted meanings [Notes p.5] | Fewer emotional crises; use of regulation skills; client rating of intensity |

| | | | |
|---|---|---|---|
| **F**unctioning | Return to or build valued roles (work, study, self-care, parenting) | Distress reduction without function is incomplete recovery | Work/school attendance; daily activity level; role performance |
| **M**aintenance and relapse prevention | Keep gains, anticipate setbacks, plan for high-risk times | Many disorders are recurrent; CBT "emphasizes relapse prevention" [Beck 2011, PDF p.31] with booster sessions [Beck 2011, PDF p.20] | Relapse-prevention plan; early-warning-sign list; follow-up scores |

**MNEMONIC — OBJECTIVE DOMAINS**

## S C R I B E – F M

| | |
|---|---|
| **S** | Symptom relief |
| **C** | Coping skills |
| **R** | Relationships |
| **I** | Insight |
| **B** | Behaviour change |
| **E** | Emotion regulation |
| **F** | Functioning |
| **M** | Maintenance (relapse prevention) |

**How to recall:** a *scribe* on *FM radio* reports every outcome. **Apply:** client with social anxiety — S: lower fear ratings; C: breathing/grounding; R: more friendships; I: sees safety behaviours; B: attends parties; E: tolerates anxiety; F: gives work presentations; M: plan for stress periods.

## 6. Who sets the objectives? Competing goals and ethical issues

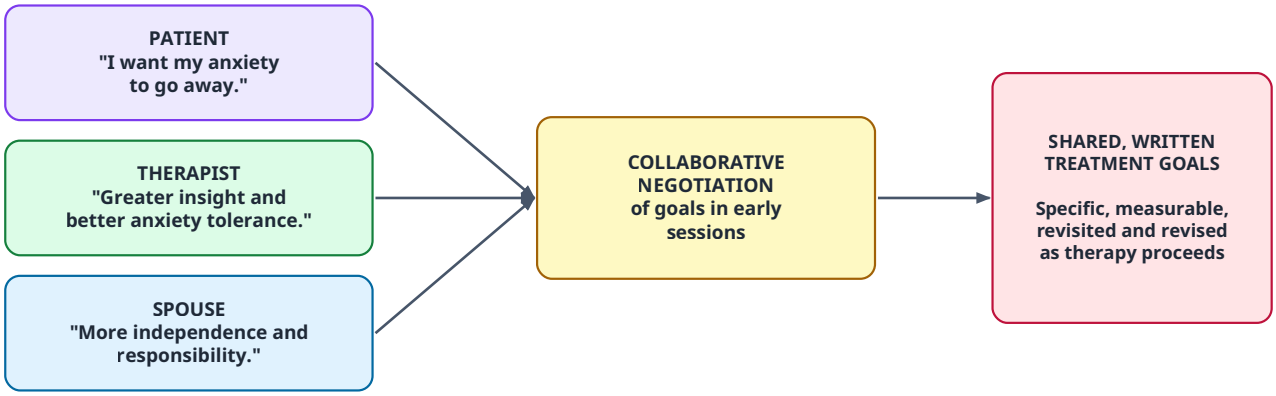



**Figure 2.3** Reconciling goals (example pattern described in Notes p.4; wording illustrative).
Source/basis: Notes-Psychotherapies PDF p.4

The goals of therapy differ for the patient, the significant others (spouse, parents, teacher) and the therapist. A patient may want less anxiety, the therapist greater insight, and the spouse more independence and responsibility from the patient [Notes p.4]. Goal conflict is therefore normal, and managing it is part of treatment.

- **Beneficence and non-maleficence** beneficence and non-maleficence (the duties to do good and to do no harm): therapy should help patients with their problems, make them feel better and do them no harm. Most therapists pursue this by increasing the patient's capacity for self-reflection and awareness of links between feelings and actions [Notes p.5].
- **Different time-courses.** CBT may make people feel better in the short term by removing symptoms, whereas psychodynamic therapy may not. A patient with trauma may want horrible feelings *taken away*; psychotherapists do not remove feelings and memories but help patients deal with them better [Notes p.5]. A CBT patient may wish to eliminate all negative emotion while the therapist aims to decrease *excessive* emotion.
- **Consent link.** Because therapists cannot always predict what patients will experience or the outcome, informed consent is complex; Notes advise warning that therapy is not always comfortable and that many patients feel worse before they feel better (though possibly not in CBT) and that some people do not get better [Notes p.5].
- **Ethical rule of thumb.** The client's autonomy is respected through *negotiation*, not imposition (Topics 5, 6, 8). The therapist may still decline goals that are harmful, illegal or beyond competence.

## 7. How objectives vary

### 7.1 By client factors

- **Age and development.** For children and adolescents, work with families and schools aims to promote and maintain complementary change [Notes p.3]. Objectives are often behavioural and environmental.

- **Severity and chronicity.** Severe mental illness may require long-term support, symptom containment and periodic booster sessions, while many patients progress with weekly sessions [Beck 2011, PDF p.78]. Brief therapies are not usually appropriate for chronic and severe illness [Notes p.24].
- **Motivation and readiness.** Following Prochaska and DiClemente, brief therapists target changes the client is *ready* for; if the client is not aware of needed changes, the goal becomes advancing readiness [Notes p.24].
- **Culture and values.** Individual choice and autonomy are not universal values; collectivist cultures may prioritise what is good for the group [Corey 2024, PDF p.78]. Goals must be culturally congruent.

## 7.2 By diagnosis

Jongsma's planner shows how objectives are written differently for each problem; for anxiety, for example, one objective is to “verbalize an understanding of the role that cognitive biases play” in worry, which is directly measurable because it either happens or not [Jongsma 2014, PDF p.24]. Compare: depression (activity, mood, hopelessness), OCD (rituals), substance use (abstinence/harm reduction), psychosis (insight, relapse signature, adherence, social functioning).

## 7.3 By treatment approach

**Table 2.4 Typical emphasis of objectives by approach**

| Approach | Characteristic objectives | Source in repository |
|---|---|---|
| CBT | Concrete, measurable change in dysfunctional thoughts, emotions and behaviour; teach the patient to be her own therapist; relapse prevention | [Notes p.4]; [Beck 2011, PDF p.31] |
| Psychodynamic | Insight into unconscious conflicts and early relational patterns; corrective emotional experience; improved relational functioning | [Notes p.3] |
| Humanistic / experiential | Actualisation, growth, self-awareness, authenticity; the person's own perception of reality | [Notes p.4] |
| Dialectical Behaviour Therapy (DBT) | Overarching goal: “a life worth living”, which includes eliminating suicidal and self-harming behaviour; therapist is coach, not “fixer” | [DBT Made Simple, PDF p.34, 72] |
| Motivational Enhancement Therapy (MET) | Rapid, internally motivated change by mobilising the client's own resources (four sessions in Project MATCH); not step-by-step guidance | [MET Manual, PDF p. 11] |
| Brief therapies | Specific focus on patterns, not broad personality change; active therapist and client; between-session tasks | [Notes p.24] |
| Family / systems | Change the relational pattern maintaining the problem; changes that benefit the family may not suit each member | [Notes p.3]; [Corsini 2011, PDF p.459] |

### 7.4 By setting

- **Outpatient clinic:** symptom and function goals within weekly sessions.
- **Inpatient / psychiatric ward:** stabilisation, safety, adherence, step-down planning.
- **Crisis intervention:** immediate safety and coping, not personality change.
- **School:** behaviour, learning, family collaboration.
- **Forensic / mandated treatment:** goals may be partly set by the court; voluntariness is an ethical problem [Notes p.6].
- **Community / low-resource:** brief, manualised interventions delivered by non-specialists, such as WHO's Problem Management Plus, aim to reduce distress and improve functioning at scale GUIDELINE [WHO PM+ article, PMC4592660].

ILLUSTRATIVE EXAMPLE **SAME PROBLEM, DIFFERENT OBJECTIVES**

Rohan, 34, has contamination-related OCD with secondary low mood. A **CBT/ERP** therapist sets: reduce ritual time from 4 hours to under 30 minutes; do graded exposure tasks daily; relapse-prevention plan. A **psychodynamic** therapist may add: understand the link between his rigid standards and early criticism; improve how he relates to his partner. A **MET**-style clinician beginning with an ambivalent Rohan first aims to strengthen his own reasons to change. A **family** clinician might help his parents stop assisting rituals. A good formulation *combines* these into an agreed plan rather than choosing one aim arbitrarily.

## 8. Indian professional and legal perspectives on objectives

- BINDING LAW **MHCA 2017** has two broad objectives: to provide mental healthcare and services for persons with mental illness, and to protect, promote and fulfil the rights of such persons in the delivery of that care (Act's long title). Therapy objectives must therefore respect those rights (Topic 5).
- GUIDELINE **RCI (2011 Professional Diploma document)** states that trainees should offer basic services by identifying and diagnosing problems and intervening with counselling and psychotherapy/behaviour therapy techniques while applying the code of conduct [RCI Diploma 2011, p.3]. Its practical examination assesses competency in conceptualising problems within behavioural, cognitive and interpersonal models, **setting treatment goals (short and long term)** and carrying out indicated interventions [RCI Diploma 2011, p.8]. Its psychotherapy record must state reasons for intervention, *areas to be focused including short- and long-term objectives*, changes in objectives and outcome [RCI Diploma 2011, p.9].
- PROFESSIONAL CODE **APA Ethics Code 10.01(a)** requires psychologists to inform clients early about the nature and anticipated course of therapy, fees, third-party involvement and confidentiality limits and to allow questions, which implies that objectives must be discussed with the client.
- SECONDARY SOURCE **IACP:** the code text was not accessed; secondary summaries describe client welfare, competence and responsibility as central themes, consistent with the above.

## 9. Limitations and challenges

- **Unrealistic or vague goals** ("be happy") cannot be monitored (see SMART goals, Topic 8).

- **Goal conflict** among client, family and therapist (Section 6).
- **Not everyone improves.** Some people do not get better with psychotherapy and some deteriorate (Lambert & Bergin, 1994, cited in [Notes p.5]); objectives and consent should acknowledge this.
- **Culture-bound objectives.** Insight and autonomy may be less valued in some cultures [Corey 2024, PDF p.78].
- **Measurement.** What matters to the client may be hard to measure (meaning, acceptance).
- **Changing aims.** Objectives should be revised as formulation evolves; RCI's record requirements explicitly ask for changes in objectives and their reasons [RCI Diploma 2011, p.9].

Exam-oriented summary (Topic 2)

- Aims » goals » objectives » interventions: from broad to measurable.
- Wolberg: **R**emove/modify/retard symptoms · **M**ediate behaviour patterns · **P**romote growth.
- Kleinke: six common goals (H-M-A-A-A-I).
- Eight domains **SCRIBE-FM**; each should be measurable.
- Goals vary with client, diagnosis, approach, setting; stakeholders differ; negotiate.
- Ethics: beneficence, non-maleficence, autonomy; warn that therapy can feel worse before better; not all improve.
- India: MHCA rights-based aims; RCI expects short- and long-term objectives in records.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (6 lines):** aims vs goals vs objectives; why they matter.
2. **Wolberg's three aims (22 lines):** each aim with its qualifier (modify, retard, character structure, maturation).
3. **Kleinke's six goals + wider aims (16 lines).**
4. **Eight domains SCRIBE-FM (28 lines):** Table 2.3 in sentences.
5. **Variation by client, diagnosis, approach, setting (22 lines):** Table 2.4, examples.
6. **Whose goals? ethics (10 lines):** stakeholders, beneficence, consent.
7. **Indian context (6 lines):** MHCA, RCI.
8. **Limitations + conclusion (8 lines).**

## 10. Practice questions (revision practice, not predictions)

**Short 3m** List Wolberg's three objectives of psychotherapy with one example each.

**Short 3m** What are Kleinke's six common therapeutic goals?

**Short 3m** Differentiate goals and objectives with an example.

**Long 5m** Discuss the objectives of psychotherapy and explain how they vary with the client, diagnosis and treatment approach.

**Compare** The objectives of CBT versus psychodynamic therapy (use Notes p.3–5).

**Application** A 30-year-old woman with a trauma history says, “I want you to take these awful feelings away.” Her husband wants her to “stop being so dependent”. How will you handle the goals?

**– Model answer framework for the application question**

1. Validate her wish; explain honestly that therapy does not remove feelings or memories but helps her deal with them better and regulate them (Notes p.5).
2. Warn that some people feel worse before they feel better, and that outcomes vary; obtain informed consent (Topic 6).
3. Meet the husband only with her consent; hear his concern as a stakeholder goal, but the client's goals take priority.
4. Negotiate shared goals: e.g., fewer flashbacks per week, safe coping skills, increased independence in two defined areas; make them SMART (Topic 8).
5. Assess risk (trauma, possible dependence) and set review points; use measures (Topic 9).
6. Document goals, consent and any revisions.

## 11. Sources used in Topic 2

Wolberg 2013 (PDF p.31–32); Notes (PDF p.3–6, 24, 72); Jongsma 2014 (PDF p.24–25); Beck 2011 (PDF p.20, 31, 78); Corey 2024 (PDF p.78); DBT Made Simple (PDF p.34, 72); MET Manual (PDF p.11); Corsini 2011 (PDF p. 459); RCI Diploma document 2011 (p.3, 8, 9); MHCA 2017 Gazette (long title); APA Ethics Code Std 10.01; WHO PM+ article (PMC4592660).

# 3 Issues Related to Training Professional Therapists

Because psychotherapy works through a relationship and can harm when done badly, *who* may deliver it and *how they are trained* are central professional issues. Examiners expect you to cover education and registration, supervised practice, personal qualities, reflective practice and supervision, cultural competence, countertransference and burnout, limits of competence and referral, and the problems of training together with workable solutions.

**AT A GLANCE**

- **Path in India:** postgraduate psychology » 2-year full-time M.Phil. Clinical Psychology with supervised clinical training (RCI-recognised) » RCI registration (Central Rehabilitation Register) » State registration under the MHCA » lifelong supervision and CPD.
- **Competence = Knowledge + Skills + Self-awareness**, built by supervised practice and sustained by supervision and CPD. Mnemonic **TRAIN-ME**.
- **Key issues:** personal qualities, boundaries, reflective practice, personal therapy debate, cultural competence, countertransference, impairment/burnout, referral and teamwork.
- **Ethics hooks:** APA Std 2.01 (boundaries of competence), 2.03 (maintain competence), 2.06 (personal problems), 10.01(c) (tell clients a trainee is supervised).

## 1. Introduction: why training is an ethical and legal issue

Wolberg warns that dealing adequately with emotional problems "requires a high degree of skill" best acquired through extensive postgraduate training and experience, and that when attempts are made to handle turmoil *in depth* without skill the relationship "may become disastrous to both participants" [Wolberg 2013, PDF p.30]. Training therefore serves three functions: it **protects clients** (non-maleficence), **protects the profession** (public trust) and **protects the therapist** (from error, burnout and liability). In India, the state backs this with law: only enrolled rehabilitation professionals (including clinical psychologists) may practise as such BINDING LAW [RCI Act 1992, s.13].

## 2. Required educational and professional preparation

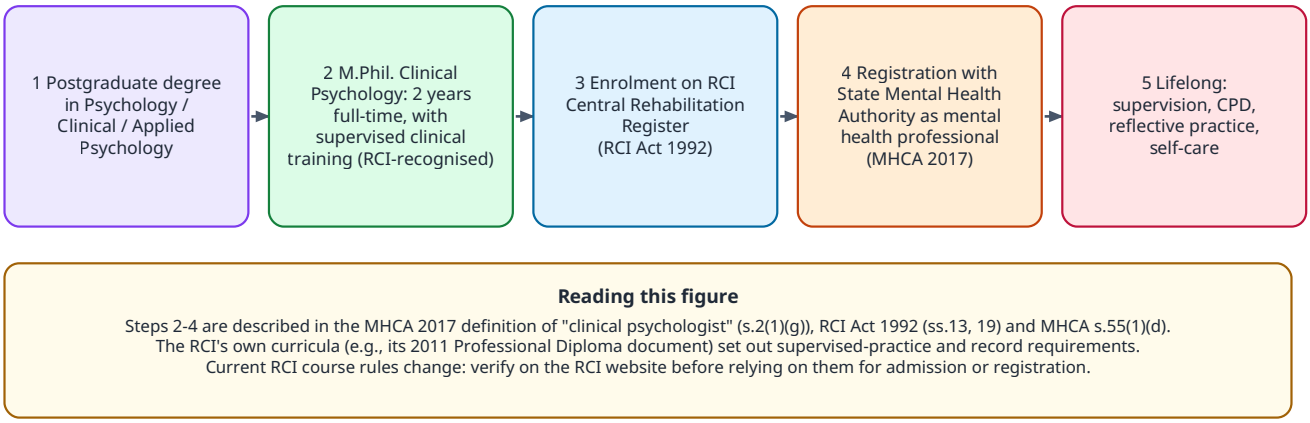



**Figure 3.1** Pathway to practise as a clinical psychologist (India).
Source/basis: MHCA 2017 Gazette text; RCI Act 1992 text; RCI Professional Diploma document 2011

### 2.1 India

- BINDING LAW **Definition of clinical psychologist (MHCA 2017, s.2(1)(g)):** (i) a recognised qualification in Clinical Psychology from an institution approved by the Rehabilitation Council of India, *or* (ii) a postgraduate degree in Psychology/Clinical Psychology/Applied Psychology **and** an M.Phil. in Clinical Psychology (or Medical and Social Psychology) obtained after a **full-time two-year course including supervised clinical training** from a UGC-recognised university approved by the RCI.
- BINDING LAW **RCI Act 1992:** the Council may prescribe minimum standards of education for recognised qualifications (s.18); the Member-Secretary enters in the Register the name of a person who has a recognised qualification (s.19); the Register is a public document (s.23); only enrolled persons may practise (s.13).
- BINDING LAW **MHCA 2017, s.55(1)(d):** the State Authority registers clinical psychologists, mental health nurses and psychiatric social workers to work as mental health professionals and publishes the list. (See also the definition of “mental health professional” in s.2(1)(r).)
- GUIDELINE **RCI's 2011 Professional Diploma in Clinical Psychology document** (a one-year course for “Clinical Psychologist (Associate)”) is useful because it states how RCI thinks about supervised training. It distinguishes the M.Phil.-qualified clinical psychologist as a full-fledged professional with higher competency and responsibility in service, academic and research areas [RCI Diploma 2011, p.2]. **These requirements are for the diploma, not the M.Phil.**, and may have been revised; use them only as an illustration:

  > Training centres need an average monthly turnover of at least 125 patients, at least two registered faculty with RCI-recognised M.Phil. in Clinical Psychology (one full-time), and at least half of cases receiving psychological treatment; face-to-face interaction only counts (tele-counselling is excluded from the count) [RCI Diploma 2011, p.4–5].
  >
  > Intake is capped at 16 per batch because training requires on-site faculty supervision; student–teacher ratios of 1:4 (lecturer level) and 1:6 (associate professor and above) [RCI Diploma 2011, p. 6].

Minimum clinical work in the year: **50 detailed work-ups, 30 psychodiagnostic cases and 20 therapy/counselling cases (about 200 hours of therapy, with at least 5 children)**, with a logbook of supervised work, and **three fully worked-out psychotherapy records** [RCI Diploma 2011, p.8–9].

- SECONDARY SOURCE The **RCI M.Phil. curriculum** and exact case numbers were not accessible. Quote them only from the RCI's current document.

### 2.2 Elsewhere (for comparison)

- **UK.** Psychotherapy as recognised by the UKCP (United Kingdom Council for Psychotherapy) is a postgraduate training of at least four years part-time, with many humanistic and integrative trainings lasting six to nine years; the aim is readiness for independent practice of long- and short-term therapies with a varied caseload, with commitments to equal opportunities and intercultural practice [Lister-Ford 2007, PDF p.114]. Training for public-sector (NHS) work additionally stresses diagnostic systems, psychopharmacology, assessment and formulation, multidisciplinary teamwork and legal–ethical frameworks [Lister-Ford 2007, PDF p.115].
- **Research trials.** In Project MATCH, therapist candidates needed at least a master's degree in a relevant field, at least two years of post-degree clinical experience and an acceptable videotaped session; training was a 3-day seminar followed by at least two supervised training cases (average three), with an average of 26 supervised sessions before certification, and ongoing monitoring against therapist "drift" [MET Manual, Appendix C, PDF p.138–140]. This shows how rigorous skill training looks when quality is the priority.
- **Task-sharing.** WHO's brief, manualised programmes (e.g., Problem Management Plus) are designed for delivery by trained and supervised non-specialists in low-resource settings GUIDELINE [WHO PM+ article, PMC4592660]; they extend access but do not replace specialist training.

## 3. Components of training and competence

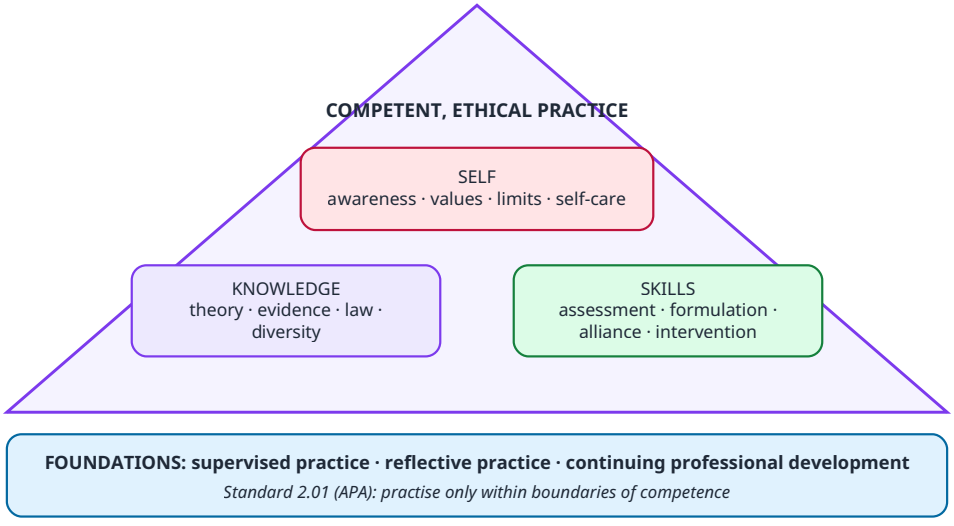



**Figure 3.2** Competence = knowledge + skills + self-awareness, sustained by supervision and CPD.
Source/basis: Corey 2024 PDF p.47–49, 62; APA Ethics Code 2017 Std 2.01, 2.03

**Table 3.1 What a trainee must acquire, how, and how it is assessed**

| Component | Content | How acquired | How assessed |
|---|---|---|---|
| **Theoretical knowledge** | Personality and psychopathology theories; schools of therapy; research methods; law and ethics; culture | Lectures, reading, seminars | Written papers; viva |
| **Assessment skills** | Clinical interview, history, tests, ratings, risk assessment | Supervised work-ups and psychodiagnostic cases | Reports, practical exams |
| **Case formulation** | Linking problems to causes and maintaining factors; choosing goals and methods | Case conferences; supervision | Psychotherapy records; viva |
| **Intervention skills** | Alliance-building, structure, specific techniques, homework, termination | Role-play, observed and recorded sessions, supervised cases | Direct observation; rating scales (e.g., Cognitive Therapy Rating Scale, Beck 2011, PDF p.16 and Appendix C) |
| **Professional competence and ethics** | Consent, confidentiality, boundaries, records, referral | Ethics teaching; supervision; ethical-decision practice | Logbook, supervisor reports |
| **Reflective practice and self-awareness** | Awareness of values, biases, countertransference, limits | Supervision, process notes, personal therapy, peer groups | Supervisor appraisal |
| **Continuing development** | New methods, updated law and evidence | CPD, conferences, supervision, audit | Registration/CPD records; outcome data (e.g., CORE system helps identify CPD needs, Lister-Ford 2007, PDF p.118) |

**MNEMONIC — BUILDING BLOCKS OF THERAPIST TRAINING**

## T R A I N – M E

| | |
|---|---|
| **T** | Theory and evidence |
| **R** | Reflective practice and self-awareness |
| **A** | Assessment and formulation |
| **I** | Intervention skills |
| **N** | Norms: ethics, law, professional standards |
| **M** | Multicultural competence |
| **E** | Experience under supervision |

**Use:** when asked “How is a therapist trained?”, walk through T-R-A-I-N-M-E and add the Indian pathway.

## 4. Personal qualities and professional boundaries

Wolberg observes that the therapist's character traits and technical skills are among the most important variables in outcome. “Effective helpers” inspire hope, faith, trust, liking and freedom to respond, and are sincere, honest, respectful and confident; Rogers' conditions of **genuineness, empathic understanding and non-possessive warmth** are cited, and Truax and Carkhuff's scales of **positive regard, accurate empathy and congruence** [Wolberg 2013, PDF p.161]. Corey lists characteristics of effective therapists: they have found their own identity, respect and appreciate themselves, are open to change, make life-oriented choices, are authentic and sincere, have a sense of humour, admit mistakes, live in the present, appreciate culture, have a sincere interest in others' welfare, possess good interpersonal skills, find meaning in their work and are passionate. He stresses that no one embodies all of them; “the willingness to struggle to become a more therapeutic person is the crucial variable” [Corey 2024, PDF p.47–49]. The relationship and the person of the therapist matter at least as much as technique in outcome research: both the relationship and methods influence results, and effective therapists use specific methods, offer strong relationships and customise both to the person [Corey 2024, PDF p.47].

**Boundaries** are the limits that separate the professional role from other roles. Beginning therapists must learn that the therapist cannot also be friend, colleague or lover to the client [Notes p.2], that boundary *crossings* (e.g., lateness, self-disclosure, accidental contact) differ from boundary *violations* (e.g., exploitation, sexual contact), and that all sexual or erotic contact between therapist and patient is a violation [Notes p.10]. Details are in Topic 4.

## 5. Reflective practice, self-awareness, supervision and personal therapy

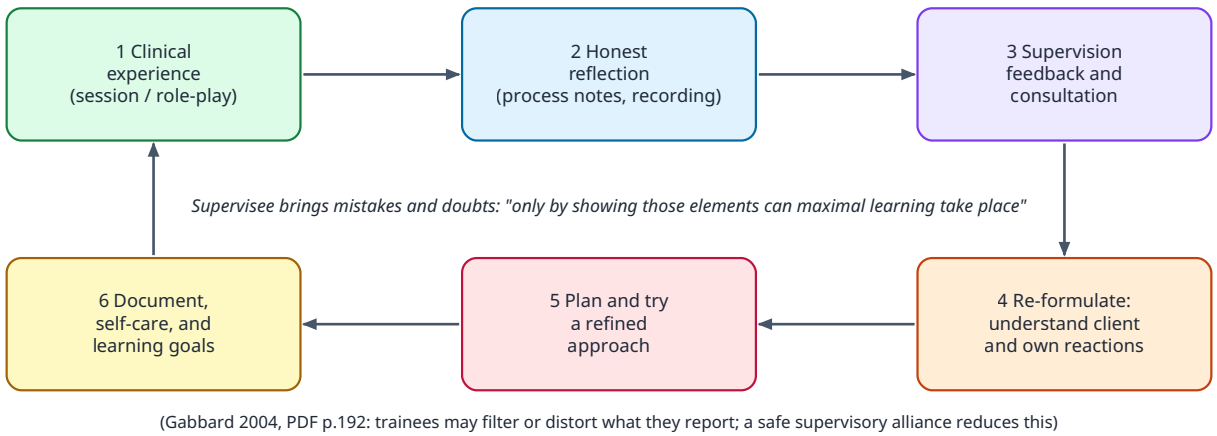



**Figure 3.3** The supervision / reflective-practice loop.
Source/basis: Gabbard 2004 PDF p.192–193; Polnay 2023 PDF p.64, 77

- **Supervision** clinical supervision (regular, structured discussion of one's cases with a more experienced professional to protect clients and develop the supervisee) is, in the psychodynamic literature, essential to reflect on transference and countertransference [Polnay 2023, PDF p.64]. Gabbard points out that the hardest part is creating an environment where trainees feel they can disclose *anything* that happens in therapy; trainees often feel "narcissistic vulnerability", may report only the patient's behaviour, and may edit or distort process material to impress. He advises supervisors to say they want to hear about mistakes and uncertainties and not to expect competence at this stage [Gabbard 2004, PDF p.192]. A key supervisory rule: the thing you most wish to avoid sharing is probably the most important to discuss [Gabbard 2004, PDF p.193].
- **Observation and recording** aid learning: behind a one-way mirror or on videotape, under a competent supervisor [Wolberg 2013, PDF p.106]. Recording requires client permission (APA Std 4.03; see Topic 7).
- **Personal therapy.** The debate is whether to require it. Corey strongly supports some form of self-exploration, noting most mental-health professionals have had personal therapy, and that the benefits include self-awareness, self-care and relationship gains [Corey 2024, PDF p.49]. Wolberg notes that a "frank encounter with oneself, buttressed if necessary by personal therapy" helps because patients may uncritically absorb the therapist's moral precepts [Wolberg 2013, PDF p.82]. Many psychodynamic trainings include personal therapy [Polnay 2023, PDF p.77].
  PROFESSIONAL CODE APA's Code regulates required therapy: where therapy is a programme requirement, students must be allowed to choose a practitioner unaffiliated with the programme, and faculty who evaluate the student's performance must not provide that therapy (Std 7.05); programmes may not require students or supervisees to disclose personal information such as sexual or abuse history, psychological treatment or family relationships unless this was clearly stated in admission materials or is needed to evaluate or help a student whose personal problems may impair training or pose a threat (Std 7.04).
- **Reflective learning cycle.** Experience » reflection » supervisory feedback » re-formulation » refined practice (Figure 3.3).

- **Practice-based learning.** A US survey reported in Lister-Ford found that even among therapists with PhDs, the most useful sources were ongoing client experience, supervision and being a client themselves, more than research articles [Lister-Ford 2007, PDF p.78]; this supports supervision-centred training but does not replace evidence (Topic 10).

## 6. Cultural competence

Competent multicultural practice involves **cultural awareness, cultural knowledge and cultural skills** (Sue et al., 2022, in [Corey 2024, PDF p.56]). Meta-analytic evidence indicates clients are more likely to engage, stay and benefit when treatment is tailored to their characteristics, so therapists should seek client feedback about cultural fit [Corey 2024, PDF p.56]. Cultural humility cultural humility (an ongoing stance of learning from the client about their culture rather than claiming expertise) and tracking satisfaction with cultural responsiveness improve engagement, retention and outcomes [Corey 2024, PDF p.59]. Therapists neglecting culture risk practising unethically; values such as individual autonomy are not universal [Corey 2024, PDF p.78]. PROFESSIONAL CODE APA Standard 2.01(b) requires training, consultation, supervision or referral when age, gender, culture, religion, language, disability or socio-economic status are essential to effective service. In India this means language, caste, religion, family structure, gender norms and help-seeking beliefs must be part of training.

## 7. Countertransference, impairment and burnout

- **Countertransference** countertransference (the therapist's emotional reactions to the client, shaped partly by the therapist's own history). Corey defines it broadly as any projection that influences how we perceive and react to a client; when unaware we may be overwhelmed or use the hour for our own purposes; personal therapy can help us recognise and manage it [Corey 2024, PDF p.62–63]. Supervision helps separate what derives from the patient from what derives from the therapist [Polnay 2023, PDF p.64].
- **Impairment** impairment (a decline in professional functioning due to personal distress, illness, substance use or burnout). You are not competent if experiencing burnout or psychological distress; it is an ethical duty to watch for impairment and act [Murdock 2017, PDF p.40]. PROFESSIONAL CODE APA Std 2.06: do not begin an activity when personal problems are substantially likely to prevent competent work; if problems arise, obtain consultation or assistance and decide whether to limit, suspend or terminate duties.
- **Burnout.** Corey stresses that therapists have considerable control over burnout, that self-care is an ethical mandate, that the helping professions involve much “one-way caring” that needs to be balanced personally, and that a wellness perspective (exercise, nutrition, sleep, relationships, meaning) is essential [Corey 2024, PDF p.64–65]. Taking excessive responsibility for clients' decisions increases burnout risk [Corey 2024, PDF p.63].

MNEMONIC — SELF-CARE AND IMPAIRMENT RESPONSE

## C · A · R · E

| | |
|---|---|
| **C** | Consult supervisor or peer early |
| **A** | Awareness of warning signs (exhaustion, cynicism, avoidance of clients, reduced empathy) |
| **R** | Rest, routine and relationships outside work |
| **E** | Evaluate whether to limit, suspend or transfer cases (APA 2.06) |

**Apply:** A trainee notices she dreads a particular client and thinks about him at night: Consult supervisor, Aware of countertransference, Rest, Evaluate caseload.

## 8. Competence limits, referral and multidisciplinary collaboration

- PROFESSIONAL CODE **APA 2.01(a):** provide services only within the boundaries of competence based on education, training, supervised experience, consultation, study or professional experience. **2.03:** undertake ongoing efforts to maintain competence.
- **Murdock** adds that attending one workshop does not necessarily make you competent in a technique, that you should ask whether it is evidence-based and whether presenters are qualified, and that competence includes knowing the areas where you are not competent [Murdock 2017, PDF p.40].
- SECONDARY SOURCE **RCI conduct regulations** (as summarised by Nyaaya) forbid practising in a field other than one's specialisation (Regulation 11(k)) [Nyaaya 2022].
- **When to refer:** client needs outside your training (e.g., acute psychosis needing medication, severe risk); a conflict that impairs objectivity (dual relationship); client not benefiting; need for a specialist assessment; your own impairment.
- **Collaboration.** Working in a multidisciplinary team with psychiatrists, social workers, nurses, school staff and family is part of training [Lister-Ford 2007, PDF p.115]. PROFESSIONAL CODE APA 10.04 advises care when offering therapy to someone already receiving services elsewhere: discuss with the client and consult other providers when appropriate.

ILLUSTRATIVE EXAMPLE **REFERRAL VIGNETTE**

Ananya, an M.Phil. trainee, is treating a student for exam anxiety. In week 3 the student reports hearing voices commenting on her and reduced sleep for ten days. Ananya has no training in psychosis assessment. She (1) assesses immediate risk; (2) tells her supervisor the same day; (3) explains to the student, with consent, that a psychiatric evaluation is needed; (4) coordinates a referral and obtains consent to share information with the psychiatrist (MHCA s.23(2)(b) permits release to other professionals to provide care); (5) documents all steps; and (6) offers continued support if appropriate. *Principle:* competence limits protect the client; referral is a professional strength.

## 9. Informing clients about trainee status

PROFESSIONAL CODE APA Standard 10.01(c): when the therapist is a trainee and the legal responsibility for treatment resides with the supervisor, the client must be told, as part of informed consent, that the therapist is in training and is supervised, and be given the supervisor's name. This is a concrete example of how training issues intersect with consent (Topic 6).

## 10. Training challenges and ways to address them

**Table 3.2 Challenges in training therapists and practical solutions**

| Challenge | Why it arises | Ways to address it |
|---|---|---|
| Anxiety and narcissistic vulnerability of beginners | Fear of failure; evaluation pressure (Gabbard) | Safe supervisory alliance; normalise mistakes; supervisors share their own errors |
| Concealing or editing material in supervision | Wish to look competent | Use recordings or verbatim notes; explicit contract about disclosure; separate formative from summative feedback |
| Theory–practice gap | Classroom knowledge vs real clients | Early observed practice; role-play; case conferences; manuals plus adaptation |
| Limited supervisors and training centres | Few RCI-registered supervisors; high caseloads (note intake and ratio limits in RCI's diploma rules) | Group supervision, tele-supervision where permitted, strict case-load caps |
| Cultural and language mismatch | Most textbooks and tools are Western | Culturally adapted material; interpreters; supervision on culture; client feedback on fit |
| Ethical dilemmas of trainees | Confidentiality and boundary issues are frequently reported by trainee therapists; a study of Indian clinical psychology trainees reported such dilemmas SECONDARY SOURCE | Ethics seminars; decision-making models (Fig. 4.2); early consultation |
| Burnout and secondary stress | Intense caseloads; one-way caring | Self-care plan (CARE); peer support; personal therapy; manageable load |
| Quality assurance | Drift from evidence-based methods | Rated sessions, adherence checks, supervision (as in Project MATCH) |

| | | |
|---|---|---|
| Over-reliance on one model | Single-school training | Exposure to integrative frameworks; formulation skills across models (Notes p.5: good therapy matches right therapy and therapist to patient) |

## 11. Ethical and legal considerations specific to training

- Never practise outside your registration and competence; a trainee works under the legal responsibility of a supervisor.
- Tell clients you are a trainee, name your supervisor, and obtain consent to supervision and to recording.
- Keep supervision discussions confidential: when consulting colleagues, do not disclose information that could identify the client unless the client consented or disclosure cannot be avoided, and disclose only what the consultation needs (APA 4.06); in teaching or writing, disguise the person, obtain written consent or have legal authorisation (APA 4.07).
- Do not exploit supervisees or students: APA Standard 7.07 deals specifically with sexual relationships with students and supervisees, and Std 3.08 prohibits exploitative relationships with supervisees (heading read from the Code).
- Maintain a logbook and records as RCI expects [RCI Diploma 2011, p.9].

Exam-oriented summary (Topic 3)

- Training protects clients, profession and therapist.
- India: PG psychology + 2-year full-time M.Phil. with supervised training (RCI) » RCI register » State registration under MHCA.
- RCI diploma illustration: on-site supervision, caps, logbook, 50/30/20 cases, three psychotherapy records (diploma, not M.Phil.).
- TRAIN-ME; competence = knowledge + skills + self-awareness.
- Personal qualities: genuineness, empathy, warmth; Corey's list; willingness to grow.
- Supervision, reflective practice, personal therapy (debated), CPD.
- Cultural competence: awareness, knowledge, skills, humility. Countertransference; impairment; burnout (CARE).
- Refer when beyond competence; tell clients about trainee status (APA 10.01(c)).

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (8 lines):** why training matters (Wolberg, harm, law).
2. **Education and registration (22 lines):** MHCA definition, RCI register, state registration, diploma illustration, UK/Project MATCH comparison.
3. **Components of training (18 lines):** Table 3.1; TRAIN-ME.
4. **Personal qualities and boundaries (12 lines).**
5. **Supervision, reflection, personal therapy, CPD (18 lines).**

6. **Cultural competence; countertransference; burnout (16 lines).**
7. **Limits, referral, teamwork (8 lines)** and trainee consent.
8. **Challenges and solutions (10 lines)** + conclusion.

## 12. Practice questions (revision practice, not predictions)

**Short 3m** Who is a "clinical psychologist" under the Mental Healthcare Act 2017?

**Short 3m** State two functions of clinical supervision and one difficulty in supervision.

**Short 3m** What does APA Standard 10.01(c) require when the therapist is a trainee?

**Long 5m** Discuss issues related to the training of professional therapists, including education, supervised practice, personal qualities, cultural competence and burnout.

**Compare** The case for and against compulsory personal therapy for trainees.

**Application** A trainee wishes to use a trauma technique learnt at a 2-day workshop on a client with complex trauma. Advise her.

**– Model answer framework for the application question**

1. Competence (APA 2.01): training, supervised experience, consultation or study are needed; a workshop alone may be insufficient (Murdock).
2. Ask whether the technique is evidence-based for this problem and whether presenters are qualified.
3. Assess client factors; complex trauma needs stabilisation and risk management; consult supervisor before any use.
4. Obtain informed consent, including that the technique is new to her and she is supervised (10.01(b), (c)).
5. If competence is lacking: refer or get supervised training first; document the reasoning.

## 13. Sources used in Topic 3

Wolberg 2013 (PDF p.30, 82, 106, 160–161); Corey 2024 (PDF p.47–49, 56, 59, 62–65, 78); Gabbard 2004 (PDF p.192–193); Polnay 2023 (PDF p.64, 77); Murdock 2017 (PDF p.40); Lister-Ford 2007 (PDF p.78, 114–115, 118); MET Manual Appendix C (PDF p.138–140); Notes (p.2, 5, 10); Beck 2011 (PDF p.16); RCI Act 1992; MHCA 2017; RCI Diploma 2011 (p.2–9); APA Ethics Code 2017 (Std 2.01, 2.03, 2.06, 4.03, 7.04–7.07, 10.01, 10.04); Nyaaya 2022; WHO PM+ (PMC4592660).

# 4 Ethical and Legal Issues Involved in Therapy Work

Ethics asks “What *should* I do?”; law asks “What *must* I do or avoid?”. They overlap but are not identical. This topic is the heart of professional practice: consent, confidentiality and its limits, competence, boundaries, records and technology, risk and safeguarding, and a systematic way of resolving dilemmas, all anchored in the Indian legal framework plus the RCI, IACP, APA and other bodies.

**AT A GLANCE**

- **Ethical principles:** autonomy · beneficence · non-maleficence · justice (+ fidelity, integrity, respect). APA's five General Principles A–E; BPS's four: respect, competence, responsibility, integrity.
- **Binding Indian law:** RCI Act 1992 (s.13 practice; s.21 conduct/ethics), MHCA 2017 (capacity, consent, confidentiality, records), POCSO 2012 (mandatory reporting), DPDP Act 2023 (data, children).
- **Confidentiality is a duty with limits.** MHCA s.23(2) lists the exceptions; mnemonic **NOCAPS**.
- **Decisions:** Corey's eight steps; mnemonic **ISCLCBED**. Document each step.
- **Conflict rule:** APA 1.02 says to clarify the conflict between ethics and law, state commitment to the Code, try to resolve it, and (if unresolvable) follow the law, but never to violate human rights.

## 1. Introduction: ethics, law and professional codes

The Notes define the ethical discourse as one of **“ought and should”** in interpersonal relationships: in psychotherapy, how therapists should behave with patients and colleagues, and what they ought to do in difficult situations. There is a close relationship between ethics and psychotherapy because ethical debate is about how we treat others, and psychotherapy explores exactly how patients relate to others; every school uses the therapeutic relationship to address the patient's problems [Notes p.9]. Whatever the school, ethical dilemmas must be *resolved one way or another*, and the overarching duty is to make the best-quality decision possible [Notes p.9].

**Table 4.1 Ethics, law and professional code compared**

| | Ethics | Law | Professional code |
|---|---|---|---|
| **Nature** | Moral reasoning about right conduct | Rules enacted by the State, enforced by courts/ authorities | Rules adopted by a regulator or association |
| **Force** | Persuasive; conscience; professional identity | Binding; penalties, liability | Binding on members/registrants (loss of membership or registration) |

| | | | |
|---|---|---|---|
| **Example** | Respect autonomy | MHCA s.23: duty of confidentiality with listed exceptions | APA Std 4.02: discuss limits of confidentiality |
| **Limit** | Different thinkers reach different answers (Tjelveit's 14 modes of reasoning, Notes p.9) | A law may be unjust: Notes recall Nazi and South African race laws (p. 10) | Cannot cover every situation; Corey: codes are guidelines for sound reasoning, not decision machines (PDF p.74) |

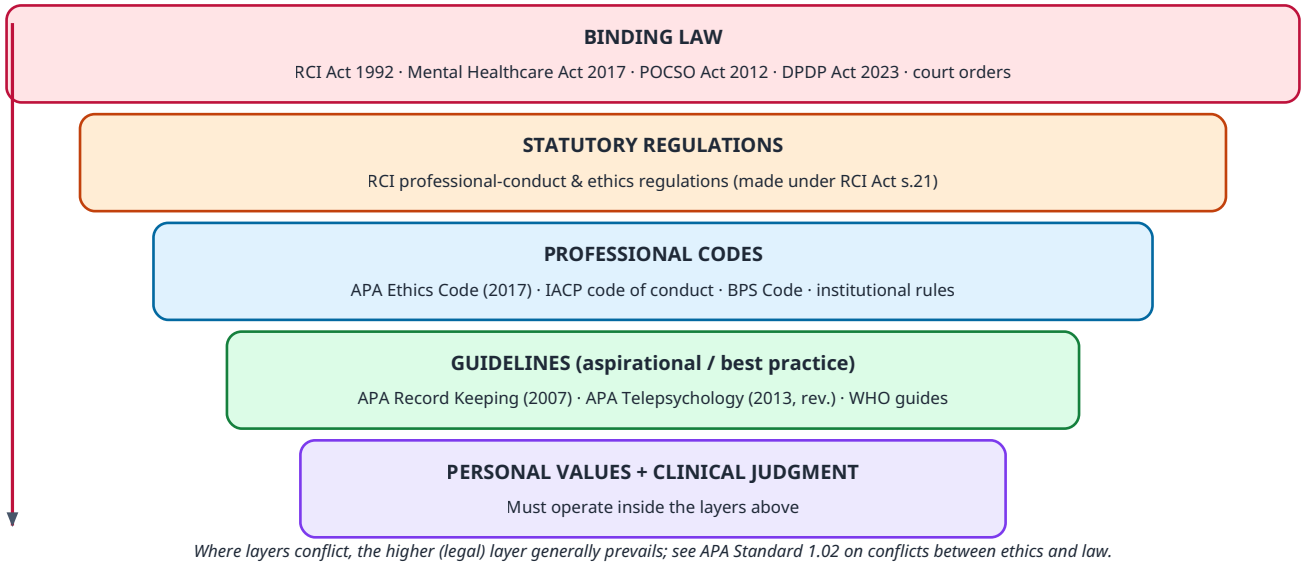



**Figure 4.1** Binding force decreases down the stack; ethical reasoning is still needed at every level.
Source/basis: RCI Act 1992 s.21; APA Ethics Code 2017 Std 1.02; Notes p.9–10

## 2. The sources of authority: who binds you, and how much

**Table 4.2 Authorities relevant to Indian clinical psychologists**

| Authority | Status | What it contributes |
|---|---|---|
| **RCI Act 1992** | BINDING LAW | Only enrolled professionals may practise (s.13); Council may prescribe standards of professional conduct, etiquette and a code of ethics, specify what is "infamous conduct"/ professional misconduct, and remove names from the Register after a hearing (s.21). |
| **RCI conduct regulations (1998)** | PROFESSIONAL CODE<br>SECONDARY SOURCE | As summarised by Nyaaya (2022): no soliciting/advertising (Reg. 3); display qualifications and registration certificate (Reg. 8); no exorbitant fees or "no payment, no cure" (Reg. 9); do not exaggerate illness (Reg. 10); no indecent activity or improper relations, harsh language, undue advantage, neglect, or practice outside specialisation (Reg. 11(b)–(k)); maintain a register of patients, prescriptions and fees (Reg. 13); violations are misconduct (Reg. 15). *Verify numbering in the primary text.* |

| | | |
|---|---|---|
| **Mental Healthcare Act 2017** | BINDING LAW | Capacity (s.4), nominated representative (ss.14–15), information (s.22), confidentiality (s.23), digital information (s. 24), access to records (s.25), dignity and equality (ss.20–21), minors' admission (s.87), decriminalised suicide attempt (s. 115). |
| **POCSO Act 2012** | BINDING LAW | Duty to report knowledge/apprehension of sexual offences against a child (s.19); failure is an offence (s.21). |
| **DPDP Act 2023 / Rules 2025** | BINDING LAW<br>SECONDARY SOURCE | Lawful processing of digital personal data; verifiable parental consent for children's data (s.9) with some health-care exemptions (Rules). Check current commencement. |
| **IACP code of conduct** | PROFESSIONAL CODE<br>SECONDARY SOURCE | Association code for professional members; the text was not accessible. Themes reported: competence, responsibility, client welfare, integrity. |
| **APA Ethics Code (2017)** | PROFESSIONAL CODE | Five General Principles (aspirational) and enforceable Standards. The Code states that Principles are not enforceable rules, while Standards are. |
| **BPS Code (2021)** | PROFESSIONAL CODE | Four principles: respect, competence, responsibility, integrity. |
| **APA guidelines** | GUIDELINE | Record Keeping (2007); Telepsychology (2013, since revised): aspirational. APA's own telepsychology guidelines explain that *guidelines* recommend while *standards* are mandatory. |
| **ICMR 2017** | GUIDELINE | Research ethics, including children's assent. |

## 3. The ethical principles

**Table 4.3 Principles and what they mean in the consulting room**

| Principle | Meaning | Clinical illustration |
|---|---|---|
| **Autonomy** (APA Principle E: respect for people's rights and dignity) | Self-determination, privacy, confidentiality; special safeguards where vulnerabilities impair autonomous decision-making | Informed consent; right to refuse or withdraw; goals negotiated |
| **Beneficence** (APA Principle A) | Strive to benefit those one works with | Evidence-informed treatment; referral when you cannot help |
| **Non-maleficence** (APA Principle A) | Take care to do no harm; resolve conflicts among obligations so as to avoid or minimise harm | Avoid exploitation; manage risks of therapy (worsening, dependency) |
| **Fidelity and responsibility** (APA Principle B) | Build trust; clarify roles; accept responsibility; manage conflicts of interest; consult, refer, cooperate | Keep contracts; role clarity with courts, employers |

| | | |
|---|---|---|
| **Integrity** (APA Principle C) | Accuracy, honesty, truthfulness; avoid fraud and misrepresentation | Truthful advertising; honest reports and billing |
| **Justice** (APA Principle D) | Fairness; equal access and quality of service for all persons | Non-discrimination; low-fee options; cultural responsiveness |
| **Respect** (BPS) | Dignity, diversity, consent, confidentiality | Same as Principle E |

MNEMONIC — APA GENERAL PRINCIPLES A–E

## B · F · I · J · R

| | |
|---|---|
| **A » B** | **B**eneficence and nonmaleficence |
| **B » F** | **F**idelity and responsibility |
| **C » I** | **I**ntegrity |
| **D » J** | **J**ustice |
| **E » R** | **R**espect for people's rights and dignity |

**Memory sentence:** "**B**e **F**aithful, **I**nspire **J**ustice and **R**espect." The APA letters A, B, C, D, E run in the same order as the sentence. **Classical four: A-B-N-J** = Autonomy · Beneficence · Non-maleficence · Justice.

## 4. Informed consent

Corey calls informed consent "an ethical and legal requirement" that is integral to therapy, establishes the basis of the working alliance and gives clients the right to be informed and to make autonomous decisions. It is "something far broader than simply making sure clients sign the appropriate forms" [Corey 2024, PDF p.75].

- **Content** (Corey): general goals; responsibilities of counsellor and client; limits and exceptions to confidentiality; legal and ethical parameters; qualifications of the practitioner; fees; services expected; approximate length; benefits and risks; and the possibility that the case will be discussed with colleagues or supervisors [Corey 2024, PDF p.75]. Written, oral or both; if oral, document it in the record [Corey 2024, PDF p.76].
- PROFESSIONAL CODE **APA 3.10:** obtain informed consent using language reasonably understandable to the person; for persons legally incapable of consent, provide an explanation, seek assent, consider preferences and best interests and obtain permission from a legally authorised person; if services are court-ordered, tell the person; document consent, permission and assent. **APA 10.01:** inform clients as early as feasible about the nature and anticipated course of therapy, fees, involvement of third parties and limits of confidentiality and allow questions; for developing treatments, inform them of the developing nature, risks, alternatives and voluntariness; if a trainee, tell the client and give the supervisor's name.

- BINDING LAW **MHCA 2017:** "informed consent" means consent given for a specific intervention without force, undue influence, fraud, threat, mistake or misrepresentation, after disclosure of adequate information including risks, benefits and alternatives in a language and manner the person understands (s.2(1)). Capacity exists if the person can understand relevant information, appreciate reasonably foreseeable consequences, and communicate the decision; information must be given in simple language or other aids; an unwise decision does not itself show lack of capacity (s.4).
- **Difficulties specific to psychotherapy** [Notes p.5–9]: (i) therapists may not predict what patients will experience or what the outcome will be; (ii) the nature of risk is unusual: untoward transference reactions, mismanaged transference/countertransference, regressive dependency and general worsening are real risks, yet few therapists warn of them; (iii) fluctuating capacity, e.g., after trauma or bereavement or in psychosis; (iv) children who agree under family pressure; (v) voluntariness in family, couple and group therapy and in forensic settings; (vi) failure to obtain consent can amount to negligence in jurisdictions with consent statutes.

MNEMONIC — VALID CONSENT

**C · V · I · D**

| | |
|---|---|
| **C** | Capacity (competence to decide; MHCA s.4) |
| **V** | Voluntary (no force, pressure, fraud) |
| **I** | Informed (nature, risks, benefits, alternatives, fees, confidentiality limits) |
| **D** | Documented (written, or oral with a record entry) |

**Apply:** before starting trauma-focused work, check that the client can understand (C), is not pressured by family (V), knows it may feel worse before better (I), and that the consent is in the file (D).

## 5. Confidentiality, privilege and their limits

### 5.1 Concepts

**Confidentiality** confidentiality (the therapist's duty not to disclose what the client reveals) is an ethical concept and usually a legal duty. **Privileged communication** privilege (a legal rule protecting confidential communications from being revealed in court without the client's permission) is a legal concept; both rest on the right to privacy [Corey 2024, PDF p.76]. Corey adds that privilege generally does not apply to group, couple, family or child/adolescent therapy, or wherever more than two people are in the room (US framework). The Notes argue that confidentiality is better understood as **informed consent to disclosure**: the therapist must obtain the patient's consent before disclosing details to anyone, including those close to the patient [Notes p.6]. The duty arises from ethical codes, licensing regulation and statute, common law and (in the US) constitutional privacy [Notes p.8].

## 5.2 Limits of confidentiality

BINDING LAW **MHCA 2017 s.23** gives a person with mental illness the right to confidentiality in respect of his mental health, mental healthcare, treatment and physical healthcare, and imposes a duty on all health professionals to keep information confidential, with these exceptions:

1. release to the **nominated representative** to enable him to fulfil his duties;
2. release to **other mental health professionals and health professionals** to enable them to provide care and treatment;
3. release if necessary to **protect any other person from harm or violence**;
4. only such information as is necessary to protect against the harm identified shall be released;
5. release only such information as is necessary to **prevent a threat to life**;
6. release on an **order** of the Board, Central Authority, High Court, Supreme Court or other competent statutory authority;
7. release in the interests of **public safety and security**.

**s.24**: no photograph or other information about a person undergoing treatment at a mental health establishment may be released to the media without that person's consent, and the right to confidentiality applies to information stored in electronic or digital form. **s.25**: persons with mental illness have a right to access their basic medical records; the mental health professional may withhold specific information if disclosure would cause serious mental harm to the person or likelihood of harm to others, and must tell the person of the right to apply to the Board for release.

MNEMONIC — MHCA S.23(2) EXCEPTIONS

**N · O · C · A · P · S**

| | |
|---|---|
| **N** | Nominated representative (to fulfil duties) |
| **O** | Other professionals providing care |
| **C** | Court / Board / statutory authority order |
| **A** | Another person at risk of harm or violence |
| **P** | Public safety and security |
| **S** | Saving life: prevent a threat to life (e.g., serious suicide risk) |

**Rule that goes with it:** release *only the minimum necessary* (s.23(2)(d)/(e)). Think "**NO CAPS** on confidentiality, but no more than needed."

PROFESSIONAL CODE **APA:** 4.01 primary obligation to protect confidential information, recognising that limits may be set by law, institutions or the relationship; 4.02 discuss limits at the outset (and thereafter as new circumstances arise), including with persons legally incapable of consent and their representatives, and tell clients of privacy risks when services are electronic; 4.05 disclose without consent only as mandated by law or where permitted by law for a valid purpose (provide needed services; obtain consultation; protect client, psychologist or others from harm; obtain payment),

limited to the minimum necessary; 4.06 consultations without identifying details unless the client consents or disclosure is unavoidable.

BINDING LAW **POCSO 2012 s.19(1)**: any person (including the child) who has apprehension that an offence under the Act is likely to be committed, or has knowledge that one has been committed, **shall** provide the information to the Special Juvenile Police Unit or the local police. s.19(7): no civil or criminal liability for giving the information in good faith. s.21(1): failure to report is punishable with imprisonment up to six months or fine or both; s.21(2): a person in charge of an institution who fails to report in respect of a subordinate faces up to one year; s.21(3): not applicable to a child.

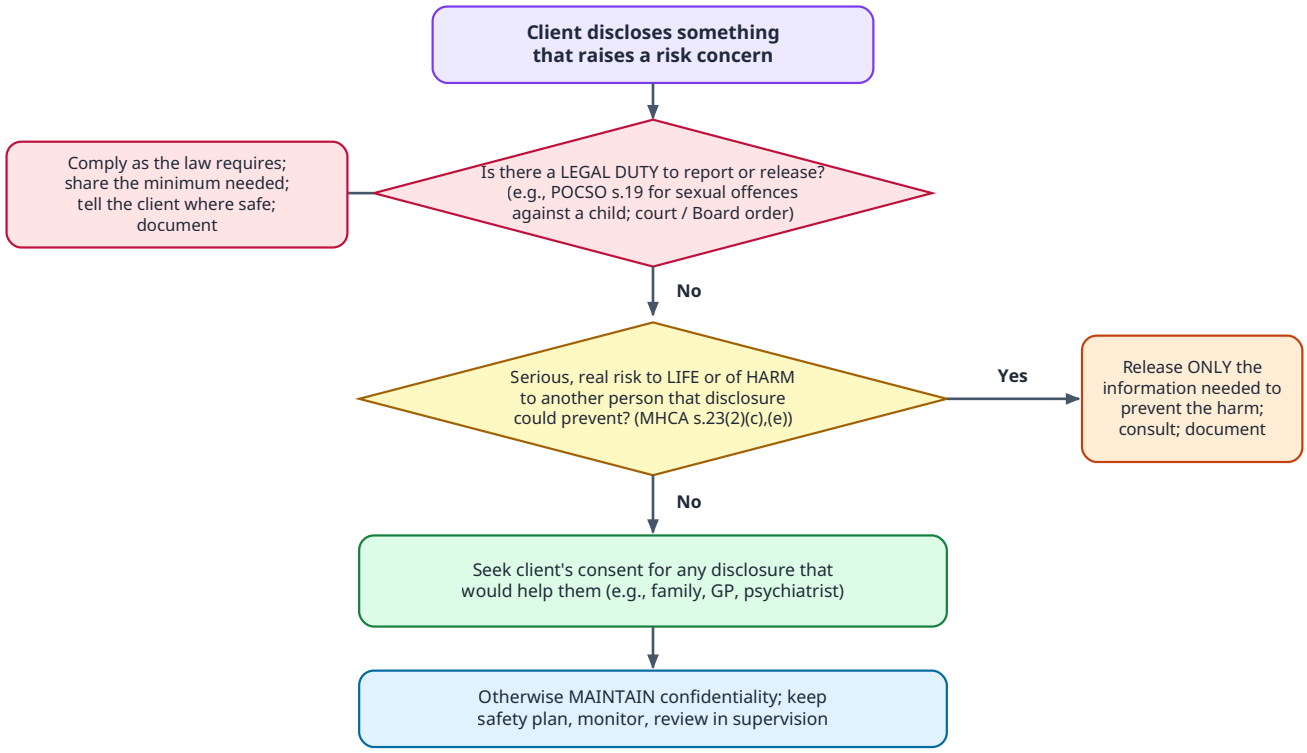



**Figure 4.3** Confidentiality: when to maintain and when to disclose (teaching schematic, not legal advice).

Source/basis: MHCA 2017 s.23(2); POCSO 2012 s.19; APA Ethics Code Std 4.01-4.05; Corey 2024 PDF p.76-77

ILLUSTRATIVE EXAMPLE **DILEMMA: THREAT TO A THIRD PARTY**

Sandeep, 41, in therapy for anger, says: "If she goes to her mother's house I will finish her." He names his wife.

1. **Identify:** a possible threat to life of a named person (ethical + legal issue).
2. **Assess risk:** intent, plan, means, history of violence, alcohol, access.
3. **Consult** supervisor immediately; check relevant law (MHCA s.23(2)(c),(d)).
4. **Options:** intensive de-escalation and safety planning with Sandeep; request voluntary hospital evaluation; notify potential victim and/or police *only as necessary*.
5. **Act and document:** what was disclosed, to whom, why, the minimum information shared, and the follow-up.
6. **Tell the client** where it does not increase danger, and revisit the consent discussion.

*Ethical balance:* autonomy and privacy (his) versus non-maleficence and protection of life (hers). The law explicitly permits releasing only the information necessary to protect against the harm identified.

## 6. Competence and integrity

PROFESSIONAL CODE APA 2.01: practise within the boundaries of competence based on education, training, supervised experience, consultation, study or professional experience; obtain training, consultation, supervision or referral when diversity factors are essential. 2.03: maintain competence. 2.04: base work on established scientific and professional knowledge. 2.06: personal problems and conflicts (Topic 3). Murdock stresses that a workshop alone does not equal competence and that being burnt out or impaired means you are not competent [Murdock 2017, PDF p.40].
SECONDARY SOURCE RCI Reg. 11(k): practise only in your field of specialisation. Integrity: APA Principle C and Standard 5.01 (heading: avoidance of false or deceptive statements); RCI regulations restrict advertising (Reg. 3) and require truthful display of qualifications (Reg. 8) SECONDARY SOURCE .

## 7. Boundaries, dual relationships, conflicts of interest and self-disclosure

The first recorded acknowledgement that a professional cannot also be the patient's lover is the Hippocratic prohibition; the boundary applies to **all** professional carers [Notes p.10]. Pope and Wedding trace the prohibition on therapist–patient sexual involvement to ancient codes and state: "No circumstances or rationale justify sexual involvement with a patient" [Corsini 2011, PDF p.597].

**Table 4.4 Boundary crossings versus boundary violations (after Notes p.10)**

| Crossings (may or may not be intended; not always harmful) | Violations (intended/initiated by the therapist; harmful or exploitative) |
|---|---|

- Lateness, earliness, changing or cancelling sessions without notice
- Self-disclosure (verbal or non-verbal)
- Discussing patient material with others, even with consent
- Arguments or jokes with patients
- Accidental contact outside sessions (common in institutions)
- Any physical contact
- Abrupt termination by the therapist without warning
- Excessive self-disclosure, especially of the therapist's distress
- Prolonged or repeated angry outbursts; humiliating or demeaning remarks
- Coercive behaviour (verbal, non-verbal or financial); financial exploitation
- Planned contact outside therapy; prolonged or repeated physical contact
- Any sexual or erotic contact; negligent therapy

- PROFESSIONAL CODE **Multiple relationships (APA 3.05):** they arise when a psychologist is in a professional role with a person and also in another role with that person, or with someone closely associated, or promises a future relationship. The psychologist refrains if it could reasonably impair objectivity, competence or effectiveness, or risk exploitation or harm; multiple relationships that would not reasonably be expected to impair or exploit are **not** unethical. If one arises unforeseen, take reasonable steps to resolve it in the client's best interests. **Conflict of interest (3.06):** refrain from a role when personal, financial, legal or other interests could impair objectivity or expose the client to harm or exploitation. **Exploitative relationships (3.08):** do not exploit clients, students, supervisees, participants or employees.
- PROFESSIONAL CODE **Sexual intimacies (APA 10.05–10.08):** prohibited with current clients; with close relatives, guardians or significant others of current clients (and therapy may not be terminated to circumvent this); therapy with former sexual partners is not accepted; with former clients, never within **two years** of termination, and after two years only in the most unusual circumstances, with the psychologist bearing the burden of showing no exploitation (seven factors listed). Murdock points out that stricter professional codes exist (e.g., five years in the counselling association's code) and that legality does not make it wise [Murdock 2017, PDF p.40].
- SECONDARY SOURCE **RCI Reg. 11(b),(c):** no indecent activity or improper relations with persons with disability; violations are misconduct that can lead to removal from the Register (Act s.21).
- **Small communities and rural/Indian settings** make some dual contact unavoidable (family doctor and neighbour; same temple). Corsini lists accepted decision aids (Gottlieb, Younggren, Sonne and others) and stresses that the topic is complex and that boundary issues relate to community, orientation and setting [Corsini 2011, PDF p.602, 605]. Practical rule: avoid when possible; if unavoidable, discuss, minimise, document and consult.
- **Self-disclosure:** Corey warns of two extremes: hiding behind a professional facade, or burdening clients with spontaneous impressions and personal material. Consider **what, when and how much** to reveal, and examine your motives; self-disclosure should serve the client [Corey 2024, PDF p. 60]. Excessive disclosure of the therapist's distress is classified as a violation [Notes p.10].
- **Fees and gifts:** APA 6.04 (agree fees early; heading) and 6.05 (barter with clients) limit financial entanglements; RCI Reg. 9 prohibits exorbitant fees and “no payment, no cure” deals SECONDARY SOURCE . APA 6.03: do not withhold records needed for a client's emergency treatment solely because payment has not been received.

ILLUSTRATIVE EXAMPLE **DUAL-RELATIONSHIP VIGNETTE**

In a town with one school, a school counsellor's own child studies in the class of a student she counsels. **Analysis:** multiple relationship risk (counsellor + parent of classmate). Steps: consider whether another counsellor can see the student; if not, discuss roles and limits openly, avoid discussing the student's issues with her child, record the arrangement, and review with a supervisor. APA 3.05 permits a multiple relationship only if it is not reasonably expected to impair objectivity or risk harm.

## 8. Records, privacy, digital communication and telepsychology

- PROFESSIONAL CODE **APA 6.01–6.02:** create, maintain, store, retain and dispose of records to facilitate later services, meet institutional requirements, ensure billing accuracy and comply with law; maintain confidentiality in creating, storing, accessing, transferring and disposing of records in any medium; remove personal identifiers when data enter shared databases; plan in advance for protecting records if you withdraw from practice. **APA 4.03:** obtain permission from all persons (or legal representatives) before recording voices or images.
- GUIDELINE **APA Record Keeping Guidelines (2007):** in the absence of a superseding requirement, psychologists *may consider* retaining full records until 7 years after the last service for adults, or 3 years after a minor reaches majority, whichever is later; protect security of paper and electronic records. Details in Topic 7.
- BINDING LAW **MHCA s.24(2):** confidentiality extends to information stored in electronic or digital format “in real or virtual space”.
- **Technology and privacy.** Corey advises discussing technology risks during consent, being careful with emails or texts to a client's workplace or home, and limiting exchanges to basic information such as appointment times [Corey 2024, PDF p.75–76].
- **Telepsychology.** Pope and Wedding's scenario shows the legal puzzles when therapist and client are in different jurisdictions: licensing, authority of boards, standards of competence for online therapy, which state's confidentiality and reporting laws apply, emergency plans, and billing transparency [Corsini 2011, PDF p.594–595]. GUIDELINE APA's Guidelines for the Practice of Telepsychology (2013) cover eight areas: competence with the technology; standards of care; informed consent; confidentiality of data; security and transmission of data; disposal of data and technologies; testing and assessment; and interjurisdictional practice. A revised 2024 version has been published; check the current text. For India: apply RCI/MHCA obligations, secure platforms, verify identity and location, plan for emergencies, and see DPDP duties SECONDARY SOURCE .

## 9. Risk assessment, safeguarding and protective action

- **Risk assessment** risk assessment (a structured estimate of danger to self or others, including intent, plan, means, history and protective factors) is a core duty. CBT therapists review items on hopelessness and suicidality on measures such as the BDI-II and, if elevated, conduct a risk assessment and develop a safety plan before proceeding [Beck 2011, PDF p.85]. Structured tools such as the Columbia Suicide Severity Rating Scale (in the repository) support this.
- BINDING LAW **MHCA s.115(1):** a person who attempts suicide is presumed, unless proved otherwise, to have severe stress and shall not be tried and punished under the Indian Penal Code

provision cited in the Act. Therapists should therefore treat suicidality as a clinical, not criminal, matter.

- **Children:** POCSO s.19 and s.21 (above). Corey reports that there is a legal requirement to break confidentiality in cases of child abuse, abuse of the elderly or dependent adults, and danger to self or others, and that practitioners must know these duties [Corey 2024, PDF p.77]; Murdock adds that in the United States every state mandates reporting of child abuse [Murdock 2017, PDF p.39] (a US comparison; Indian duties are in POCSO). See Topic 6 for adolescents.
- **Principles of protective action:** assess; consult; try less intrusive options first (safety plan, voluntary admission, family involvement with consent); disclose the minimum necessary; document reasons; follow up.

## 10. Law and the therapist: duty of care and standard of care

Psychotherapeutic relationships are not outside civil or criminal law. Statutes and case law that apply to medical treatments also apply to therapy: therapists acquire a **legal duty of care** and must practise in accordance with a reasonable body of (psychotherapeutic) opinion; exercise reasonable skill; make logical treatment decisions; conform to ethical guidance where it exists; gain informed consent; and tell the truth when asked [Notes p.11]. Negligence claims against psychotherapists have increased [Notes p.8]. Distinguish: **civil** liability (compensation), **criminal** liability (e.g., POCSO non-reporting), and **professional discipline** (RCI removal from the Register).

**WHEN ETHICS AND LAW SEEM TO CONFLICT**

PROFESSIONAL CODE APA 1.02: clarify the nature of the conflict, make known your commitment to the Ethics Code, and take reasonable steps to resolve it consistent with the Code's principles; if it is unresolvable, you may adhere to the law, but “under no circumstances” may this be used to justify violating human rights. Example: a statutory reporting duty (POCSO) overrides a client's wish for secrecy, and the therapist should inform the client of this limit at the outset and handle the report supportively.

## 11. A step-by-step ethical decision-making framework

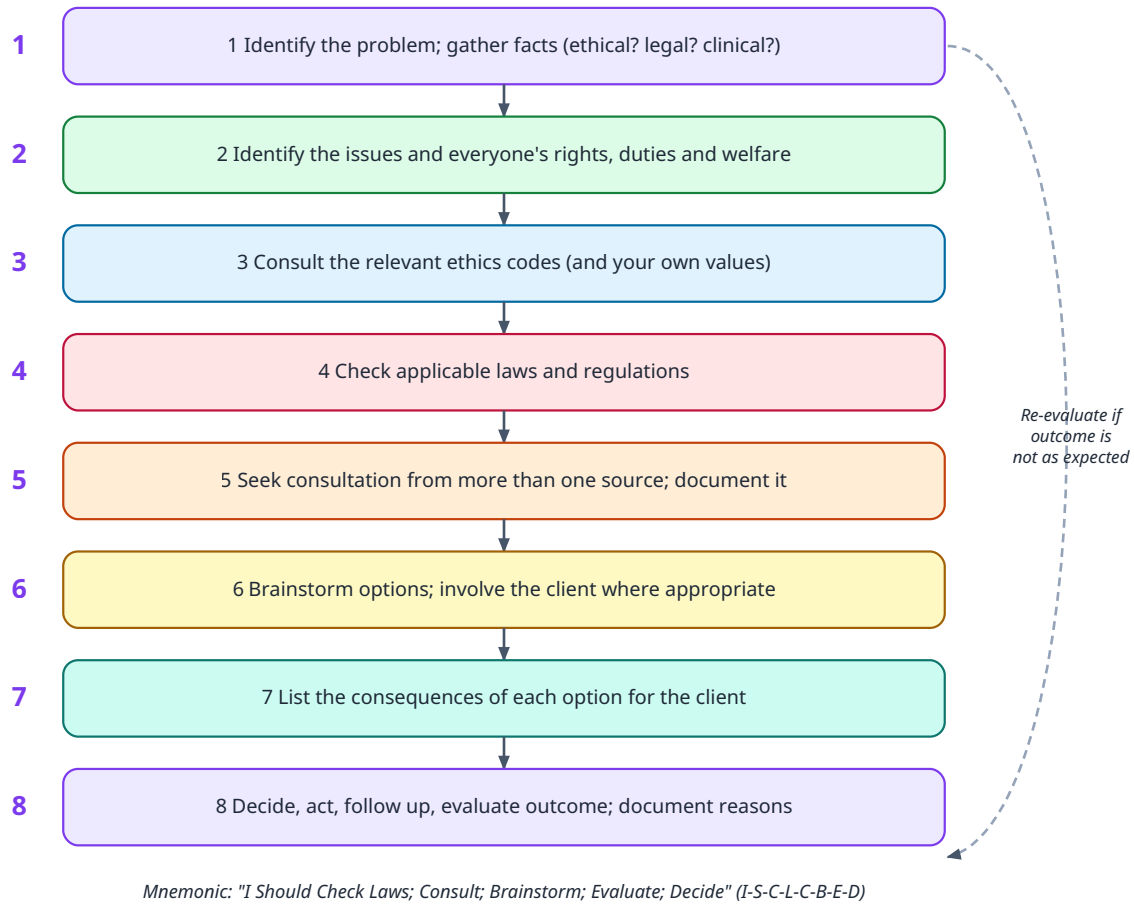



**Figure 4.2** Ethical decision-making steps (after Corey et al., 2024).
Source/basis: Corey 2024 PDF p.74–75

Corey and colleagues propose these steps, and stress that practitioners may make different decisions, that you should consult more than one source, document the consultation, involve the client in considering options and in documenting how, and follow up [Corey 2024, PDF p.74–75].

**MNEMONIC — EIGHT STEPS OF ETHICAL DECISION-MAKING**

## I · S · C · L · C · B · E · D

| | |
|---|---|
| **I** | Identify the problem (ethical? legal? clinical?) |
| **S** | Stakeholders and issues: rights, duties, welfare |
| **C** | Codes: check ethics codes and own values |
| **L** | Laws and regulations |
| **C** | Consult more than one source; document |
| **B** | Brainstorm options (include the client) |
| **E** | Evaluate consequences of each option |
| **D** | Decide, act, follow up, document |

**Sentence:** "**I S**hould **C**heck **L**aws; **C**onsult; **B**rainstorm; **E**valuate; **D**ecide." **Apply:** run every dilemma in the exam through these eight words.

### – Other approaches to ethical reasoning (supplementary)

Tjelveit (1999) identified at least 14 modes of ethical reasoning that may be used in psychotherapy, including casuistry (specific circumstances), classic liberal individualism (autonomy and justice), communitarianism (interests of society), feminist ethics, hermeneutics, narrative, pragmatic, rational, religious, virtue ethics and others [Notes p.9]. In practice you will most often combine principle-based reasoning (A-B-N-J), virtue ethics (what would a person of good character do?) and consequentialist thinking (what outcomes follow?). Corsini lists widely used decision models for boundary dilemmas (Gottlieb 1993; Lamb & Catanzaro 1998; Younggren 2002; Sonne 2006) [Corsini 2011, PDF p.605].

## 12. Common ethical dilemmas and suggested handling

**Table 4.5 Frequent dilemmas**

| Dilemma | Principles in tension | Suggested approach |
|---|---|---|
| Parent demands session details of an adult son in therapy | Confidentiality vs family pressure | Explain confidentiality; seek client's consent for agreed information; listen to the parent without disclosing; document. Notes p.7: where a third party asks you not to tell the patient about contact, balance honesty against possible harm. |

| | | |
|---|---|---|
| Client declines to inform a partner of a serious infection | Confidentiality vs protection of third party | Encourage disclosure; consult; apply the law and professional guidance; disclose only if justified (the Notes discuss this in UK context). |
| Therapist wishes to use case material in teaching or publication | Public good vs autonomy | Obtain consent, disguise (APA 4.07); some journals require the patient to have read the article (Notes p.7). |
| Client asks for a certificate/diagnosis letter | Client's interests vs clinical judgment | A therapist is not obliged to write a letter on request, but should explain reasons and consider the client's needs; follow institutional rules and law SECONDARY SOURCE (Nyaaya reader discussion; verify). |
| Client shows no improvement | Beneficence vs financial interest | Review, refer, or end (APA 10.10: terminate when the client no longer needs the service, is unlikely to benefit, or is being harmed; offer pretermination counselling and alternatives). |
| Social media friend request from client | Boundaries | Decline politely in line with the consent document; document; avoid dual contact. |
| Observing a colleague's unethical conduct | Loyalty vs protection of clients | APA 1.04: informal resolution first when appropriate and consistent with confidentiality; otherwise report (1.05 reporting; RCI complaint route). Murdock: first try informal resolution, then file a complaint with the professional organisation or authority [Murdock 2017, PDF p.40]. |

## 13. Advantages, limitations and challenges of ethical codes

| Advantages | Limitations / challenges |
|---|---|
| • Protect the public and professionals.<br>• Provide common language and shared standards.<br>• Support consent, confidentiality and fairness.<br>• Basis for complaints and discipline. | • Cannot cover every situation; need judgment (Corey PDF p.74).<br>• Mostly Western in origin; may not fit collectivist contexts (Corey PDF p.78).<br>• Different bodies differ (e.g., 2-year versus 5-year rules).<br>• Legal duties (e.g., reporting) can conflict with client wishes.<br>• Defensive practice can harm therapy: Notes p.8 urge working knowledge of clinical–legal issues to avoid maladaptive defensive practices. |

Exam-oriented summary (Topic 4)

- Ethics = ought; law = must; code = binding on members. Know the hierarchy.
- Principles: A-B-N-J; APA B-F-I-J-R; BPS: respect, competence, responsibility, integrity.
- Consent: CVID. MHCA s.4 capacity; s.2 informed consent.
- Confidentiality exceptions (MHCA s.23(2)): NOCAPS; minimum necessary. POCSO s.19 reporting; s.21 penalty.

- Boundaries: crossing vs violation; APA 3.05; 10.05–10.08 (2-year rule); self-disclosure: what/when/how much.
- Records and tech: APA 6.01–6.02, 4.03, MHCA s.24(2), telepsychology guidelines (8).
- Decision-making: ISCLCBED. If law and ethics conflict: APA 1.02.
- RCI Act s.13 (practice) and s.21 (misconduct, removal).

Answer skeleton: 5-mark long answer (~120–150 lines)

1. **Introduction (8 lines):** ethics/law/codes; hierarchy figure.
2. **Principles (12 lines):** A-B-N-J; APA A–E; BPS.
3. **Consent (16 lines):** content, APA 3.10/10.01, MHCA definition and capacity, difficulties.
4. **Confidentiality and limits (24 lines):** concepts, MHCA s.23 exceptions, APA 4.01–4.06, POCSO.
5. **Competence and boundaries (22 lines):** Table 4.4, APA 3.05, 10.05–10.08, self-disclosure.
6. **Records, digital, telepsychology (12 lines).**
7. **Risk and safeguarding (10 lines).**
8. **Indian legal framework (12 lines):** RCI Act, MHCA, POCSO, DPDP.
9. **Ethical decision-making steps (12 lines)** with a dilemma; conclusion (4 lines).

## 14. Practice questions (revision practice, not predictions)

**Short 3m** List the exceptions to confidentiality under s.23 of the Mental Healthcare Act 2017.

**Short 3m** Distinguish confidentiality from privileged communication.

**Short 3m** What is a boundary crossing, and how does it differ from a boundary violation?

**Short 3m** State the steps of ethical decision-making.

**Long 5m** Discuss the ethical and legal issues in psychotherapy with reference to Indian law and the codes of the RCI, IACP and APA.

**Compare** APA's Principles and Standards; or ethical guidelines versus legal requirements.

**Ethical dilemma** A 16-year-old tells you in confidence that her uncle has been touching her. She begs you not to tell anyone. What do you do? (see also Topic 6)

**– Model answer framework for the dilemma**

1. **Identify:** disclosure of possible sexual abuse of a child; legal duty under POCSO s.19(1) applies to any person with knowledge or apprehension; failure to report is punishable (s.21).
2. **Respond to the girl:** believe, thank, do not promise secrecy; explain, in simple words, that you cannot keep this secret because it is about her safety and the law requires reporting, and that you will stay with her.
3. **Consult** supervisor immediately; check agency reporting protocol; avoid contacting the suspected perpetrator or any family member who may be implicated.
4. **Report** to the Special Juvenile Police Unit/local police as required; ensure safety and medical care; involve child protection resources.
5. **Document** the disclosure verbatim, steps and consultation; share only the minimum.
6. **Continue therapy**, supporting her through the process.

Ethical logic: protection from serious harm and compliance with law outweigh confidentiality, which was properly limited at the outset (APA 4.02).

## 15. Sources used in Topic 4

Notes (PDF p.5–11); Corey 2024 (PDF p.60, 74–78); Corsini 2011 (PDF p.594–597, 602, 605); Murdock 2017 (PDF p.38–40); Beck 2011 (PDF p.85); RCI Act 1992 (ss.13, 18, 19, 21, 23); MHCA 2017 (ss.2, 4, 14–15, 22–25, 55, 87, 115); POCSO 2012 (ss.2(d), 19, 21); APA Ethics Code 2017 (1.02, 1.04, 2.01–2.06, 3.04–3.10, 4.01–4.07, 6.01–6.03, 10.01–10.10); APA Record Keeping Guidelines 2007; APA Telepsychology Guidelines 2013; Nyaaya 2022; BPS web pages; DPDP commentary (secondary).

# 5 Rights and Responsibilities in Psychotherapy

A therapy relationship is a two-way arrangement with unequal power. Rights protect the weaker party (the client); responsibilities ensure the process works and protect the therapist. A complete answer presents **client rights, client responsibilities, therapist rights and therapist responsibilities** in a comparison table, and then explains how to handle attendance, fees, communication, termination, complaints, disagreements, treatment refusal and requests for records.

**AT A GLANCE**

- **Client rights (PRIDE):** Privacy & confidentiality · Respect & non-discrimination · Information · Decide (consent, refuse, withdraw) · Exit & Escalate (end therapy, complain).
- **Client responsibilities (HAFT):** Honesty · Attendance · Fees · Taking part (collaboration, homework, safety plan).
- **Therapist responsibilities (3C + B + 3R):** Competence · Consent · Confidentiality · Boundaries · Records · Referral/continuity · Respect.
- **Law:** MHCA 2017 Chapter V (ss.18–28) is the key statute of rights for persons with mental illness; APA 3.10, 4.02, 6.03–6.04, 10.01, 10.10 give the professional detail.

## 1. Introduction: rights, duties and the therapeutic contract

A **right** is an entitlement that others must respect; a **duty** is an obligation owed to someone; a **responsibility** is a role-related expectation that may be ethical, contractual or legal. In therapy, the client's rights usually create matching duties for the therapist (Figure 5.1). Client responsibilities are mostly *collaborative expectations* agreed in the consent process, not legal obligations: failing to attend or pay does not entitle a therapist to breach confidentiality or abandon an unsafe client.

Corey notes that by educating clients about their rights *and* responsibilities the therapist both empowers them and builds trust, and that this education begins in the first session and continues throughout [Corey 2024, PDF p.75]. Murdock lists what clients should be told: the approach, the rights and responsibilities of each party, potential risks, expected benefits, the counsellor's qualifications, fees and billing, and the limits of confidentiality [Murdock 2017, PDF p.39].

The Indian statutory statement of rights is the Mental Healthcare Act 2017, whose long title commits it to provide mental healthcare and to *protect, promote and fulfil the rights* of persons with mental illness during delivery of care. It was drafted to harmonise Indian law with the UN Convention on the Rights of Persons with Disabilities (preamble). Where a client is not a “person with mental illness” as defined, the same principles still apply as ethical standards (APA, RCI conduct rules, IACP code).

## 2. Client rights

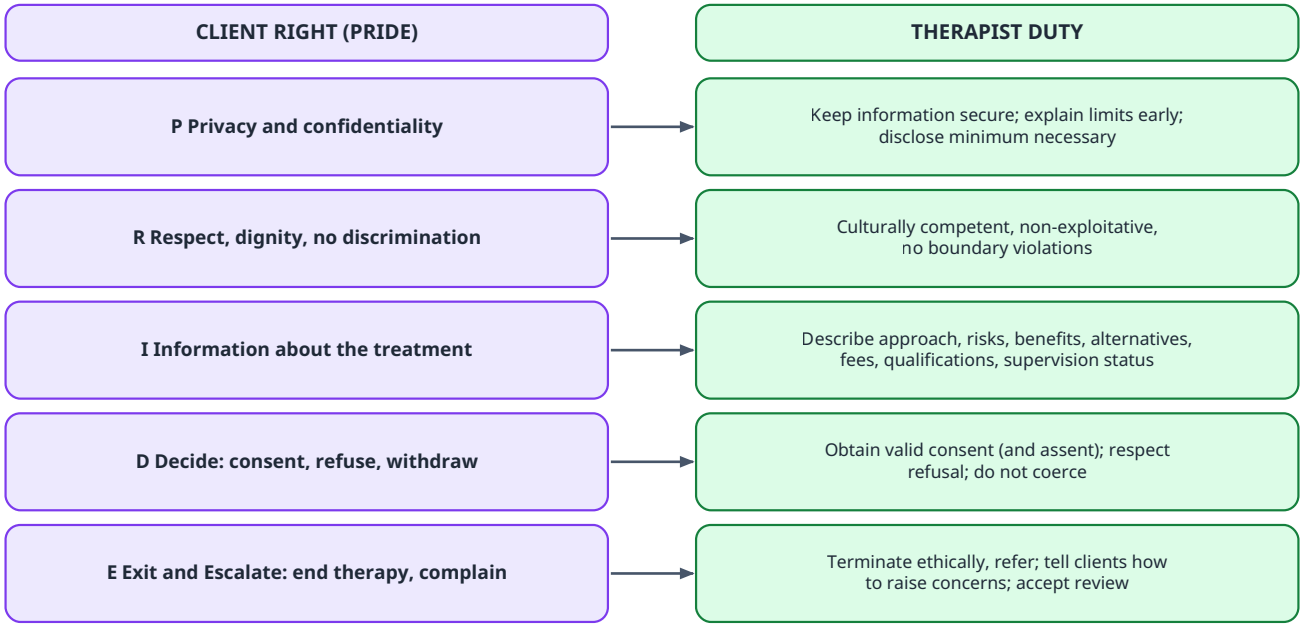



**Figure 5.1** Rights imply duties. Mnemonic **PRIDE**.
Source/basis: MHCA 2017 ss.4, 20-25; APA Ethics Code Std 3.10, 4.01-4.02, 10.01, 10.10; Corey 2024 PDF p.75

**Table 5.1 Client rights, with Indian statute and professional-code support**

| Right | Meaning in therapy | Indian law / code | APA / other |
|---|---|---|---|
| **Dignity and respect** | Be treated as a person, free from cruel, inhuman or degrading treatment, humiliation or exploitation | BINDING LAW MHCA s.20: right to live with dignity; protection from cruel, inhuman or degrading treatment in a mental health establishment, including privacy SECONDARY SOURCE RCI Reg. 11 (no harsh language, undue advantage) | PROFESSIONAL CODE Principle E; Std 3.08 (no exploitation); 10.05 (no sexual intimacy); BPS: respect |
| **Equality, non-discrimination** | Same quality of care regardless of gender, sexual orientation, religion, culture, caste, class, disability | BINDING LAW MHCA s.21(1)(a); s.18(2) | PROFESSIONAL CODE Std 3.01; Principle D (justice) |
| **Informed consent, refusal and withdrawal (autonomy)** | Decide about starting, continuing or stopping therapy after understanding it; a poor decision does not by itself show incapacity | BINDING LAW MHCA s.4 (capacity); s.2 definition of informed consent | PROFESSIONAL CODE Std 3.10, 10.01 (including voluntary nature of developing treatments) |

| | | | |
|---|---|---|---|
| **Information about treatment** | Know the nature of the problem, proposed treatment and known side effects, in a language and form understood; early information on fees, third parties, limits of confidentiality; trainee status | BINDING LAW MHCA s.22(1)(c),(d); s.4(2) | PROFESSIONAL CODE Std 10.01(a),(c); 6.04 (fees); 4.02 |
| **Privacy and confidentiality** | Information stays protected, including in electronic form; disclosure only within defined limits | BINDING LAW MHCA s.23, s. 24 | PROFESSIONAL CODE Std 4.01–4.07; 6.02 |
| **Access to one's records** | See basic medical records; if specific information is withheld, be told and be able to apply to the Board | BINDING LAW MHCA s.25 | GUIDELINE APA Record Keeping Guidelines; Notes p.7: patient access may be legally possible unless harmful |
| **Safe, competent service** | Treatment by a qualified person using accepted methods; no abandonment | BINDING LAW RCI Act s.13 (only enrolled professionals); SECONDARY SOURCE RCI Reg. 11(h),(i) timely care, no neglect | PROFESSIONAL CODE Std 2.01; 10.10; 3.12; 10.09 |
| **Complaint and redress** | Complain about deficiencies in care and seek review or legal remedy | BINDING LAW MHCA s.28 (to the professional in charge, then the Board, then the State Authority; judicial remedy remains); RCI Act s.21 (professional misconduct) | PROFESSIONAL CODE Std 1.04–1.05 (peer conduct) |
| **Legal aid and contact** | Free legal services to exercise rights; personal contacts and communication in an establishment | BINDING LAW MHCA s.27, s. 26 | — |
| **Recording and use of material only with permission** | Voices/images recorded or case material used for teaching/research only with permission | BINDING LAW MHCA s.24(1) (media); GUIDELINE ICMR 2017 for research | PROFESSIONAL CODE Std 4.03, 4.07, 8.03 |

**MNEMONIC — CLIENT RIGHTS**

## P · R · I · D · E

| | |
|---|---|
| **P** | Privacy and confidentiality (incl. digital) |
| **R** | Respect, dignity, non-discrimination |
| **I** | Information about treatment, fees, risks, therapist |
| **D** | Decide: consent, refuse, withdraw |
| **E** | Exit and Escalate: end therapy, access records, complain |

**Use:** a client says, "I don't want to continue, and I want my file." D (decide to stop) + E (exit, records) + I (be told if anything is withheld and why).

## 3. Client responsibilities

Therapy depends on active participation of both client and therapist [Notes p.2]. Typical responsibilities are agreed in the consent discussion; they are expectations, not legal duties, and they do not override the client's rights.

**Table 5.2 Client responsibilities and the reasoning behind each**

| Responsibility | Why it is expected | Limit |
|---|---|---|
| **H**onesty about relevant history, symptoms, risk and other treatments | Accurate formulation and safety; allows coordination with others (APA 10.04 asks the therapist to consider treatment elsewhere) | Client is not required to disclose everything; withholding in itself is not misconduct |
| **A**ttendance and punctuality; give notice if cancelling | Continuity, safe use of limited sessions; agreed in contract | Illness, crisis and cultural factors are valid reasons; missed sessions are clinical information, not just a breach |
| **F**ees paid as agreed | Sustains service; APA 6.04: agree compensation early | Therapist must give warning before collection measures (6.04(e)) and cannot withhold records needed for emergency treatment solely for non-payment (6.03) |
| **T**aking part: collaboration, between-session tasks, safety plans, feedback | Change occurs largely outside sessions; CBT teaches the patient to be her own therapist (Beck, PDF p. 31) | Homework is negotiated; non-completion is explored, not punished |
| Respect for the therapist's boundaries and policies | Maintains the frame (time, fee, intimacy) | Clients may ask questions about the therapist's qualifications and approach |

**MNEMONIC — CLIENT RESPONSIBILITIES**

## H · A · F · T

**H**onest · **A**ttend · **F**ees · **T**ake part. Think: *"the client holds **HAFT**, the handle of the tool"*; a tool without a handle (client effort) cannot be used.

## 4. Therapist rights

- **To practise within competence and decline cases beyond it** (APA 2.01; SECONDARY SOURCE RCI Reg. 11(k)). Not every request must be accepted; for example, a therapist is not obliged to write a diagnostic letter simply because a client asks, and may decide on clinical grounds (view expressed in Nyaaya's discussion; check local rules) SECONDARY SOURCE .
- **To fair remuneration** as agreed, and to use collection procedures after giving notice (APA 6.04(e)), within law; RCI prohibits exorbitant fees SECONDARY SOURCE .
- **To safety and respect.** A psychologist may terminate therapy when threatened or otherwise endangered by the client or by someone connected with the client (APA 10.10(b)).
- **To consult** supervisors and colleagues (APA 4.06) and to receive supervision.
- **To set professional policies** (cancellation, availability, communication channels, social media), provided they are disclosed in advance and are lawful.
- **To fair process.** Before removing a name from the Register the RCI must give the person a reasonable opportunity of being heard (RCI Act s.21(3)) BINDING LAW .
- **To withhold specific record information** if disclosure would cause serious mental harm to the person or likely harm to others (MHCA s.25(2)), while informing the person of the right to apply to the Board BINDING LAW .
- **Limits:** the therapist cannot assume to "own" the notes absolutely: patients may have legal access to records in some jurisdictions unless access would harm them or others [Notes p.7].

## 5. Therapist responsibilities

**Table 5.3 Core professional responsibilities**

| Responsibility | Content | Source |
|---|---|---|
| **C**ompetence | Practise within training, maintain competence, seek supervision, refer | APA 2.01, 2.03, 2.06; RCI Act s.13 |
| **C**onsent | Obtain informed consent and assent; explain fees, risks, confidentiality limits, trainee status | APA 3.10, 10.01; MHCA s.2, s.4, s.22 |
| **C**onfidentiality | Protect information; discuss limits early; release only the minimum necessary | MHCA s.23–24; APA 4.01–4.07; POCSO s.19 |
| **B**oundaries | No exploitation, sexual contact or harmful dual relationships | APA 3.05, 3.08, 10.05–10.08; RCI Reg. 11 SECONDARY SOURCE |

| | | |
|---|---|---|
| **R**ecords | Keep accurate, secure records; respond to access requests lawfully | APA 6.01–6.02; MHCA s.25; RCI Reg. 13 SECONDARY SOURCE |
| **R**eferral and continuity | Refer when appropriate; plan for interruption; terminate ethically with pretermination counselling and alternative providers | APA 3.12, 10.09, 10.10 |
| **R**espect and justice | Non-discrimination; cultural responsiveness; fair and honest billing and advertising | APA 3.01, 5.01, 6.04–6.06; MHCA s.21; RCI Reg. 3, 8, 9 SECONDARY SOURCE |
| Accountability | Cooperate with complaints and regulators; report serious breaches by colleagues as appropriate | RCI Act s.21; APA 1.04–1.05; MHCA s.28 |

**MNEMONIC — THERAPIST RESPONSIBILITIES**

## 3C + B + 3R

**3C** = Competence, Consent, Confidentiality. **B** = Boundaries. **3R** = Records, Referral/continuity, Respect. **Apply:** in any case vignette, scan the list in order and name which responsibility is at stake.

## 6. Comparison table: rights and responsibilities side by side

**Table 5.4 Client and therapist rights and responsibilities in key areas (as requested in the syllabus)**

| Area | Client rights | Client responsibilities | Therapist rights | Therapist responsibilities |
|---|---|---|---|---|
| **Dignity and respect** | Respectful, non-discriminatory care | Treat therapist and staff respectfully | Safe, respectful working conditions; may end therapy if threatened (APA 10.10(b)) | Treat with dignity; avoid exploitation; cultural responsiveness |
| **Consent and information** | To be informed; to consent, refuse, withdraw | Ask questions; give honest information to allow informed decisions | To define the services offered and policies | Provide full information in understandable language; document consent/assent |

| | | | | |
|---|---|---|---|---|
| **Confidentiality and limits** | Privacy of disclosures; to be told the limits at the start | Respect others' privacy (e.g., group members); do not record sessions without permission | To share necessary information with supervisors; to disclose within legal limits to protect life | Maintain confidentiality; explain NOCAPS limits; release minimum necessary |
| **Attendance and timing** | Reasonable scheduling; notice of therapist's absence and arrangements for coverage | Attend; arrive on time; give notice of cancellation | To apply the cancellation policy that was agreed in advance | Be punctual; give notice; plan for interruption (APA 3.12) |
| **Fees and billing** | To know fees early; accurate bills; not to be refused emergency records because of unpaid fees (APA 6.03) | Pay as agreed; raise financial difficulty early | To fair payment; to use collection procedures after warning (APA 6.04(e)) | Agree fees early, do not misrepresent, no “no payment, no cure” deals; honest reports to payors (APA 6.04–6.06; RCI Reg. 9 SECONDARY SOURCE ) |
| **Communication between sessions** | To know how and when the therapist can be reached and what to do in an emergency | Use agreed channels; use emergency services for crises | To set limits on availability and channels | State contact policy; keep digital communication secure and limited (Corey p.75–76) |
| **Safety and risk** | Protection; involvement in safety planning | Follow the safety plan; tell the therapist about risk | To act to protect life within law | Assess and manage risk; report as required (POCSO s.19; MHCA s. 23(2)) |
| **Records** | Access to basic records (MHCA s.25); correction requests | Request in writing; respect confidentiality of others named in records | To withhold specific information that would cause serious harm (s. 25(2)), with reasons | Keep accurate, secure records; respond lawfully; retain/dispose safely |
| **Complaints and disagreement** | To complain and seek review (MHCA s.28; RCI) | Raise concerns first with the therapist where safe; give feedback | To be informed of the complaint and heard (RCI Act s.21(3)) | Welcome feedback; explain complaint routes; cooperate; do not retaliate |

| | | | | |
|---|---|---|---|---|
| **Treatment refusal** | To refuse; unwise choices do not by themselves show incapacity (MHCA s.4(3)) | Discuss reasons and risks | To decline to continue if therapy cannot be safe or beneficial; to refer | Explore reasons; give options; document; avoid coercion |
| **Termination** | To end at any time; to pretermination counselling and alternatives | Discuss intention to stop, ideally in a closing session | To terminate when no benefit, harm, or endangerment (APA 10.10) | Terminate ethically; offer referrals; avoid abandonment; relapse prevention |

## 7. Handling difficult situations

### 7.1 Complaints

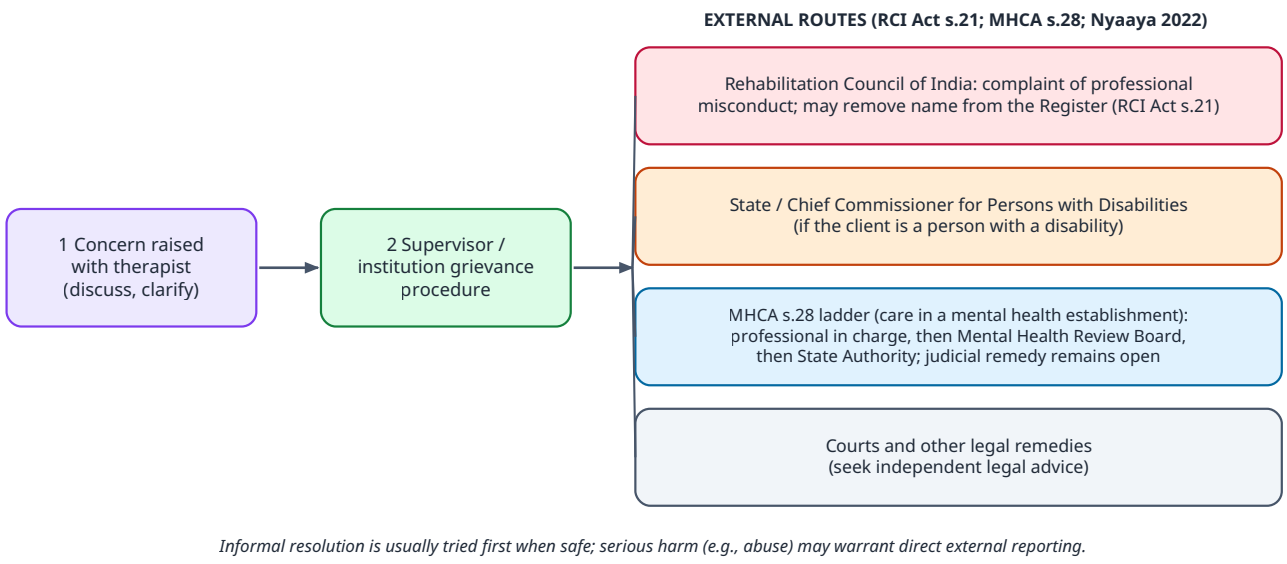



**Figure 5.2** Where a client or colleague can take a concern.
Source/basis: RCI Act 1992 s.21; MHCA 2017 s.28; Nyaaya 2022 (secondary); Murdock 2017 PDF p.40

1. Invite the client to voice the concern to you; listen without defensiveness; clarify, apologise where appropriate, and agree remedies.
2. Offer the institution's grievance procedure or supervisor.
3. Inform about external options: RCI (professional misconduct; Register); State/Chief Commissioner for Persons with Disabilities where the client has a disability; and, for care in a mental health establishment, the MHCA s.28 ladder (professional in charge, then the Board, then the State Authority). Judicial remedies remain open [MHCA 2017 s.28(2)]. Routes 2–3 are described by Nyaaya SECONDARY SOURCE .
4. Never retaliate, threaten or breach confidentiality to defend yourself except as law permits; consult your supervisor or legal adviser; document.

## 7.2 Disagreements about the plan

Revisit goals collaboratively (Topic 8). If the disagreement persists, consider a second opinion, a different approach or referral. Therapists should be honest that outcomes are uncertain and that people may feel worse before better [Notes p.5].

## 7.3 Treatment refusal

Competent adults may refuse. BINDING LAW MHCA s.4(3): a decision perceived as inappropriate or wrong does not by itself mean lack of capacity. Steps: check capacity (understand, appreciate, communicate); explore reasons (fear, cost, stigma, family pressure); provide information in simple language; offer alternatives (lower intensity, different approach, family sessions); document informed refusal; maintain an open door; if risk is high, follow the risk pathway (Topic 4). For minors, see Topic 6.

## 7.4 Requests for records

1. Confirm identity and authority (client, nominated representative, court order, other professional with consent).
2. For persons with mental illness, MHCA s.25(1) grants access to basic medical records; specific information may be withheld if serious mental harm or harm to others is likely (s.25(2)); inform the person of the right to apply to the Board (s.25(3)).
3. Separate your **clinical record** from **personal process notes** (Topic 7); do not assume process notes are beyond reach [Notes p.7].
4. Do not withhold records needed for emergency treatment solely because of unpaid fees (APA 6.03).
5. Document the request, decision, date and what was released.

## 7.5 Missed sessions, non-payment and communication

Discuss early: APA 6.04(d) requires discussion as early as feasible of limits to services because of financing. If a client frequently misses sessions, treat it clinically first (ambivalence, avoidance, practical barriers) before applying penalties. State response times and emergency routes in writing.

## 7.6 Termination

APA 10.10: terminate when it is reasonably clear that the client no longer needs the service, is not likely to benefit, or is being harmed; may terminate if threatened; before termination provide pretermination counselling and suggest alternative providers as appropriate (except where precluded by client or payor actions). Notes record that about half of terminations appear to be unilateral, initiated by the client [Notes p.24]. Wolberg's Q&A explains why patients resist termination: failure to resolve a positive transference, fear of giving up the comfortable niche of treatment, separation anxiety, and sometimes the therapist's own difficulty in letting go [Wolberg 506 Q&A, PDF p.31]. CBT plans termination from the start, spacing sessions, building relapse-prevention skills and offering booster sessions [Beck 2011, PDF p.78].

## 8. A sample therapy agreement (informed-consent contract)

ILLUSTRATIVE EXAMPLE

### – Elements to include (adapt to your institution and law)

1. Who I am: qualifications, registration (RCI), supervisor (if trainee), approach.
2. What therapy involves; likely benefits; risks (including feeling worse for a time); alternatives; how progress will be measured.
3. Session length, frequency, location/platform; what happens if either of us is late.
4. Fees, payment, cancellation policy; what happens if you cannot pay; insurance/third-party payors.
5. Contact between sessions; response times; emergency plan; out-of-hours.
6. Confidentiality and its limits: threat to life, risk to others, child abuse/sexual offences against minors, court order, consultation with supervisors, sharing with other professionals for your care (NOCAPS).
7. Records: what I keep, where, how long, how you can ask to see them; recording of sessions only with your written permission.
8. Digital communication: security, risks, jurisdiction, what you should not send.
9. Your rights and responsibilities (PRIDE and HAFT); how to complain.
10. Ending therapy: how we will decide; referrals; follow-up.
11. Signatures and date (and assent/guardian consent for minors).

Exam-oriented summary (Topic 5)

- Rights » duties. PRIDE (client rights); HAFT (client responsibilities); 3C+B+3R (therapist responsibilities).
- MHCA ss.18–28: access, dignity (s.20), equality (s.21), information (s.22), confidentiality (s. 23), digital data (s.24), records (s.25), contacts (s.26), legal aid (s.27), complaints (s.28).
- Termination (APA 10.10): no need/benefit/harm/threat; pretermination counselling; alternatives.
- Records: s.25; withholding only if serious harm; inform of right to apply to Board.
- Responsibilities of clients are collaborative expectations, not licences to breach rights.
- Complaints: therapist » institution » RCI / disability commissioner / MHCA ladder / courts.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (8 lines):** rights vs duties; power asymmetry; MHCA purpose.
2. **Client rights (24 lines):** PRIDE with MHCA sections.
3. **Client responsibilities (10 lines):** HAFT with limits.
4. **Therapist rights (10 lines).**

5. **Therapist responsibilities (16 lines):** 3C+B+3R.
6. **Comparison table (20 lines):** Table 5.4 in condensed form.
7. **Practical handling (16 lines):** complaints, refusal, records, termination.
8. **Ethical-legal note + conclusion (6 lines).**

## 9. Practice questions (revision practice, not predictions)

**Short 3m** List five rights of a client in psychotherapy.

**Short 3m** What does s.25 of the Mental Healthcare Act 2017 provide about access to records?

**Short 3m** When may a psychologist terminate therapy under APA Standard 10.10?

**Long 5m** Discuss the rights and responsibilities of client and therapist in psychotherapy, presenting them in a comparison table.

**Compare** Rights versus responsibilities of clients; ethical duty versus legal duty of the therapist.

**Application** A client with unpaid fees asks for his case summary to give to a new psychiatrist. What should you do?

**– Model answer framework for the application question**

1. The request relates to continuing care; APA 6.03 says records needed for *emergency* treatment may not be withheld solely for non-payment; more generally, withholding to pressure payment is ethically risky.
2. Check consent: the client himself is asking, so releasing a summary to the psychiatrist is consistent with MHCA s.23(2)(b) (release to other professionals for care) and with his right of access (s.25).
3. Release a clinically adequate summary, minimum necessary; document.
4. Address the fee issue separately and respectfully (APA 6.04(d),(e)): discuss payment options; if collection is intended, give notice first.

## 10. Sources used in Topic 5

MHCA 2017 (preamble; ss.4, 18–28); RCI Act 1992 (ss.13, 21); APA Ethics Code 2017 (1.04–1.05, 3.01, 3.08, 3.10, 3.12, 4.01–4.07, 6.01–6.06, 10.01, 10.04–10.10); APA Record Keeping Guidelines 2007; Corey 2024 (PDF p.75–76); Murdock 2017 (PDF p.39); Notes (p.2, 5, 7, 24); Beck 2011 (PDF p.31, 78); Wolberg 506 Q&A (PDF p. 31); Nyaaya 2022 (secondary).

# 6 Issues Related to Consent (and Assent in the Case of Minors)

Adults give **informed consent**. Children and adolescents usually cannot give legally valid consent, so a parent or legal guardian consents while the minor gives **assent** assent (the child's own agreement to take part, after an age-appropriate explanation). A good answer explains both, the minor's developing capacity, confidentiality and parental involvement, risk disclosures, disagreements, and the Indian legal position, without pretending there is one rule for every minor.

**AT A GLANCE**

- **Consent** = legally effective permission (capacity + voluntariness + information + documentation: **CVID**). **Assent** = the minor's agreement, sought as an ethical duty, not a legal substitute for consent.
- **APA Std 3.10(b)** for persons legally incapable of consent: **E**xplain · seek **A**ssent · consider **P**references/best interests · obtain **P**ermission from a legally authorised person (mnemonic **EAPP**).
- **Indian law:** MHCA "minor" = under 18; legal guardian is the nominated representative (s.15) unless the Board orders otherwise; POCSO s.19 reporting; DPDP s.9 parental consent for children's data SECONDARY SOURCE .
- **No single rule:** what is required depends on the applicable law, the circumstances and the type of service (therapy, assessment, research, school counselling, emergency).

## 1. Introduction

The Notes remind us that the ethical principle behind consent is **respect for autonomy**, and that legally valid consent must be voluntary, given by a competent person, and adequately informed [Notes p.5]. Children are a group who may lack competence to make treatment decisions, and the problem is rarely global incompetence but fluctuating capacity: the Notes ask how to interpret a child's refusal of therapy that was accepted a day earlier, as an informed choice or as evidence that the work has begun; they also warn of children who "agree" only under family pressure [Notes p.6]. Consent in family, couple and group work is harder still because members may agree only because of pressure from their emotional network.

## 2. Informed consent versus assent

**Table 6.1 Consent and assent compared**

| | Informed consent | Assent |
|---|---|---|
| **Who gives it** | Competent adult; for a minor, the parent/legal guardian (or other legally authorised person) | The minor (child or adolescent) |

| | | |
|---|---|---|
| **Legal force** | Legally effective permission to treat | Not by itself legally sufficient; an ethical and (in some research rules) required addition |
| **Content** | Nature of therapy, risks, benefits, alternatives, fees, limits of confidentiality (MHCA s.2; APA 3.10, 10.01) | The same ideas in language the child can understand; what will happen, who will know, that they may ask questions or say no |
| **If refused** | Treatment cannot proceed with that consent | The wish should be respected where possible and explored; "mere failure to object" is not assent (ICMR 2017, in research) |
| **Documentation** | Signed form or record entry | Child's own signature or a note of oral assent; age-appropriate form |

Murdock explains that clients under 18 generally cannot give informed consent to treatment (with a few exceptions), so parent or guardian consent is needed; depending on age, counsellors should also obtain assent by explaining the consent document in a way the child understands and getting agreement, sometimes with a written assent form. It is particularly important that minors understand what may be disclosed to parents, while recruiting parents as collaborators [Murdock 2017, PDF p.39].

## 3. Legal and professional framework

- PROFESSIONAL CODE **APA Standard 3.10(b):** for persons who are legally incapable of giving informed consent, psychologists nevertheless (1) provide an appropriate explanation, (2) seek the individual's assent, (3) consider such persons' preferences and best interests, and (4) obtain appropriate permission from a legally authorised person if substitute consent is permitted or required by law; if not, they take reasonable steps to protect the person's rights and welfare. **3.10(d):** document consent, permission and assent. **4.02(a):** discuss limits of confidentiality with persons legally incapable of consent and their representatives *to the extent feasible*.
- BINDING LAW **MHCA 2017:** a "minor" is a person who has not completed 18 years (s.2(1)). The legal guardian is the minor's **nominated representative** unless the Board, on application by a mental health professional or other person acting in the minor's best interest, finds the guardian is not acting in the minor's best interest or is unfit, and appoints another suitable person (s.15). A legal guardian may make an advance directive for a minor until majority (s.5(4)). Admission of a minor to a mental health establishment follows s.87: the nominated representative applies; two professionals independently examine the minor; admission must be in the minor's best interests, taking the minor's wishes into account if ascertainable, with community alternatives having failed. s.4 capacity (understand, appreciate, communicate) is framed for any person, with information given in simple language or other aids.
- BINDING LAW **POCSO 2012:** "child" = a person below 18 (s.2(d)); any person, including the child, who has knowledge or apprehension of a sexual offence shall report to the Special Juvenile Police Unit or local police (s.19(1)); reports from children are recorded in simple language, with an interpreter if needed (s.19(3),(4)); good-faith reporters are protected (s.19(7)); failure to report is an offence (s.21), but s.21 does not apply to a child (s.21(3)).
- BINDING LAW SECONDARY SOURCE **DPDP Act 2023, s.9:** a data fiduciary must obtain verifiable consent of the parent or lawful guardian before processing personal data of a child (under 18),

with exemptions in the Rules for certain healthcare uses. Commencement is phased; check the current position when handling digital records, tele-sessions or apps with minors.

- GUIDELINE **ICMR 2017 (research):** children are individuals who have not attained the legal age of consent (up to 18); parental/LAR consent is required; in addition verbal/oral or written assent, as approved by the ethics committee, should be obtained from children aged 7–18; if the child objects, the wish must be respected; mere failure to object is not assent (§6.5.3–6.5.4). This is a *research* rule and should not be transferred automatically to routine therapy.
- SECONDARY SOURCE **RCI:** no separate consent rule for minors was found in the sources accessed; RCI's diploma document requires at least 5 children among the 20 supervised therapy cases, which makes this a core skill [RCI Diploma 2011, p.9].

**THERE IS NO SINGLE UNIVERSAL RULE**

Whether a 16-year-old can consent alone to counselling, whether school counselling needs written parental consent, and whether a clinician may see a minor in an emergency without a parent depend on the statute, the setting, institutional policy and urgency. The sources I could verify do not set a specific age at which a minor can consent alone to outpatient psychological therapy in India. In practice, obtain guardian consent as the legal default, seek the minor's assent, and ask your institution or lawyer when unsure. Never claim a rule you cannot cite.

The flowchart below turns these rules into a practical sequence.

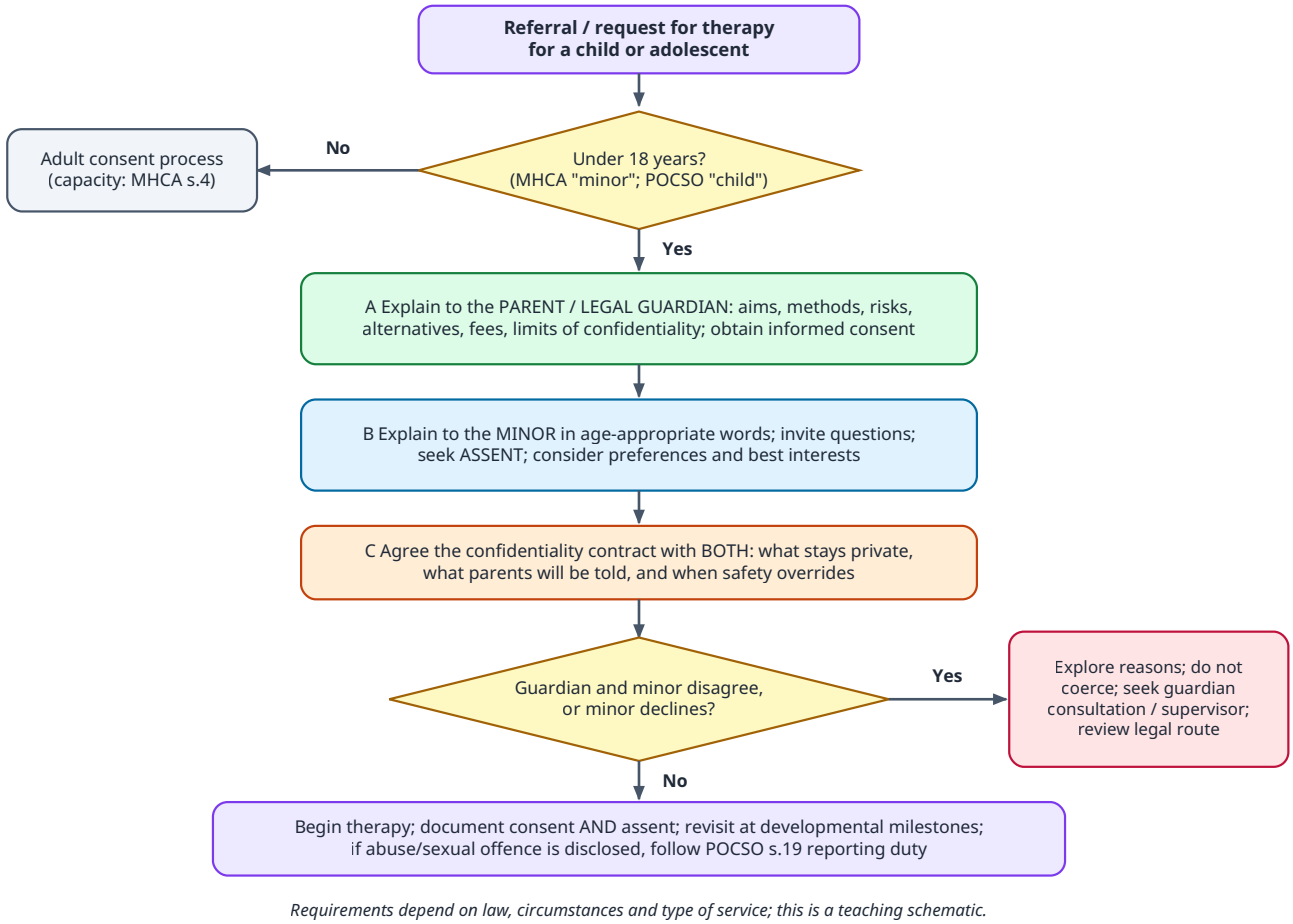



**Figure 6.1** Practical flow for consent (guardian) and assent (minor).
Source/basis: APA Ethics Code Std 3.10(b), 4.02; MHCA 2017 ss.2, 4, 15; POCSO 2012 ss.2(d), 19; Case Approach PDF p.39

## 4. Developmental capacity: how much should a minor decide?

Capacity is not all-or-nothing and grows with age and experience. It involves four abilities (reflected in MHCA s.4): **understanding** relevant information; **appreciating** consequences; **reasoning** about options; and **expressing** a choice. ICMR notes that at younger ages children are vulnerable because they cannot fully understand details, and at older ages they may understand but still lack legal capacity to consent [ICMR 2017, PDF p.76]. Hence the ethical aim is **progressive involvement**: the child participates as far as development allows.

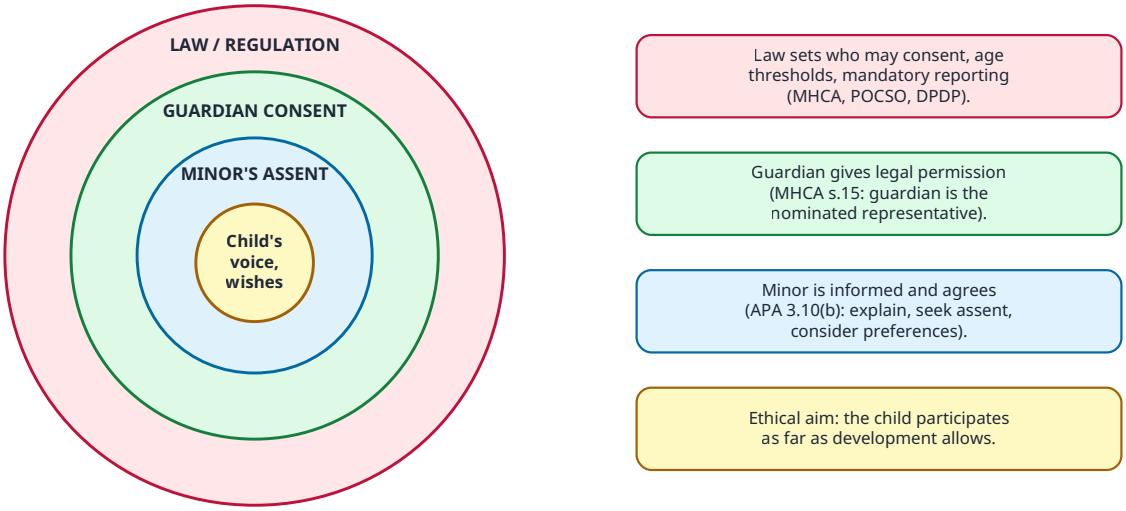



**Figure 6.2** Nested layers of permission.
Source/basis: APA Ethics Code Std 3.10(b); MHCA 2017 s.15

**Table 6.2 Age-sensitive ways of involving minors** ILLUSTRATIVE EXAMPLE **(teaching guide, not a legal threshold)**

| Developmental band | How to explain therapy | What assent looks like |
|---|---|---|
| Young child (about 4–7) | Concrete, play-based: "This is a place where we play and talk about worries. Mummy and I will keep you safe." | Willingness to enter room and take part; verbal "OK"; observe distress and withdrawal |
| Middle childhood (about 8–11) | Simple causal explanation, examples, drawings; what will stay private and what will be shared | Oral assent; may sign a child-friendly form |
| Early adolescent (about 12–14) | Respectful, reasons given; explain confidentiality limits clearly and early | Oral or written assent; invite questions and alternatives |
| Older adolescent (about 15–17) | Near-adult explanation; discuss autonomy, parental role and legal limits | Written assent; participation in goal-setting; ask whether they want a say in what parents hear |

## 5. Explaining therapy to children and adolescents in age-appropriate language

ILLUSTRATIVE EXAMPLE **SCRIPT FOR A 10-YEAR-OLD (AARAV, AFRAID OF SCHOOL)**

"Aarav, I am a psychologist. I help children who have big worries. We will meet here for about 45 minutes, once a week. We will talk, draw and play games, and I will teach you tricks to make worries smaller. Your mother wants you to come, but I also want *you* to tell me what you think. What I know from you is private, like a secret box. But if I think someone might hurt you, or you might hurt yourself, I must tell the people who can keep you safe, and I will tell you first. Is it okay to try a few sessions? You can say yes, no or 'I'm not sure'. You can ask me anything."

*Analysis:* who I am; what happens; duration; role of parent; the child's voice; confidentiality and its limits in simple terms; invitation to question; permission to say no.

**MNEMONIC — APA 3.10(B) DUTIES FOR THOSE WHO CANNOT CONSENT**

**E · A · P · P**

| | |
|---|---|
| **E** | Explain appropriately |
| **A** | Assent sought |
| **P** | Preferences and best interests considered |
| **P** | Permission from a legally authorised person |

**Sentence:** "**E**very **A**dolescent **P**ossesses **P**erspective." **Apply:** before starting with a 14-year-old, tick E, A, P, P and document each.

## 6. Confidentiality, parental involvement and safeguarding

- **Agree the confidentiality contract with both** the minor and the parent at the start: what is private; what parents will be told (e.g., attendance, progress in general terms, safety concerns); and what overrides privacy. Corey stresses that it is too late to tell a minor you will consult parents after they have disclosed a highly personal matter; young clients have a right to know the limits of confidentiality before they disclose [Corey 2024, PDF p.75].
- Privilege usually does not apply in child and adolescent therapy in the US framework Corey describes [Corey 2024, PDF p.76]; Indian courts and institutions should be consulted for local rules.
- **Parents as partners:** work with families and schools to promote and maintain change [Notes p.3]; recruit parents as collaborators [Murdock 2017, PDF p.39]. Still keep the child's content private unless safety requires disclosure.
- **Risk disclosures:** suicide risk, self-harm, abuse, neglect, trafficking, substance use, pregnancy and relationship violence may require action. Sexual offences against a child trigger the POCSO s.19

duty regardless of the child's wishes. MHCA s.23(2)(c),(e) allows release of the minimum information needed to protect from harm or prevent a threat to life.

- **Who the client is:** in child work the child is the client even when the parent pays; clarify roles (APA 3.07 third-party requests; 10.02 on therapy with several related persons, heading) and avoid secret alliances.
- **Suspected abuse by a parent:** do not confront the suspected parent; follow the reporting pathway and child-protection agencies, and ensure safety first.

## 7. When the minor and the guardian disagree

**Table 6.3 Common conflicts and sensible handling**

| Situation | Handling |
|---|---|
| Parent consents; adolescent refuses to attend | Explore reasons (fear, stigma, privacy). Do not coerce. Offer an initial meeting to explain, negotiate frequency, and clarify confidentiality. Treatment may not be effective without engagement; persisting refusal should be discussed with the guardian and supervisor. Document. |
| Adolescent wants therapy; parent refuses | Legal consent normally rests with the guardian. Explore the parent's concerns (cost, stigma, mistrust); provide psychoeducation. If there is risk or abuse, escalate through safeguarding routes. Under MHCA s.15 a Board may appoint another nominated representative where the guardian is not acting in the minor's best interests. Seek legal/supervisory advice; avoid covert treatment. |
| Parents separated and disagree | Clarify who holds legal authority; seek documents; avoid taking sides; consider consent from both where practicable. |
| Parent demands session content | Refer to the agreement; share general progress and safety-related information; explain that privacy supports the child's honesty. |
| Child reveals a secret the parent should not hear (e.g., sexual orientation, a romantic relationship) | Respect confidentiality unless safety or law requires; support the child's own decision about when to tell; avoid disclosure that could cause harm. |

## 8. Practical examples ILLUSTRATIVE EXAMPLE

**EXAMPLE A: 9-YEAR-OLD WITH SELECTIVE MUTISM**

Mother brings Ria (9) because she does not speak at school. **Consent:** mother signs the informed consent after hearing aims, methods (play-based graded exposure), duration and limits of confidentiality. **Assent:** the therapist explains to Ria with drawings, asks whether she is willing to try, and lets her choose the first activity. **Confidentiality:** mother will receive progress summaries; Ria's drawings stay private unless she agrees. **Documentation:** consent form, assent note, school-liaison permission.

**EXAMPLE B: 16-YEAR-OLD WITH SELF-HARM**

Kabir (16) discloses cutting and thoughts of dying. The consent agreement said parents would be informed if his life was at risk. After a structured risk assessment, the therapist tells Kabir she must involve his parents for safety, helps him decide how to tell them, and meets all three together. Information shared is limited to what is needed for safety (MHCA s.23(2)(e)); therapy continues with his assent.

**EXAMPLE C: 14-YEAR-OLD DISCLOSING SEXUAL ABUSE**

Neha (14) says a relative touches her. The therapist does not promise secrecy, thanks her, explains in simple words that she must tell people whose job is to keep children safe, consults the supervisor and reports to the police/SJPU as POCSO s.19 requires, supports Neha during the process, and records verbatim. Neha is told what will happen next; her assent to continuing therapy is sought afresh.

## 9. Advantages, limitations and challenges

| Benefits of seeking assent | Challenges |
|---|---|
| • Respects the child's dignity and developing autonomy.<br>• Improves engagement and adherence.<br>• Reduces coercion; builds trust.<br>• Teaches participation in health decisions. | • No single age threshold in many Indian settings; ambiguity needs institutional policy.<br>• Parents may use authority to override the child's wishes, or the reverse.<br>• Confidentiality limits can discourage disclosure.<br>• Divorce, kinship care and guardianship questions.<br>• Children's capacity fluctuates (Notes p.6).<br>• Digital data of minors adds consent duties (DPDP). |

Exam-oriented summary (Topic 6)

- Consent (guardian) + assent (minor); minor under 18 (MHCA, POCSO, DPDP).
- APA 3.10(b): EAPP; 3.10(d) document; 4.02 explain limits to those unable to consent.
- MHCA s.15: guardian is nominated representative unless Board orders otherwise; s.87 admission safeguards; s.4 capacity.
- POCSO s.19: report sexual offences against children; s.21 penalty; ICMR 7–18 assent (research only).
- Agree confidentiality limits with both parent and minor; disclose minimum necessary on risk.
- Disagreement: explore, do not coerce, consult, use Board/legal routes where necessary.
- Never state a universal rule: law, circumstances and service type matter.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (8 lines):** autonomy; capacity; children.
2. **Consent vs assent (14 lines):** Table 6.1; Murdock.
3. **Legal/professional framework (28 lines):** APA 3.10(b); MHCA ss.2, 4, 15, 87; POCSO; DPDP; ICMR.
4. **Developmental capacity + age-appropriate explanation (18 lines):** Table 6.2; script.
5. **Confidentiality, parents, safeguarding (16 lines).**
6. **Disagreements (12 lines):** Table 6.3.
7. **Examples (8 lines);** limitations and conclusion (8 lines).

## 10. Practice questions (revision practice, not predictions)

**Short 3m** Distinguish informed consent from assent.

**Short 3m** Who is the nominated representative of a minor under the Mental Healthcare Act 2017?

**Short 3m** What do the four duties in APA Standard 3.10(b) require?

**Long 5m** Discuss issues related to consent and assent when providing psychotherapy to minors.

**Compare** Guardian consent and minor assent; research assent (ICMR) versus routine clinical assent.

**Application** A 15-year-old asks you not to tell her parents about her self-harm. Her mother has consented to therapy. What do you do?

### – Model answer framework for the application question

1. Check the consent agreement: what limits were explained at the start?
2. Assess risk of self-harm and suicide; safety plan.
3. Explain to her honestly what must be shared (anything about immediate danger) and what may remain private (details of feelings, relationships).
4. Involve her in deciding how and when parents are told; offer to be present.
5. Share the minimum necessary (MHCA s.23(2)(e)); document reasoning.
6. Maintain her assent and the alliance; schedule follow-up; consult supervisor.
7. If abuse is suspected, follow POCSO/child-protection duties.

## 11. Sources used in Topic 6

APA Ethics Code 2017 (3.07, 3.10, 4.02, 10.02 heading); MHCA 2017 (ss.2, 4, 5(4), 15, 23, 87); POCSO 2012 (ss. 2(d), 19, 21); ICMR 2017 (PDF p.76–77); DPDP commentary (secondary); Murdock 2017 (PDF p.39); Corey 2024 (PDF p.75–76); Notes (p.3, 5, 6); RCI Diploma 2011 (p.9).

# 7 Planning and Recording of Therapy Sessions

Good therapy is planned and good notes are the memory of that plan. This topic covers preparing for a session, selecting interventions, closing the session, and the documentation system: clinical notes, progress notes, process notes, SOAP/DAP formats, recording of audio/video, storage, access, retention and disposal. It ends with a realistic sample note and a reusable blank template.

**AT A GLANCE**

- **Before:** review previous note and plan, check measures and risk, choose intervention, prepare practicalities (**PREP**).
- **During:** agenda, work on priority problems, jot key facts. **After:** write a timely, objective note; update formulation and plan; take it to supervision.
- **Notes:** SOAP (Subjective · Objective · Assessment · Plan) or DAP (Data · Assessment · Plan). Qualities: **CARTO** (Confidential, Accurate, Relevant, Timely, Objective).
- **Kinds of records:** clinical record (shared, accountable) · personal process notes (therapist's thinking) · research records (separate, consented).
- **Law and ethics:** APA 6.01–6.02, 4.03; MHCA ss.24–25; RCI records and register SECONDARY SOURCE ; RCI diploma psychotherapy record elements.

## 1. Introduction: why plan and why record?

APA's Record Keeping Guidelines explain that records benefit both client and psychologist through documentation of treatment plans, services provided and client progress; they allow the psychologist to monitor their own work, are especially important when there are significant gaps between contacts or when the client moves to another professional, and help protect both parties in legal or ethical proceedings GUIDELINE [APA Record Keeping Guidelines 2007]. The APA Ethics Code states the purposes: facilitate later services, meet institutional requirements, ensure accurate billing and comply with law (Std 6.01) PROFESSIONAL CODE . Beck adds a clinical reason: even experienced therapists cannot remember all the important items without written notes; notes also help refine the conceptualisation, track what is covered and plan future sessions [Beck 2011, PDF p.143].

## 2. Planning therapy: from the whole course to a single session

### 2.1 Course-level plan (treatment plan)

Jongsma and colleagues describe six steps: **(1) problem selection** (choose the most significant problems; set others aside); **(2) problem definition** (behaviourally specific, linked to diagnostic criteria); **(3) goal development** (broad, long-term); **(4) objective construction** (behaviourally measurable); **(5) intervention creation** (at least one per objective; add new ones if the objective is not met); **(6) diagnosis determination** (using DSM-5/ICD criteria). The plan must be individualised, not mass-produced [Jongsma 2014, PDF p.24–26]. Beck's chapter 19 treats planning treatment across sessions and planning individual sessions [Beck 2011, PDF p.20].

## 2.2 Session-level plan

1. **Review the previous note and plan:** agenda items left over, homework assigned, risk status.
2. **Check measures and risk:** review scores before the session; if hopelessness or suicidality items are raised, plan a risk assessment [Beck 2011, PDF p.85].
3. **Update the formulation** case formulation (a working explanation of how the client's problems developed and what maintains them) and decide the session's purpose (assessment, skill teaching, exposure, review).
4. **Select the intervention** matched to formulation, objectives and evidence (Topic 10); prepare materials (worksheets, scales).
5. **Prepare the agenda skeleton** but remain collaborative: the client's agenda items come first.
6. **Practicalities:** room privacy, time, consent for recording, interpreter, digital platform security.
7. **Plan the close:** summary, homework, feedback, next appointment.

**MNEMONIC — PRE-SESSION PREPARATION**

**P · R · E · P**

| | |
|---|---|
| **P** | Previous notes and plan |
| **R** | Risk and measures check |
| **E** | Evidence-based choice of intervention |
| **P** | Practicalities (room, consent, materials, close) |

**Apply:** five minutes before a session with a depressed client: read the last note (P), look at the BDI score and suicidality item (R), decide on behavioural activation (E), set out the activity chart and check the recording consent (P).

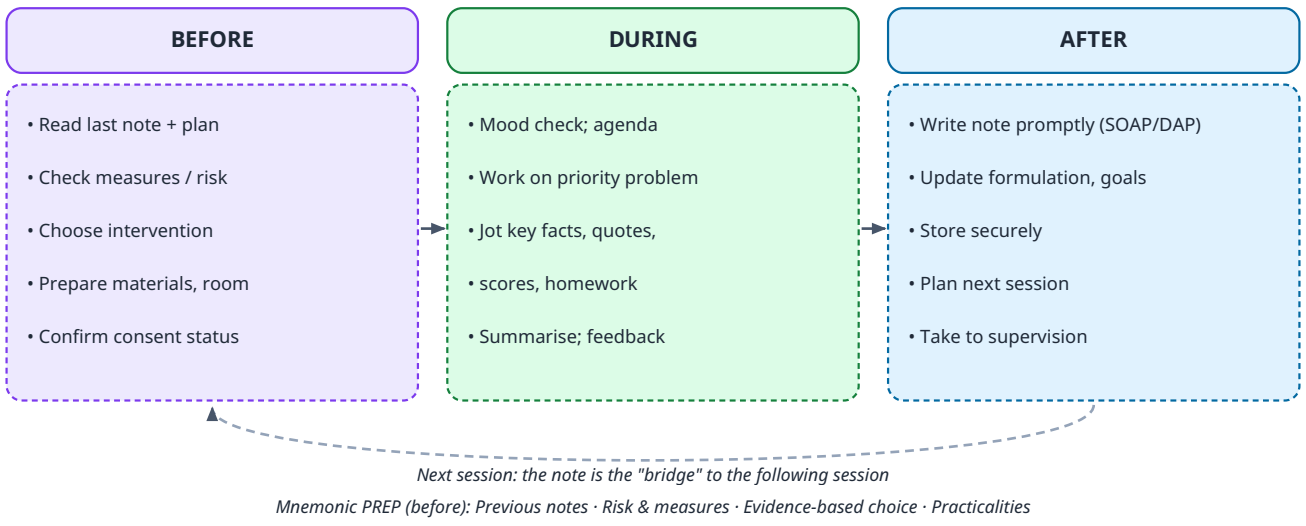



**Figure 7.1** The planning and recording cycle.
Source/basis: Beck 2011 PDF p.122-123, 143; Jongsma 2014 PDF p.24-26; APA Record Keeping Guidelines 2007

### 2.3 Closing a session

Beck's closing sequence: provide or elicit a summary; review homework; elicit feedback ("What did you think about the session? Was there anything that bothered you or that I misunderstood?"), which strengthens the alliance and often is the first time a patient has been asked [Beck 2011, PDF p.82, 122–123; p.42]. Agree the next session's date and what to do if the problem worsens in the meantime.

## 3. Types of clinical documentation

**Table 7.1 Kinds of documentation**

| Type | Purpose | Typical content | Who may access |
|---|---|---|---|
| **Intake / assessment record** | Baseline and formulation | Identifying data, presenting problems, history, mental state, risk, measures, diagnosis, consent, initial plan (APA RK: consent or assent, mandated disclosures, guardianship status, health and developmental history) | Clinical team; client per policy/law |
| **Treatment plan** | Roadmap with measurable objectives | Problems, goals, objectives, interventions, timeline (Jongsma six steps) | Client (shared); team; payors as needed |
| **Progress note** (session note) | Account of each contact | Date, duration, type of service, content, intervention, client response, measures, risk, homework, plan | Clinical record |
| **Process note** | Therapist's reflective record of process, hypotheses and reactions | Themes, transference/ countertransference, supervision points; useful for thinking and supervision [Notes p.7] | Kept separate; but legal access is not impossible |
| **Therapy notes taken for the client** (CBT) | Learning aids | Client takes home key ideas each session (Beck PDF p.31) | Client |
| **Summary / discharge / referral letter** | Continuity of care | Course, outcome, recommendations | Recipient with consent |
| **Psychotherapy record for training** (RCI) | Competence evidence | See below | Examiners; supervisors |
| **Correspondence, billing, consent, release forms** | Administration | Fees, appointments, releases | Per policy |

**RCI'S EXPECTATION OF A WORKED-OUT PSYCHOTHERAPY RECORD (DIPLOMA DOCUMENT, 2011)** GUIDELINE

A summary of the clinical history organised under relevant headings, and discussion of: **a)** reasons for intervention(s); **b)** areas to be focused, including short- and long-term objectives; **c)** approach and technique(s) employed and rationale; **d)** intervention processes; **e)** changes in therapy or objectives, if any, and the reasons; **f)** outcome; **g)** integration strategies employed; and a statement of **future plans** [RCI Diploma 2011, p.9]. The trainee also keeps a logbook with client particulars, diagnosis, duration and nature of interventions and number of sessions [RCI Diploma 2011, p.9]. Use this as a ready outline for case records in M.Phil. work, and check the current M.Phil. guidance.

What should a record contain? APA's guidelines list: identifying data; contact information; fees and billing; guardianship status where appropriate; documentation of informed consent or assent; waivers and releases; mandated disclosures (e.g., a child-abuse report or court order); presenting complaint and diagnosis; plan for services; health and developmental history; and for each substantive contact the date and duration, type of service, nature of intervention, and formal or informal assessment of client status; plus optional items such as client responses, current risk factors, emergency interventions, plans, consultations and relevant cultural and sociopolitical factors [APA Record Keeping Guidelines 2007].

## 4. Formats: SOAP and DAP

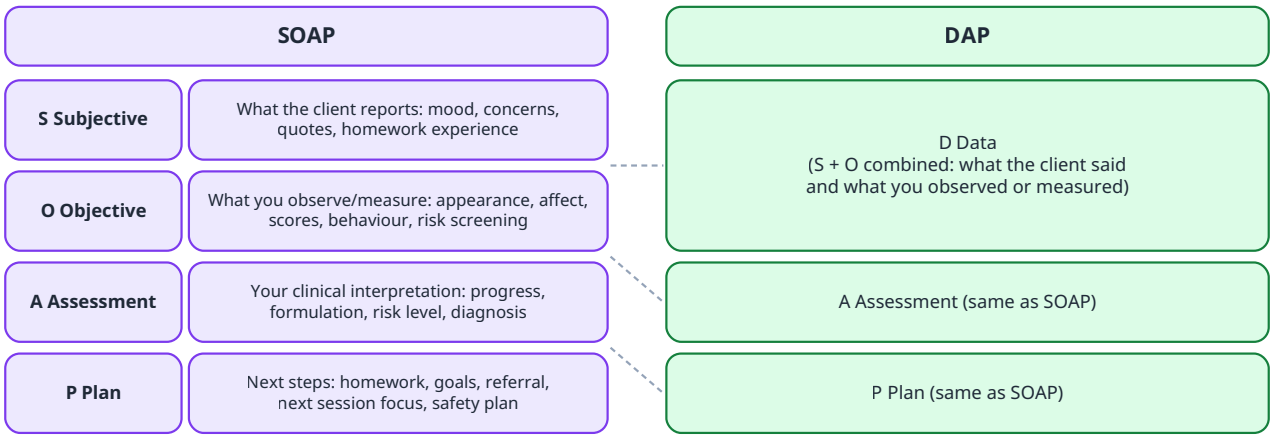



**Figure 7.2** SOAP (four parts) versus DAP (three parts).
Source/basis: Standard clinical documentation formats; APA Record Keeping Guidelines 2007 (content of records)

**Table 7.2 SOAP versus DAP**

| | SOAP | DAP |
|---|---|---|
| Parts | Subjective · Objective · Assessment · Plan | Data · Assessment · Plan |
| Best when | You want to separate client report from observation (medical/hospital settings) | You want a compact note that combines both |

| | | |
|---|---|---|
| Risk | Over-fragmentation | Blending report and observation |
| Both require | Objective language; risk status; measures; next steps; date, duration and signature | |

**MNEMONIC — QUALITIES OF A GOOD NOTE**

## C · A · R · T · O

| | |
|---|---|
| **C** | Confidential (stored and shared securely) |
| **A** | Accurate (facts, quotes, dates) |
| **R** | Relevant (only what serves care; APA 4.04: only information germane to the purpose) |
| **T** | Timely (written soon after the session) |
| **O** | Objective (describe, do not label or judge) |

**Apply:** write “client wept and said ‘I can't go on’” (objective, accurate) rather than “client was manipulative” (judgment).

### 4.1 Writing principles

- **Objectivity:** separate observation from inference; use quotations; avoid pejorative terms.
- **Relevance:** record what is needed for care; keep third-party details minimal.
- **Accuracy and time-stamping:** date, time, duration; never alter earlier entries silently; correct by dated addendum.
- **Risk:** record risk assessments, safety plans, mandated disclosures with reasons and the minimum information shared.
- **Language:** note interpreters used and the language of consent.

## 5. A realistic sample session note ILLUSTRATIVE EXAMPLE

**ILLUSTRATIVE SOAP NOTE (FICTITIOUS CLIENT)**

| Field | Entry |
|---|---|
| Client ID / Name | CP-0231 · "R.S." (age 29) |
| Date / time / duration | 18 Oct 2026 · 11:00–11:50 · Session 7 of planned 12 |
| Modality / setting | Individual CBT, outpatient clinic; therapist: trainee M.Phil. under Dr. X (supervisor) |
| Consent / recording | Written consent on file (12 Sep); oral permission to audio-record today; assent N/A (adult) |
| Measures | PHQ-9 = 14 (previous 17; baseline 21); GAD-7 = 11 (previous 13) |
| **S**ubjective | R.S. reports lower mood on 4/7 days, "I still feel I will fail the exam"; sleep 5–6 h; completed 3/5 activity-schedule tasks; avoided asking a colleague for help. Denies suicidal thoughts; says "I don't want to die, I'm just tired." |
| **O**bjective | Appropriately dressed; good eye contact; affect constricted but reactive; speech normal rate; no psychotic features observed. C-SSRS screen: negative for ideation/behaviour today. |
| **A**ssessment | Partial response (PHQ-9 down 7 points from baseline, still above target). Maintaining factors: perfectionistic prediction of failure, avoidance of help-seeking. Risk: low at present. Alliance: collaborative. |
| **P**lan | Next session: behavioural experiment on asking for help; thought record on "I will fail". Homework agreed: two thought records, one experiment. Review PHQ-9 weekly; reassess at session 8 if PHQ-9 > 12. Discuss plan with supervisor Thursday. |
| Signature / date | Initials and date; supervisor countersignature |

### 5.1 Blank reusable template (copy and use)

– **Session note template (SOAP) with plan section**

| Field | Entry |
|---|---|
| Client ID / initials / age | |
| Date · start–end time · session number | |

| |
|---|
| Therapist · supervisor · setting · modality |
| Consent / assent status; recording permission |
| Measures today (name · score · change from baseline) |
| Agenda items (client first) |
| **S** Subjective: client report, quotes, homework review |
| **O** Objective: appearance, behaviour, affect, mental state, risk screen |
| Intervention(s) used and client response |
| **A** Assessment: progress vs objectives, formulation update, risk level, alliance |
| **P** Plan: homework, next-session focus, referral/consultation, safety plan, next date |
| Disclosures or consultations (what, to whom, why, minimum shared) |
| Signature · date · supervisor countersignature |

## 6. Process notes, clinical records and research records

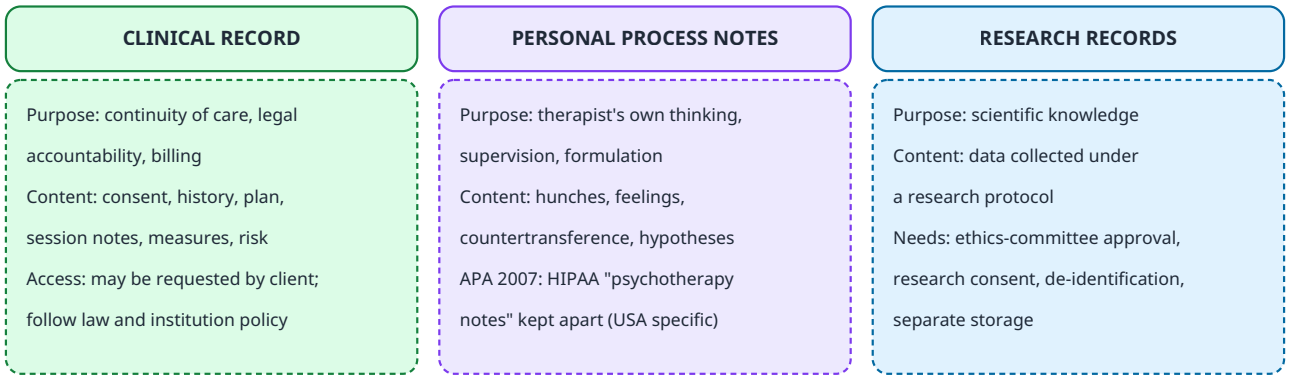



**Figure 7.3** Routine clinical records versus process notes versus research records.
Source/basis: Notes PDF p.7-8; APA Record Keeping Guidelines 2007; APA Ethics Code Std 6.01-6.02, 8.02-8.03

- **Notes p.7:** the author keeps a brief note in the medical record that the session took place and some brief notes of the main themes in a separate file in the office; the American Psychiatric Association recommends this practice and it is consistent with Royal College of Psychiatrists advice. Legal jurisdictions in the UK and US make it theoretically possible for patients to access those notes unless this would cause harm; therapists cannot assume they own them. Keeping a record also helps the therapist's thinking and supervision [Notes p.7].

- GUIDELINE APA Record Keeping Guidelines say psychotherapy notes as defined by HIPAA (a US privacy law) are necessarily kept apart from the rest of the record; client files may be divided into sections; material generated by clients or third parties may be in a separate section [APA Record Keeping Guidelines 2007]. HIPAA is US law and does not apply in India.
- **Research records** are collected for a study under a protocol; they need ethics-committee approval and research consent (APA 8.02–8.03; ICMR 2017) and should be stored separately from clinical files; do not use clinical notes as research data without permission.
- **Publication/teaching:** disguise identity and obtain consent (APA 4.07); some journals require that the patient has read the article [Notes p.7].
- **Records in training:** case write-ups for supervisors and examiners must be de-identified.

## 7. Recording audio or video of sessions

1. **Permission first.** PROFESSIONAL CODE APA 4.03: obtain permission from all persons (or legal representatives) before recording voices or images. For minors, obtain guardian consent and the child's assent.
2. **Explain purposes** (supervision, training, accuracy, research), who will see or hear it, storage and deletion; the client may refuse or stop at any time without penalty to care.
3. **Secure storage:** encrypted devices, restricted access, no personal phones/cloud without safeguards; de-identified file names.
4. **Limit use** to the stated purpose; destroy after supervision or per policy.
5. **Benefits:** supervision, adherence feedback (e.g., rating on the Cognitive Therapy Rating Scale; Beck PDF p.369 suggests listening to sessions and rating them), and accuracy. Wolberg notes videotape helps supervision [Wolberg 2013, PDF p.106]. **Risks:** loss of privacy, inhibition, data breach.

## 8. Storage, access, retention and disposal

- **Storage and security:** APA Record Keeping Guideline 6 (Security) asks psychologists to protect paper and electronic records from unauthorised access, damage and destruction; keep records in locked cabinets and encrypted systems.
- **Access:** clients can access basic records under MHCA s.25 BINDING LAW , with limited grounds for withholding; handle requests as in Topic 5.
- **Retention:** GUIDELINE APA Record Keeping Guidelines: in the absence of a superseding requirement, psychologists may consider retaining full records until **7 years after the last service for adults** or **3 years after a minor reaches majority**, whichever is later; longer retention may be considered. No Indian statutory period for psychologists was identified in the sources I read; follow your institution's policy and any hospital or court rule, and verify. SECONDARY SOURCE RCI Reg. 13 requires a register of patients, prescriptions and fees (Nyaaya summary).
- **Disposal:** APA Guideline 13 and Std 6.02: plan for transfer of records if you leave practice, and dispose of them by methods that preserve confidentiality and prevent recovery (shredding, secure erasure).
- **Digital:** MHCA s.24(2) applies confidentiality to electronic and digital information; DPDP duties may apply to personal data processing SECONDARY SOURCE .

## 9. Advantages, limitations and challenges of documentation

| Advantages | Limitations / challenges |
|---|---|
| • Continuity, safety and quality of care.<br>• Evidence of reasoning and consent in disputes.<br>• Supports supervision and learning.<br>• Enables outcome monitoring (Topic 9). | • Time burden; templates may fragment the story.<br>• Notes can be read by the client or a court; wording matters.<br>• Defensive documentation can displace clinical thinking.<br>• Security risks of electronic storage.<br>• Unclear Indian retention rules. |

Exam-oriented summary (Topic 7)

- Plan: treatment plan (Jongsma six steps) and session plan (PREP); close with summary, homework, feedback.
- Record types: intake, plan, progress, process, discharge; clinical vs process vs research.
- SOAP vs DAP; CARTO qualities; objective language; risk documented.
- Recording: permission first (APA 4.03); secure; limited purpose.
- Retention (APA guideline): 7 years adults; 3 years after majority for minors, unless superseded; disposal preserves confidentiality.
- India: MHCA s.24–25; RCI register SECONDARY SOURCE ; RCI psychotherapy record outline.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (6 lines). Planning (24 lines):** six-step treatment plan; PREP; closing.
2. **Types of records (18 lines):** Table 7.1; RCI outline.
3. **SOAP/DAP + CARTO (16 lines);** sample note (10 lines).
4. **Process/clinical/research records (12 lines).**
5. **Recording, storage, access, retention, disposal (18 lines).**
6. **Ethics/law and limitations (8 lines);** conclusion (4 lines).

## 10. Practice questions (revision practice, not predictions)

**Short 3m** Write a SOAP note outline and explain each letter.

**Short 3m** Differentiate progress notes from process notes.

**Short 3m** What must you do before audio-recording a session?

**Long 5m** Describe the planning and recording of therapy sessions, including documentation formats, confidentiality, retention and disposal.

**Compare** SOAP versus DAP; clinical record versus personal process notes.

**Application** A supervisor asks to listen to your session recordings, but you only have notes. How will you obtain consent and handle storage?

**– Model answer framework for the application question**

1. Explain to the client the purpose (supervision), who will listen, storage, destruction; obtain written permission (APA 4.03); for a minor, guardian consent and child assent.
2. Use an encrypted device; de-identified file names; restricted access.
3. Allow refusal without penalty; note it in the record.
4. Use recordings only for the stated purpose; delete after supervision per policy; document.

## 11. Sources used in Topic 7

APA Ethics Code 2017 (4.03, 4.04, 4.07, 6.01–6.03, 8.02–8.03); APA Record Keeping Guidelines 2007 (guidelines on content, confidentiality, security, retention, disposition); Beck 2011 (PDF p.20, 31, 42, 82, 85, 122–123, 143, 369); Jongsma 2014 (PDF p.24–26); Notes (p.7–8); RCI Diploma 2011 (p.9); MHCA 2017 (ss.24–25); Wolberg 2013 (PDF p.106); Nyaaya 2022 (secondary); ICMR 2017.

# 8 Structuring Therapy Sessions and Setting Goals

Structure gives therapy direction and safety; goals give it purpose and a way of knowing when it has worked. This topic covers the whole course (intake » assessment » formulation » plan » intervention » review » termination), the structure of a single session, short- and long-term goals, SMART and other frameworks, collaborative goal setting, monitoring and revising goals, and how structure varies across approaches and contexts.

**AT A GLANCE**

- **Process phases:** Intake · Assessment · Formulation · Plan · Intervention · Review · Termination (**IAF-PIRT**).
- **Session (CBT):** beginning (mood check, agenda, update, homework review, prioritise) · middle (work on problem, homework) · end (summary, new homework, feedback). Mnemonic: *My Aunt Usually Helps Prepare Wonderful Sweet Hot Food*.
- **Goals:** long-term goals are broad; objectives are measurable (Jongsma). **SMART** = Specific · Measurable · Achievable · Relevant · Time-bound.
- **Principle:** structure is explained to the patient, because it "demystifies" therapy and keeps it on track (Beck).

## 1. Introduction: why structure?

Beck writes that most patients feel comfortable when the therapist explains how and why sessions are structured; doing so demystifies the process and keeps treatment on track [Beck 2011, PDF p.81]. Structure ensures that the most important problems are addressed, that time is used well, that homework links sessions, and that the patient is an active partner. It is not rigid: a good therapist flexes the structure to the client's needs (for example, a crisis takes priority over the agenda).

## 2. The therapy process from intake to termination

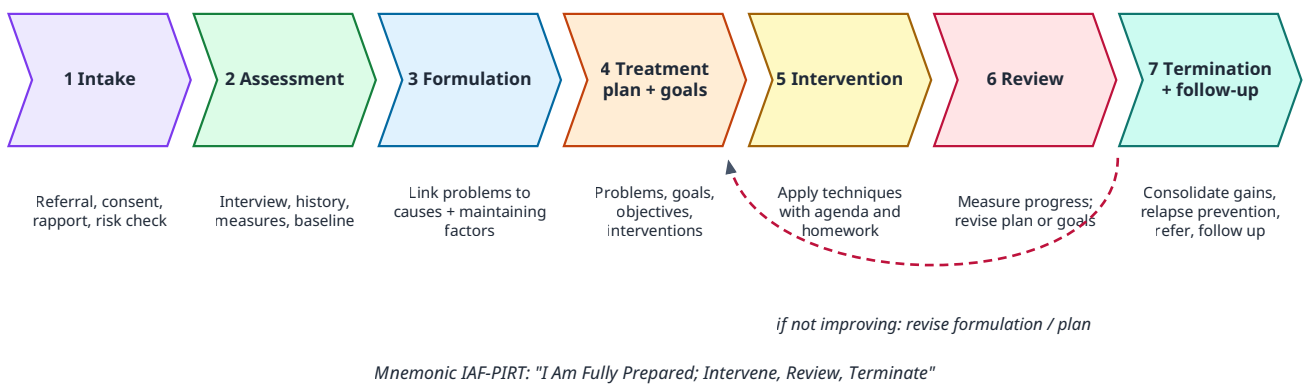



**Figure 8.1** The therapy process from intake to follow-up.
Source/basis: Jongsma 2014 PDF p.24-26; Beck 2011 PDF p.70-71; RCI Diploma doc 2011 (record elements)

**Table 8.1 Phases of therapy: tasks and outputs**

| Phase | Main tasks | Output |
|---|---|---|
| **1 Intake** | Referral details; explain therapy; informed consent/ assent; rapport; initial risk screen; practicalities | Consent, basic information, immediate safety plan if needed |
| **2 Assessment** | Clinical interview, history, mental state, measures, collateral information; set baseline (Topic 9) | Baseline scores, problem list, diagnosis hypothesis |
| **3 Formulation** | Integrate how problems developed (predisposing), what triggers (precipitating) and maintains them (perpetuating), and strengths (protective) | Working formulation shared with the client |
| **4 Treatment plan** | Select problems; define goals and measurable objectives; choose interventions; agree frequency and duration (Jongsma steps) | Written, agreed plan |
| **5 Intervention** | Deliver techniques with structured sessions, agenda and homework; maintain alliance | Session notes, skills learned |
| **6 Review** | Monitor measures; compare with objectives; revise formulation and plan; address non-response (Topic 9) | Updated plan or change of approach |
| **7 Termination and follow-up** | Consolidate gains; relapse-prevention plan; space sessions; booster sessions; referral; follow-up assessment | Discharge summary; follow-up date |

MNEMONIC — THERAPY PHASES

**I A F – P I R T**

**I**ntake · **A**ssessment · **F**ormulation · **P**lan · **I**ntervention · **R**eview · **T**ermination. Sentence: "**I A**m **F**ully **P**repared; **I**ntervene, **R**eview, **T**erminate."

### 2.1 The three broad stages: beginnings, middles and ends

Many writers divide the process into three phases (Mearns & Thorne's 'beginnings, middles, ends'; Egan's three stages: identify and clarify problems, develop programmes for constructive change, implement goals) [Notes p.77]. The **beginning** phase includes negotiating expectations, assessing suitability, forming the alliance, agreeing a contract and helping the client tell their story. The **middle** phase is where most learning and change occurs (assimilation of problematic experiences is one model of change, Notes p.78). The **final** phase involves negotiating the ending, referral, dealing with loss, ensuring transfer of learning to real life and anticipating and preventing relapse [Notes p.77–78].

### 2.2 Process units

Elliott suggests viewing therapy at different time scales: the speaking turn (micro-process of a minute or two); the episode (a task or topic over several minutes); the session; and the whole treatment relationship [Notes p.77]. Structuring operates at each level.

## 3. Structuring an individual session

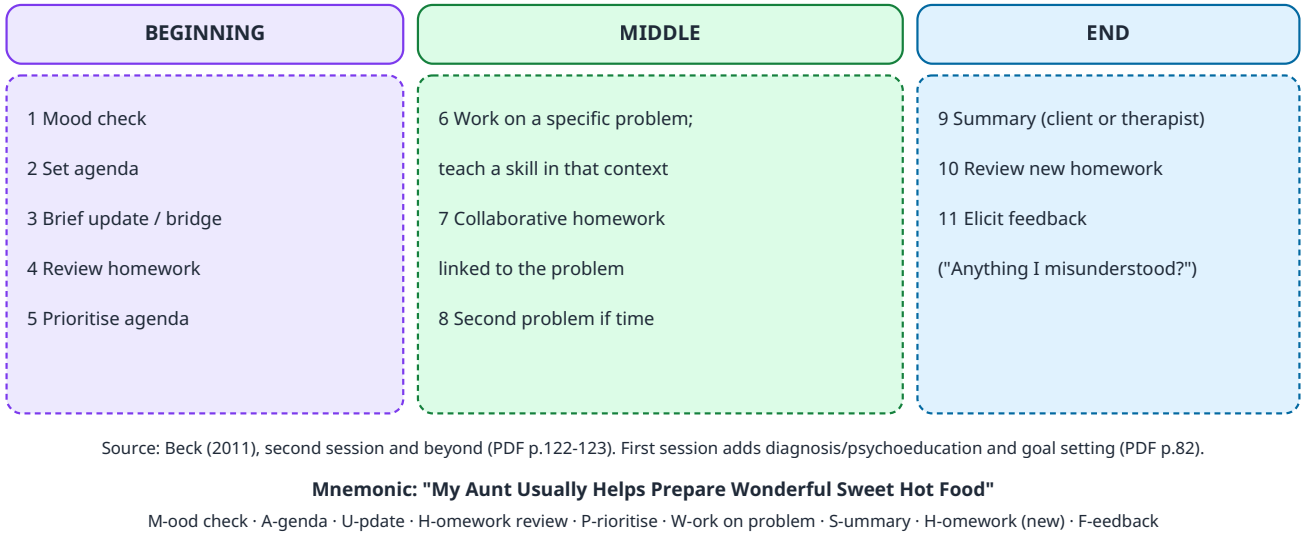



**Figure 8.2** Session skeleton (CBT). Other approaches vary the sequence but keep the logic.
Source/basis: Beck 2011 PDF p.82, 122-123

### 3.1 First session (CBT example)

Beck's format for session 1 [Beck 2011, PDF p.82]: **Initial part:** 1 set the agenda (with a rationale); 2 do a mood check; 3 obtain an update since the evaluation; 4 discuss the diagnosis and do

psychoeducation. **Middle:** 5 identify problems and set goals; 6 educate the patient about the cognitive model; 7 discuss a problem. **End:** 8 provide or elicit a summary; 9 review homework; 10 elicit feedback.

### 3.2 Second session and beyond (CBT)

Beck's typical agenda [Beck 2011, PDF p.122–123]: **Initial part:** 1 mood check; 2 set the agenda; 3 obtain an update (bridge from previous session); 4 review homework; 5 prioritise the agenda. **Middle:** 6 work on a specific problem and teach skills in that context; 7 follow-up discussion with relevant, collaboratively set homework; 8 work on a second problem. **End:** 9 provide or elicit a summary; 10 review new homework; 11 elicit feedback. Because experienced and novice therapists differ, a novice can usually discuss in depth about two problems from the agenda in a session [Beck 2011, PDF p.143].

**MNEMONIC — ONGOING CBT SESSION**

**M A U H P W S H F**

| | |
|---|---|
| **M** | Mood check |
| **A** | Agenda set (with client) |
| **U** | Update / bridge from last session |
| **H** | Homework review |
| **P** | Prioritise agenda items |
| **W** | Work on the problem; teach skills |
| **S** | Summary |
| **H** | (new) Homework |
| **F** | Feedback |

**Sentence:** "**M**y **A**unt **U**sually **H**elps **P**repare **W**onderful **S**weet **H**ot **F**ood." **Apply:** run through the nine words when planning any CBT session.

### 3.3 Establishing rapport, reviewing progress and applying interventions

- **Rapport and alliance:** warmth, empathy, collaboration. The alliance is present when the patient feels helped and feels the therapist is collaborating towards common goals [Gabbard 2004, PDF p.58].
- **Mood check and update:** a brief review (symptom scores, week's events) so that the session starts with the client's current state; teach the client to be concise [Beck 2011, PDF p.125].
- **Agenda:** explain the rationale; ask for the client's items; if the client wants another use of time, negotiate [Beck 2011, PDF p.83–84].
- **Apply interventions** inside the agenda (e.g., thought record, activity scheduling, exposure planning).

- **Summarise** (capsule summaries during the session; final summary) and **assign homework** collaboratively; **elicit feedback**, which strengthens the alliance [Beck 2011, PDF p.42].

ILLUSTRATIVE EXAMPLE **SETTING AN AGENDA (CBT DIALOGUE)**

**Therapist:** "Before we begin, can we decide what to talk about today? What are the most important things for you?"
**Client:** "My exam stress, and I had a fight with my roommate."
**Therapist:** "I'd also like 10 minutes to look at the homework. We'll start with the exam stress since that's affecting your sleep, then, if time allows, the roommate. Does that sound right?"

## 4. How structure varies across approaches and contexts

**Table 8.2 Variation in structure**

| Approach / context | Typical structure | Source |
|---|---|---|
| **CBT** | Highly structured: agenda, mood check, homework, summary, feedback | Beck 2011, PDF p.82, 122–123 |
| **Psychodynamic** | Structure is the *frame* of therapy: regular time, fee, boundaries, free association and the therapist's neutrality; supervision watches deviations from the frame | Gabbard 2004, PDF p. 193 |
| **Person-centred / humanistic** | Client-led; structure is the conditions of the relationship, not an agenda | Corsini 2011, PDF p.166 (non-directive attitude) |
| **DBT** | Levels of disorder and stages of treatment; weekly individual therapy plus skills group; targets in order of priority (life-threatening first) | Notes p.52; DBT Made Simple PDF p.72 |
| **MET** | Four planned sessions: first two give structured feedback from assessment and discuss plans and motivation; last two (midpoint and end) reinforce progress and reassess | MET Manual, PDF p.11 |
| **Brief therapy** | Focus on a specific pattern; active therapist and client; homework; client selection; time-limit from the outset | Notes p.24 |
| **Crisis intervention** | Immediate safety, assessment, practical support; limited aims | RCI Diploma 2011 Paper II Unit V (crisis) |
| **Group / family** | Sessions have roles, rules and process phases; assessment of the group or family system | Notes p.55–58 |
| **Children** | Play-based, shorter, with family/school involvement | Notes p.3–4, 101–105 |
| **Tele-therapy** | Same structure plus technology check, privacy check, emergency plan | APA Telepsychology 2013 |

Wolberg's Q&A emphasises that time-limited (brief) work requires a focus agreed early, and that homework sessions and spacing of visits (weekly to fortnightly, then occasional boosters) can help the end of therapy [Wolberg 506 Q&A, PDF p.104].

## 5. Setting goals

### 5.1 Short-term and long-term goals

| | Long-term goal | Short-term objective |
|---|---|---|
| Nature | Broad desired outcome; need not be measurable (Jongsma) | Behaviourally measurable step; time-bound |
| Timescale | End of therapy or beyond | Weeks (one to a few sessions) |
| Example | “Return to work with manageable anxiety.” | “Attend a 30-minute meeting twice this week with anxiety <= 5/10.” |
| Review | At review points (e.g., every 4–6 sessions) | Every session via homework |

RCI's practical examination expects trainees to be able to conceptualise problems and set **short- and long-term goals** and carry out indicated interventions [RCI Diploma 2011, p.8].

### 5.2 Collaborative goal setting in practice

1. **Elicit problems** in the client's words and list them (Beck's problem identification: break big concerns into manageable problems) [Beck 2011, PDF p.90].
2. **Turn problems into goals** framed positively (what the client wants more of) and in behavioural terms.
3. **Prioritise:** risk and safety first; then the problem most distressing or most likely to yield early success; then the others (Jongsma: an effective plan deals with only a few selected problems or loses direction).
4. **Check feasibility and consent:** do the client and relevant others agree? Are resources and time adequate?
5. **Write down** goals and objectives; give the client a copy.
6. **Monitor progress** with measures and homework; **revise** goals when achieved, unrealistic or no longer relevant, and record the reason for the change (RCI record outline asks for changes in objectives and reasons).

## 5.3 SMART goals and other frameworks

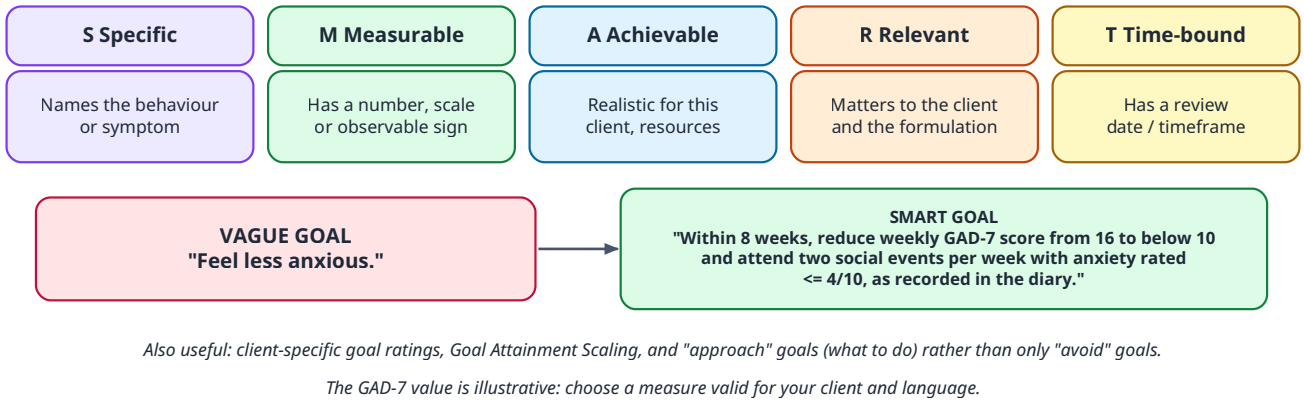



**Figure 8.3** SMART criteria and an illustrative conversion.
Source/basis: Jongsma 2014 PDF p.24-26 (measurable objectives); SMART is a widely used goal-setting framework

**Table 8.3 Converting vague goals into measurable clinical goals** ILLUSTRATIVE EXAMPLE

| Vague goal | Measurable (SMART) version |
|---|---|
| “Feel better.” | “Within 8 weeks, reduce PHQ-9 from 20 to below 10 and complete at least four pleasant activities per week.” |
| “Be less anxious in public.” | “By week 10, travel by bus alone on three days a week for four consecutive weeks, with peak anxiety rated <= 4/10.” |
| “Stop OCD.” | “Reduce daily washing ritual time from 3 hours to under 45 minutes within 12 weeks, as recorded in a daily log; complete 5 exposure tasks weekly.” |
| “Get along with my husband.” | “Hold a 20-minute calm conversation about household tasks twice weekly and reduce reported shouting episodes from 5 to 1 per week within 8 weeks.” |
| “Handle anger.” | “Use a time-out plan in at least 4 of 5 anger episodes in the next month; reduce anger intensity ratings by 3 points.” |
| “Become confident.” | “Speak once per class in the next 4 weeks and give one 5-minute presentation by week 8, rating confidence at least 6/10.” |

- **SMART** is a general goal-setting framework (not specific to psychotherapy): Specific, Measurable, Achievable, Relevant, Time-bound.
- **Other tools:** goal attainment scaling (rate goals from much worse to much better than expected), individualised target-complaint ratings, approach vs avoidance goals, and client-specific measures. The choice depends on the client and service.
- **Pitfalls:** too many goals; goals set by others; perfectionistic targets; goals that ignore culture or resources; goals that never get revised.

MNEMONIC — SMART GOALS

## S · M · A · R · T

**S**pecific · **M**easurable · **A**chievable · **R**elevant · **T**ime-bound. **Apply** by testing a goal: Can I count it? When will we check? Does the client want it? Check each letter before writing it into the plan.

## 6. Advantages, limitations and challenges

| Advantages | Limitations / challenges |
|---|---|
| • Efficient use of time; clear direction.<br>• Alliance and engagement improve when structure is explained.<br>• Goals enable outcome monitoring.<br>• Supports supervision and audit. | • Over-rigid structure can silence emerging material (Beck advises dealing with patients who resist the agenda).<br>• Measurable goals may miss meaning-based change.<br>• Clients in crisis cannot follow an agenda.<br>• Cultural differences in directness and decision-making.<br>• Difficulty interrupting or socialising patients to structure (Beck ch.8). |

Exam-oriented summary (Topic 8)

- Phases: IAF-PIRT; beginnings, middles, ends.
- Beck session 1: 10 steps; later sessions: 11 steps; MAUHPWSHF.
- Goals vs objectives; short vs long term; collaborative, prioritised, monitored, revised.
- SMART; vague » measurable examples.
- Structure varies: CBT (agenda), psychodynamic (frame), humanistic (client-led), DBT (stages), MET (4 sessions), brief therapy (focus).
- Explain structure to the client; flex for crisis.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (6 lines). Phases (22 lines):** Table 8.1; beginnings/middles/ends.
2. **Session structure (26 lines):** Beck formats; MAUHPWSHF; rapport; example.
3. **Variation across approaches/contexts (14 lines):** Table 8.2.
4. **Goals (30 lines):** long vs short; collaborative steps; SMART with table of examples.
5. **Monitoring/revising goals (6 lines);** limitations (4 lines); conclusion (4 lines).

## 7. Practice questions (revision practice, not predictions)

**Short 3m** List the steps of a typical ongoing CBT session.

**Short 3m** What are SMART goals? Convert “feel less anxious” into a SMART goal.

**Short 3m** Differentiate long-term goals from objectives.

**Long 5m** Describe how a therapy session is structured and how goals are set and reviewed in psychotherapy.

**Compare** Structure in CBT versus psychodynamic therapy.

**Application** A client repeatedly derails the agenda with new crises. What do you do?

**– Model answer framework for the application question**

1. Assess whether a real crisis or avoidance of the agenda; always check risk first.
2. Explain the rationale of the agenda again and ask for the client's reaction.
3. Collaboratively set a short crisis slot ("what's on your mind this week?") then move to the agenda; allow flexibility.
4. Use problem-solving to turn crises into agenda items; link them to the formulation.
5. Record the pattern and discuss with supervisor.

## 8. Sources used in Topic 8

Beck 2011 (PDF p.42, 81–84, 90, 122–123, 125, 143); Jongsma 2014 (PDF p.24–26); Notes (p.24, 52, 55–58, 77–78, 101–105); Gabbard 2004 (PDF p.58, 193); MET Manual (PDF p.11); DBT Made Simple (PDF p.72); Corsini 2011 (PDF p.166); Wolberg 506 Q&A (PDF p.104); RCI Diploma 2011 (p.8); APA Telepsychology 2013.

# 9 Pre- and Post-Assessment

You cannot say therapy worked unless you know where the client started, how they changed during treatment, where they ended up, and whether the change lasted. This topic covers baseline assessment, ongoing monitoring, post-treatment and follow-up assessment; methods; psychometric properties; selecting measures; clinical significance; responding to non-response and deterioration; and why one test score is not proof of effectiveness.

**AT A GLANCE**

- **Four time points:** Baseline · Monitoring (during) · Post · Follow-up (**B-M-P-F**).
- **Methods:** clinical interview, behavioural observation, standardised measures, functional assessment, client-reported outcomes, collateral information.
- **Good measure:** Valid, Reliable, Sensitive to change, Practical, Appropriate to language/ population, with Norms (**SPAN-VR**).
- **Meaning of change:** statistical (reliable change), *clinical significance* (moved to the functional range), and meaningful to the client.
- **Caution:** improvement on one test does not prove treatment worked.

## 1. Introduction

Assessment is part of every phase of therapy, not just the start. Beck's practice illustrates this: patients complete symptom measures at the evaluation, and the therapist reviews them for hopelessness and suicidality items before deciding on the session's focus [Beck 2011, PDF p.85]; scores are compared each week ([Beck 2011, PDF p.125]), recorded on the session note ([Beck 2011, PDF p.143]), and reviewed at follow-up and booster sessions ([Beck 2011, PDF p.350]). A lack of progress or deterioration, identified from self-report or objective tests, is a signal to change the plan [Beck 2011, PDF p.370].

RCI treats assessment as a core competency: its diploma document lists “competency in selecting, administering, scoring and interpreting psychological tests” as Practical I [RCI Diploma 2011, p.8]. APA's Ethics Code has a whole section on assessment (9.01 bases for assessments; 9.02 use of assessments; 9.03 informed consent in assessments) PROFESSIONAL CODE .

## 2. The assessment cycle: baseline, monitoring, post and follow-up

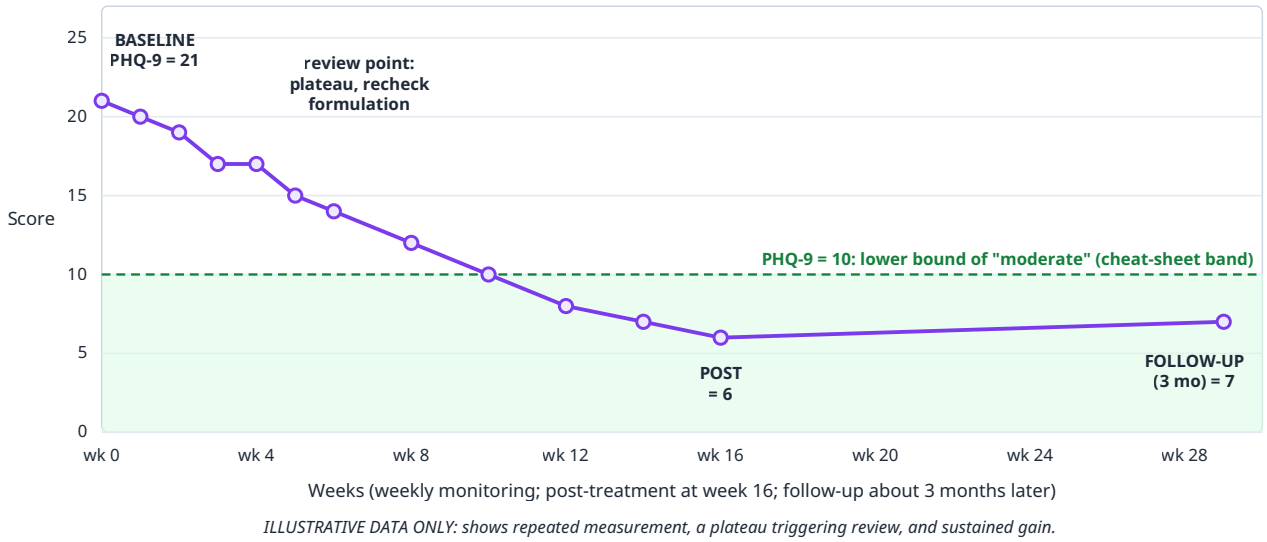



**Figure 9.1** Baseline, monitoring, post and follow-up on one measure (hypothetical client; PHQ-9 bands from the repository cheat sheet).

Source/basis: Logic from Beck 2011 PDF p.125, 143, 350, 370; Lister-Ford 2007 PDF p.118; PHQ-9 bands: Rating Scales Cheat Sheet PDF p.78

**Table 9.1 The four time points**

| Time point | Purpose | What to do |
|---|---|---|
| **B**aseline (pre-treatment) | Describe the problem and set reference values; support diagnosis, formulation and goals | Interview, history, measures of symptoms and functioning, risk screen, client-set goals; record scores before treatment begins |
| **M**onitoring (during) | Track response; adjust treatment; detect deterioration early | Brief weekly measures (e.g., PHQ-9, GAD-7), session-by-session mood check, goal ratings, alliance feedback, risk review |
| **P**ost-treatment | Evaluate outcome against baseline and objectives | Repeat the baseline measures; interview; client and significant others' views; discharge summary |
| **F**ollow-up | Test durability; identify relapse; plan boosters | Re-measure at 1, 3, 6, 12 months as feasible; ask about relapse signs and use of skills |

MNEMONIC — ASSESSMENT TIME POINTS

**B · M · P · F**

**B**aseline · **M**onitor · **P**ost · **F**ollow-up. Sentence: "**B**etter **M**easure **P**rogress **F**ully." **Apply:** when you write a case summary, add one line for each of the four letters.

## 3. Methods of assessment

**Table 9.2 Methods, strengths and cautions**

| Method | Examples | Strengths | Cautions |
|---|---|---|---|
| **Clinical interview** | Intake interview, psychiatric history, mental state, collateral interview (RCI Paper II Unit II: interviewing; Notes Unit 3) | Rich, flexible; builds alliance | Interviewer bias; variable structure; use structured formats where possible |
| **Behavioural observation** | Direct observation, frequency or duration counts, behaviour logs (Notes p.49) | Objective, context-specific | Reactivity; time-consuming |
| **Standardised self-report scales** | BDI-II, PHQ-9, GAD-7, DASS-21, BAI, Y-BOCS (clinician-rated), C-SSRS (risk) (see Rating Scales Cheat Sheet) | Quick, repeatable, comparable | Response bias, literacy and language; screening is not diagnosis |
| **Clinician-rated scales** | HAM-D, BPRS, Y-BOCS, GAF, CGI | Independent of client self-report | Rater training and inter-rater reliability |
| **Functional assessment** | Work/school attendance, ADLs, WHODAS, social roles | Shows meaningful improvement | Depends on opportunity and context |
| **Client-reported outcomes and goal ratings** | Target complaint ratings, goal attainment, session feedback | Reflects what matters to the client | Variable anchors |
| **Routine outcome systems** | CORE (Clinical Outcomes in Routine Evaluation) in UK services | Service-level monitoring, benchmarking, CPD needs (Lister-Ford 2007, PDF p.118) | Requires organisational support |
| **Psychological tests (cognitive, personality, projective)** | Intelligence, neuropsychological, MCMI-III, Rorschach, TAT (repository manuals) | Deeper profile for diagnosis and planning | Rarely used for session-by-session monitoring; scanned manuals in the repository could not be text-reviewed |

Your own *Psychological Rating Scales Cheat Sheet* includes instrument summaries. For instance, the PHQ-9 is a 9-item self-report with 0–3 responses over the past two weeks; scores are 0–4 minimal, 5–9 mild, 10–14 moderate, 15–19 moderately severe, 20–27 severe [Rating Scales Cheat Sheet, PDF p.78]. The GAD-7 has 7 items; 0–4 minimal, 5–9 mild, 10–14 moderate, 15–21 severe, with a cut-off of 10 reported as having good sensitivity and specificity [Rating Scales Cheat Sheet, PDF p.80]. Always consult the manual or validation studies for the version and language you use; cheat-sheet figures are summaries.

## 4. Psychometric concepts for outcome measurement

**Table 9.3 Key concepts**

| Concept | Meaning | Why it matters for pre/post work |
|---|---|---|
| **Reliability** reliability (consistency of measurement) | Test–retest, internal consistency, inter-rater | Low reliability means score changes may be noise |
| **Validity** validity (whether the test measures what it claims) | Content, criterion, construct validity | An invalid measure cannot show true change |
| **Sensitivity to change** sensitivity to change (ability of a measure to detect real improvement) | Responsiveness over time | Trait measures (e.g., personality inventories) often change little; symptom scales are designed to change |
| **Norms and cut-offs** | Reference group scores; thresholds for clinical range | Needed to judge whether a post-score is within the functional range; must suit the population and language |
| **Reliable change** | Change large enough not to be due to measurement error (Reliable Change Index, Jacobson & Truax, 1991) | Separates real change from fluctuation |
| **Clinical significance** | Change that moves a person from the dysfunctional toward the functional range, or beyond a cut-off | Statistical significance in a group study does not guarantee that an individual client has recovered. Lister-Ford reports that even with 8–10 sessions about half of clients show clinically significant change [Lister-Ford 2007, PDF p.62] |
| **Meaningful functional improvement** | Return to roles, relationships, work, valued activities | The ultimate aim of therapy |

MNEMONIC — CHOOSING A GOOD MEASURE

## S P A N – V R

| | |
|---|---|
| **S** | Sensitive to change |
| **P** | Practical (short, free/affordable, easy to score) |
| **A** | Appropriate for age, language, culture, literacy |
| **N** | Norms or cut-offs available for the population |
| **V** | Valid for the purpose |
| **R** | Reliable |

**Apply:** to choose a depression measure for rural Hindi-speaking clients: check a validated Hindi version (A, N), brevity (P), responsiveness (S), validity and reliability evidence (V, R).

### – Worked example: reliable change and clinical significance (hypothetical psychometrics)

ILLUSTRATIVE EXAMPLE Suppose a depression scale has SD = 5.0 in the clinical group and test–retest reliability r = .85.

1. Standard error of measurement: SEM = SD × sqrt(1 - r) = 5.0 × sqrt0.15 = 1.94.
2. Standard error of the difference: $S_{diff}$ = sqrt2 × SEM = 2.74.
3. Reliable Change Index: RCI = ($score_{post}$ - $score_{pre}$) / $S_{diff}$. For 95% confidence, |RCI| > 1.96, i.e., a change of about 1.96 × 2.74 = 5.4 points is needed.
4. A client falling from 21 to 6 (change of 15) exceeds 5.4, so the change is **reliable**. If the clinical cut-off is 10, a post-score of 6 is also below the cut-off, so the change is **clinically significant**.
5. A client falling from 21 to 17 (change of 4) is below 5.4: **not reliable** (could be noise).

Note: the numbers are invented to show the method; use the real reliability and SD from the manual or validation study of your measure. The Jacobson–Truax method is a standard psychometric approach, not drawn from the repository books.

## 5. Selecting measures

1. **Purpose:** screening, diagnosis, severity, monitoring, outcome, risk, functioning.
2. **Client factors:** age, literacy, language, culture, disability, willingness; some patients cannot or will not fill out tests, and the therapist can teach them to rate mood or anxiety on a simple scale [Beck 2011, PDF p.85].
3. **Population evidence:** validation in similar Indian populations if available; avoid using translated tools without checking.

4. **Burden:** brief measures for weekly use; longer batteries for baseline and post.
5. **Ethical use:** informed consent for assessment (APA 9.03), test security, interpretation by qualified persons (APA 9.07), and sharing results in understandable language.
6. **Multiple sources:** self-report plus clinician rating plus function plus collateral.

## 6. Interpreting change: response, non-response and deterioration

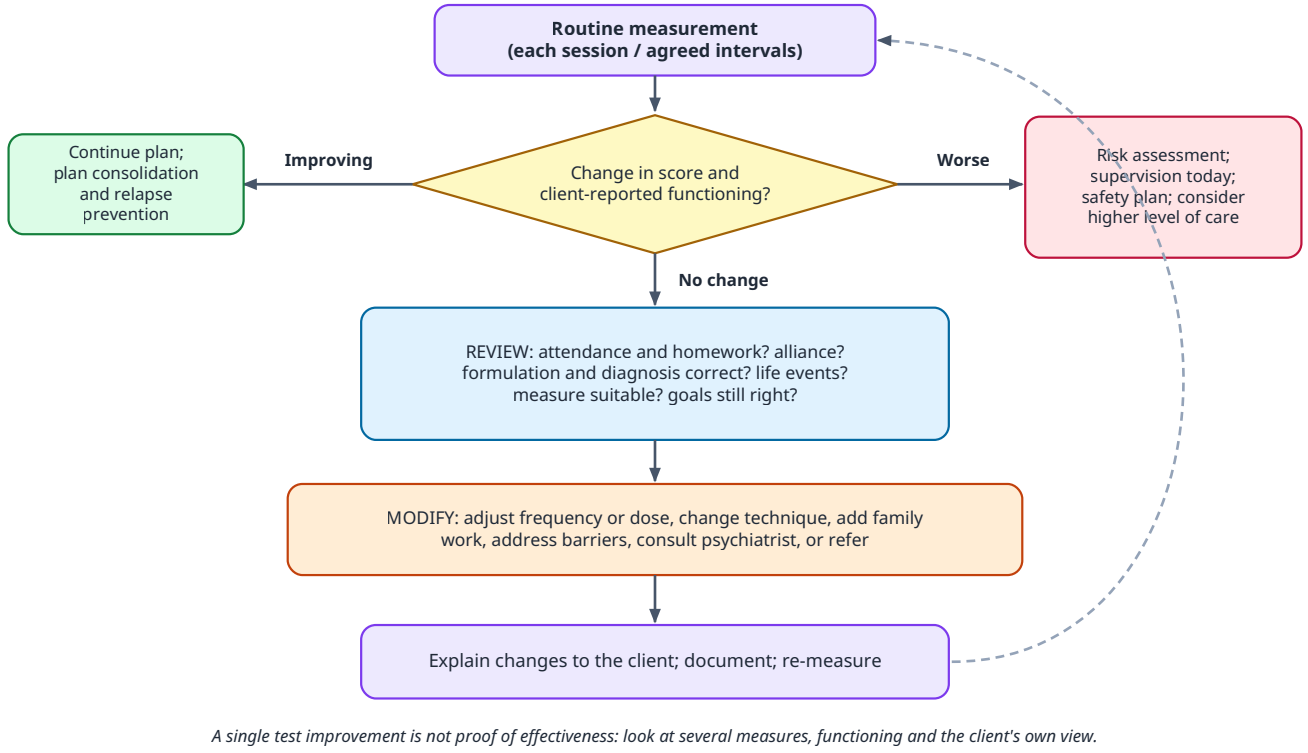



**Figure 9.2** Outcome-monitoring decisions (teaching schematic).
Source/basis: Beck 2011 PDF p.350, 369-370; Lister-Ford 2007 PDF p.118

**Table 9.4 Outcome categories and decisions**

| Category | Typical sign | Decision |
|---|---|---|
| Recovery / clinically significant improvement | Reliable change and post-score in the functional range | Consolidate; relapse prevention; space sessions; follow-up |
| Reliable improvement (not yet recovered) | Reliable change but score still in the clinical range | Continue or intensify; revisit goals |
| No reliable change (non-response) | Scores flat after an agreed number of sessions | Review adherence, alliance, formulation and diagnosis, comorbidity, life events; modify (dose, technique, add family, consult psychiatrist) or refer |
| Deterioration | Reliable worsening; new risk | Risk assessment; supervision; safety plan; reconsider treatment; possibly higher level of care |

Because some patients do not improve with psychotherapy and some get worse (Lambert & Bergin, 1994, cited in [Notes p.5]), routine monitoring is an ethical safeguard, not an optional extra. Beck advises treating lack of progress as an agenda item, collaborating on a different direction, and examining the alliance by asking about the patient's reactions [Beck 2011, PDF p.370]. Methods for checking your own work include recording sessions and rating them on the Cognitive Therapy Rating Scale and tracking progress with objective tests and the patient's subjective report [Beck 2011, PDF p. 369].

## 7. A practical example: pre, during, post and follow-up ILLUSTRATIVE EXAMPLE

**CASE: PRIYA, 24, DEPRESSION AND AVOIDANCE**

| Stage | Assessment | Findings | Action |
|---|---|---|---|
| **Baseline** (week 0) | Interview; PHQ-9; GAD-7; C-SSRS; function (college attendance); goals | PHQ-9 = 21; GAD-7 = 15; no active suicidal intent; attending 1 of 5 classes; goals set | Formulation; behavioural activation; weekly monitoring |
| **Monitoring** (weeks 1–16) | PHQ-9 weekly; activity log | Weeks 3–5 plateau at 17; avoidance of assignments | Review at week 4: add graded task plan, address sleep, involve family briefly; scores fall to 10 by week 10 |
| **Post** (week 16) | PHQ-9, GAD-7, function, client goal ratings | PHQ-9 = 6; GAD-7 = 7; attending 4 of 5 classes; goals achieved | Reliable and clinically significant change; relapse-prevention plan; spaced sessions |
| **Follow-up** (3 months) | PHQ-9; interview | PHQ-9 = 7; skills in use; one lapse after exams managed | Maintain; booster session offered if >= 10 |

## 8. Why improvement on a single test does not establish that therapy worked

- **Regression to the mean:** extreme scores tend to move toward the average on retest.
- **Natural remission and time:** many problems improve spontaneously; Lister-Ford reports improvement rates of about 40% even among untreated groups in some analyses [Lister-Ford 2007, PDF p.62].
- **Other influences:** medication, life events, social support, placebo/expectancy effects.
- **Measurement issues:** unreliable test, demand characteristics (client wishes to please), practice effects.
- **Narrow outcome:** symptoms may drop while functioning or quality of life do not.
- **No comparison group:** causal claims need controlled designs (RCTs) or at least multiple baselines; routine monitoring shows *change*, not necessarily *cause* (Topic 10).

- **Individual vs group results:** statistics from trials describe averages, not your client.

## 9. Ethical and legal considerations

- Obtain consent to assessments and explain purposes and limits (APA 9.03; MHCA s.4 and s.22 on information).
- Protect test data and results (Topic 7); interpret within competence (APA 9.01, 9.07 heading).
- Use scales that fit the client's language and culture; do not overinterpret a screening cut-off as diagnosis (the cheat sheet notes BDI-II is a severity measure, not a diagnostic tool).
- Record scores and decisions in the clinical record; share results in plain language.

Exam-oriented summary (Topic 9)

- B-M-P-F: baseline, monitoring, post, follow-up.
- Methods: interview, observation, standardised scales, function, client-reported outcomes; multiple sources.
- Properties: reliability, validity, sensitivity to change, norms; choose with SPAN-VR.
- Change: reliable change (RCI) + clinical significance + functional meaning.
- Non-response: review alliance, formulation, adherence; modify or refer. Deterioration: risk, supervision.
- One test improvement != proof of effectiveness (regression to mean, remission, other factors, no control).

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction (6 lines). B-M-P-F cycle (20 lines)** with Priya example.
2. **Methods (20 lines):** Table 9.2; scale examples from the cheat sheet.
3. **Psychometrics + clinical significance (22 lines):** reliability, validity, sensitivity, RCI, worked example.
4. **Selecting measures (10 lines):** SPAN-VR.
5. **Response, non-response, deterioration (14 lines):** Table 9.4; flowchart.
6. **Why a single test is insufficient (10 lines);** ethics (4 lines); conclusion (4 lines).

## 10. Practice questions (revision practice, not predictions)

**Short 3m** Why is baseline assessment necessary in psychotherapy?

**Short 3m** Define reliable change and clinical significance.

**Short 3m** List four criteria for selecting an outcome measure.

**Long 5m** Discuss pre- and post-assessment in psychotherapy, including methods, psychometric considerations and the interpretation of change.

**Compare** Statistical significance versus clinical significance; self-report versus clinician-rated measures.

**Application** After 6 sessions of CBT, a client's PHQ-9 is unchanged. How do you decide what to do?

**– Model answer framework for the application question**

1. Check the facts: correct administration; compare with previous scores and functioning; ask the client's view.
2. Explore attendance, homework adherence, alliance (ask for feedback), life stressors, substance use, medical issues.
3. Re-examine diagnosis and formulation; consider comorbidity (e.g., anxiety, trauma).
4. Assess risk.
5. Modify: adjust technique or frequency, address barriers, involve family, consult psychiatrist, or refer.
6. Discuss with the client and supervisor; document; re-measure.

## 11. Sources used in Topic 9

Beck 2011 (PDF p.85, 125, 143, 350, 369–370); Lister-Ford 2007 (PDF p.62, 118); Notes (p.5, 49); Rating Scales Cheat Sheet (PDF p.78, 80); RCI Diploma 2011 (p.8); APA Ethics Code 2017 (9.01–9.03 headings, 9.07 heading); Jacobson & Truax (1991) as the standard method (not in repository).

# 10 Practice of Evidence-Based Therapies

Evidence-based practice in psychology (EBPP) is a *decision process*, not just a list of therapies with research support. A strong answer defines EBPP, distinguishes it from merely using an "evidence-based treatment", explains the three components, covers research designs and their limits, manuals, fidelity and adaptation, and shows how to select, implement, monitor and modify a treatment responsibly.

**AT A GLANCE**

- **APA (2006):** EBPP = integration of the best available research with clinical expertise in the context of patient characteristics, culture and preferences.
- **Three circles:** Research · Expertise · Client (**REC**). Five steps: Ask · Acquire · Appraise · Apply · Assess (**5A**).
- **Evidence sources:** RCTs (efficacy), effectiveness and practice-based studies, systematic reviews/meta-analyses, process research, single-case designs.
- **Limits:** generalisability, researcher allegiance, manuals vs responsiveness, not all clients/conditions respond equally.
- **Responsible practice:** consent, fidelity with flexibility, outcome monitoring, modify or refer.

## 1. Introduction and definitions

**Table 10.1 Key definitions**

| Term | Definition | Source |
|---|---|---|
| **Evidence-based medicine** | "The conscientious, explicit and judicious use of current best evidence in making decisions about the care of individual patients" | Sackett et al. (1996), in Lister-Ford 2007, PDF p.116 |
| **Evidence-based practice in psychology (EBPP)** | The integration of the best available research with clinical expertise in the context of patient characteristics, culture and preferences | APA Presidential Task Force on EBP (2006, p.273), quoted in Corey 2024, PDF p.82 |
| **Empirically supported treatment (EST) / evidence-based treatment** | A specific intervention for a specific problem shown to be efficacious in controlled research | Corey 2024, PDF p.82; Hofmann & Weinberger 2007, PDF p.19 (APA Division 12 list) |
| **Efficacy vs effectiveness** | *Efficacy*: results under controlled research conditions; *effectiveness*: results in routine clinical settings (practice-based evidence) | Lister-Ford 2007, PDF p.118; Hofmann 2007, PDF p.28–29 |

**EBPP is not the same as "using an evidence-based therapy".** Corey notes that evidence-based practices tend to emphasise only the first pillar (best available research), while Norcross and colleagues advocate *inclusive* evidence-based practice with three pillars: best research, clinical expertise, and the client's characteristics, culture and preferences. Research consistently shows that the client, therapist, relationship, therapist's style, treatment method and context all contribute to success or failure [Corey 2024, PDF p.82]. Using a manualised protocol mechanically, without attention to the client and the alliance, is therefore not EBPP.

APA's Council of Representatives (2012) concluded that psychotherapy is effective and highly cost-effective, produces benefits that markedly exceed those of not receiving it, and should be included in health care as an established evidence-based practice; it encouraged further research on comparative effectiveness [APA 2012 Resolution, conclusion] PROFESSIONAL CODE . The APA Ethics Code (2.04) requires that work be based upon established scientific and professional knowledge of the discipline.

## 2. The three components of EBPP

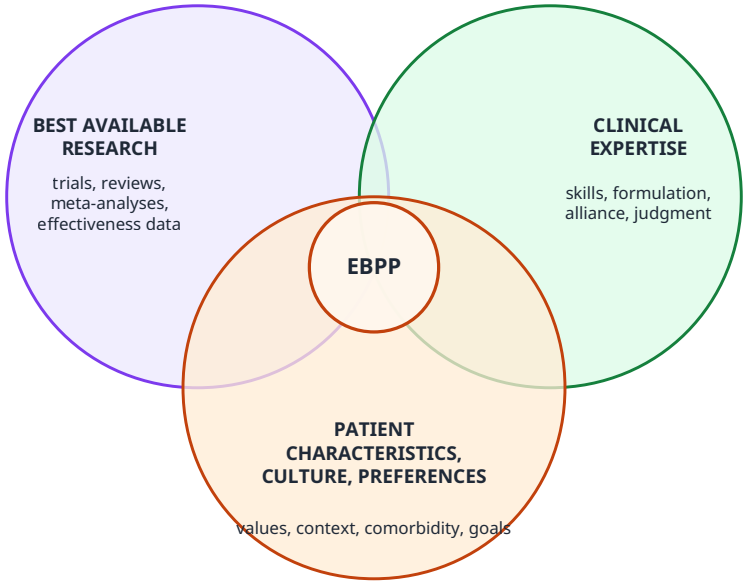



**Figure 10.1** The three-circle model of EBPP.
Source/basis: APA Presidential Task Force on EBP 2006, p.273 (as quoted in Corey 2024 PDF p.82)

**Table 10.2 Components and how to apply each**

| Component | Meaning | Clinical application |
|---|---|---|
| **Best available research evidence** | Systematic findings on efficacy, effectiveness, mechanisms, safety | Use guidelines, reviews and trials relevant to the problem and population; appraise quality |
| **Clinical expertise** | Skills in assessment, formulation, relationship, judgment, ethics | Tailor the protocol; handle comorbidity; manage alliance ruptures; know when evidence does not apply |

| | | |
|---|---|---|
| **Patient characteristics, culture and preferences** | Values, goals, beliefs, context, resources, language | Shared decision-making; cultural adaptation; offer options and respect refusal. Matching treatment to client characteristics improves engagement, retention and outcomes (Soto et al., 2018, in Corey PDF p.56) |

**MNEMONIC — THREE COMPONENTS**

**R · E · C**

**R**esearch · **E**xpertise · **C**lient. Think "**REC**ord all three before you decide". **Apply:** before choosing a treatment, write one line for each: what does the research say; what can I competently deliver; what does this client want and need?

## 3. Evidence-based treatments and their indications

No therapy works equally for every client or condition. The table lists examples whose evidence and indications are described in the repository texts; confirm current guidelines (e.g., NICE, APA Division 12, WHO) before applying.

**Table 10.3 Examples of evidence-informed psychological treatments and typical indications**

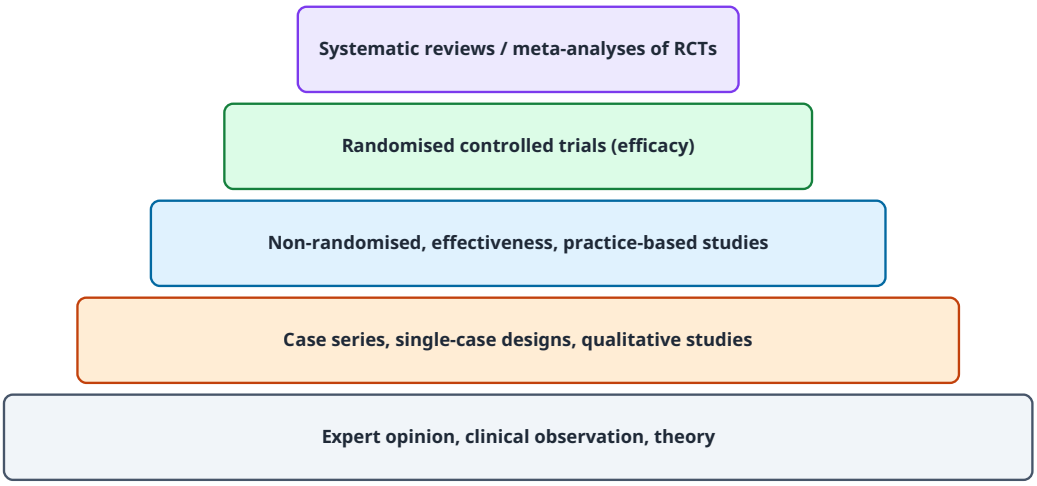
| Treatment | Typical indication | Repository source |
|---|---|---|
| CBT (cognitive restructuring, behavioural activation) | Depression, anxiety disorders; adapted for many other problems | Beck 2011; *CBT for Anxiety* manual; GAD workbook |
| Exposure and response prevention (ERP) within CBT | Obsessive–compulsive disorder | Foa et al. ERP therapist guide; Clark, *CBT for OCD* |
| CBT for psychosis | Psychotic symptoms, as adjunct to medication | Hagen et al., *CBT for Psychosis* |
| DBT | Emotion dysregulation and self-harm, borderline personality disorder | Linehan, *DBT Skills Training Manual*; Van Dijk, *DBT Made Simple* |
| Motivational Enhancement Therapy (MET) | Alcohol and substance-use problems; ambivalence about change | MET Manual (Project MATCH) |
| Transference-focused psychotherapy (TFP) and psychodynamic therapies | Personality disorders; recurrent interpersonal patterns | TFP texts; Gabbard 2004; Polnay 2023 |
| Interpersonal therapy (IPT) | Depression (time-limited, interpersonal focus) | Notes p.60; Sharf 2010 (IPT chapter) |
| Family psycho-education | Schizophrenia (relapse prevention by reducing expressed emotion) | Notes p.109–110; repository journal-club slides |

| | | |
|---|---|---|
| Play and CBT-based child therapies | Child anxiety, behaviour problems | Notes p.101–108 |
| Brief, manualised programmes by non-specialists (WHO PM+) | Common mental health problems in adversity and low-resource settings | WHO PM+ article (PMC4592660) GUIDELINE |

**AVOID OVER-CLAIMING**

"Evidence-based" refers to evidence for a *specific treatment for a specific problem in specific populations*. The Notes state that different major approaches are often equally effective and that this may be explained by common factors [Notes p.4]; the evidence-based treatment movement argues that some treatments are better supported for particular disorders. Lister-Ford adds that the absence of evidence of effectiveness for a given approach does not mean it is ineffective [Lister-Ford 2007, PDF p.118]. In an exam, present both views.

## 4. Research evidence: designs, quality and limits




*Higher levels are generally less biased, BUT the questions differ: process, alliance and individual response also need non-RCT evidence.*
*Lister-Ford (2007, PDF p.116-118) criticises over-reliance on RCTs and values "practice-based evidence"; Hofmann (2007, PDF p.28) urges multiple designs.*

**Figure 10.2** A general evidence hierarchy (with caveats).
Source/basis: General evidence hierarchy; Lister-Ford 2007 PDF p.116-118; Hofmann & Weinberger 2007 PDF p. 28-29

**Table 10.4 Research designs**

| Design / concept | What it shows | Strengths | Limitations (psychotherapy) |
|---|---|---|---|

| | | | |
|---|---|---|---|
| **Randomised controlled trial (RCT)** | Causal effect under controlled conditions (efficacy) | Random assignment reduces bias; manuals and adherence checks; considered the gold standard (Lister-Ford PDF p.116) | Selected samples, single diagnoses, standardised procedures that differ from routine practice; randomisation removes client choice; researcher allegiance (Lister-Ford PDF p.117) |
| **Systematic review** | Structured synthesis of studies on one question | Transparent search and appraisal | Depends on the quality of included studies |
| **Meta-analysis** and **effect size** | Statistical combination of results across studies as effect sizes; Smith and Glass analysed 475 controlled studies and found the average treated person better off than about 80% of untreated persons (Lister-Ford PDF p.62) | Greater power; shows size of effect | Heterogeneity; publication bias; averages hide individual variation |
| **Effectiveness / practice-based studies** | Outcomes in routine services (e.g., CORE, Practice Research Networks) | Representative clients and therapists | Less control; selection and confounding |
| **Process and process–outcome research** | What happens in therapy and how it relates to change (e.g., alliance, empathy) | Informs how therapy works | Correlational |
| **Single-case and qualitative designs** | Detailed individual change; client experience | Fit routine practice | Limited generalisation |

- **Effect size** effect size (the magnitude of an effect, independent of sample size): Cohen's d conventions (about 0.2 small, 0.5 medium, 0.8 large) are common guides but are context dependent. They are general statistical conventions, not drawn from the repository books.
- **Generalisability** generalisability (how far findings apply to other people and settings): most trials come from high-income countries; clients in routine practice have complex and multiple problems; the notion of discrete diagnostic categories is questioned. Lister-Ford argues that these differences reduce the external validity of RCTs for practitioners [Lister-Ford 2007, PDF p.116–117]. Hofmann and Weinberger's volume proposes triangulating on conclusions using multiple designs, including hybrid designs that run RCT methodology in the community [Hofmann 2007, PDF p.28–29].
- **Common factors vs specific ingredients.** Lambert (1992) estimated that outcome variance is explained by client/extra-therapeutic factors (about 40%), the therapeutic relationship (about 30%), expectancy/placebo (about 15%) and technique (about 15%) [Corsini 2011, PDF p.194]. These are rough estimates but show why EBPP must include the relationship and the client. Wampold's meta-analysis (cited by Corey) found personal and interpersonal components essential and technique relatively less influential [Corey 2024, PDF p.47].

- **Danger of over-reliance:** Parry warns that valuable therapies may become unavailable and innovation damaged if research is used to drive funding cuts rather than inform practice (cited in Lister-Ford PDF p.117). In the US, emphasis on empirically supported treatments, shaped by managed care, led to lists of approved therapies; in the UK, evidence reviews (Roth & Fonagy 1996) supported a range of psychotherapies, an example of two conclusions from similar evidence [Lister-Ford 2007, PDF p.76, 116].

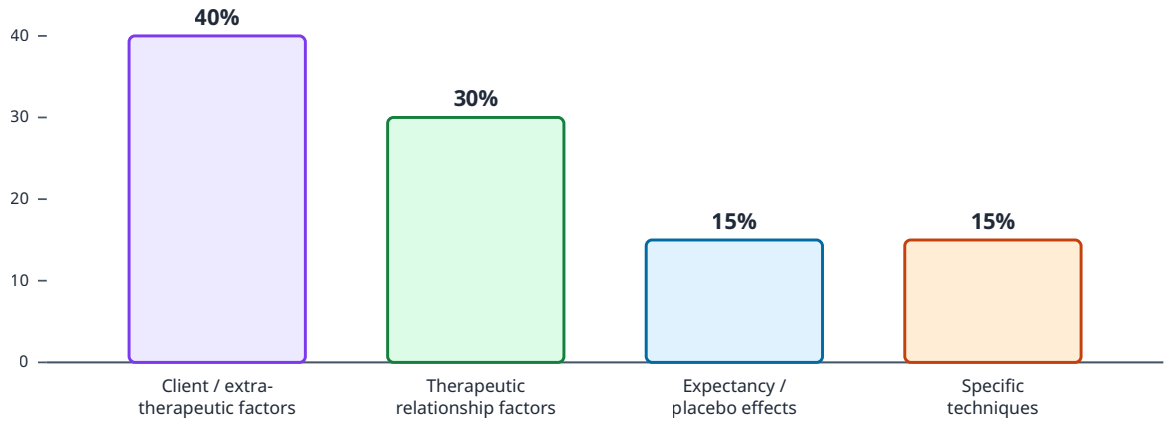



**Figure 10.4** Why EBPP must include the relationship and the client, not only technique.
Source/basis: Corsini & Wedding 2011 PDF p.194 (Lambert 1992)

## 5. Treatment manuals, guidelines, fidelity, adaptation and competence

- **Treatment manual** treatment manual (a written guide specifying the principles, structure and techniques of a therapy). Manuals are intended to define and differentiate psychotherapies, to standardise therapist technique and to permit replication; but they must be implemented by qualified therapists trained to perform them (Project MATCH Appendix C) [MET Manual, PDF p.138].
- **Clinical practice guidelines** summarise evidence and recommend treatments (e.g., NICE, APA, WHO mhGAP). Guidelines inform, but do not replace, individual decision-making. Lister-Ford warns that guidelines can become paternalistic if clients are given what is thought best for them rather than involved in choice [Lister-Ford 2007, PDF p.79].
- **Treatment fidelity** fidelity (how closely the treatment delivered matches the intended protocol) has three parts: adherence (delivering the specified elements), competence (skill in delivery) and differentiation (distinct from other therapies). In Project MATCH, therapists were selected for experience, trained in a 3-day seminar plus supervised training cases (average 3 cases; 26 supervised sessions), all sessions videotaped and reviewed against adherence, skillfulness, structure, empathy and alliance, with a proportion rated and supervisors alerted to therapist "drift" [MET Manual, PDF p.138–140]. In routine practice, rating scales such as the Cognitive Therapy Rating Scale measure CBT competence [Beck 2011, PDF p.16, 369].
- **Flexibility within fidelity.** Manuals do not undermine the relationship: a strong alliance is essential even in manual-based behaviour therapy, because it engages the patient, enhances motivation and facilitates homework adherence [Corsini 2011, PDF p.267]. Lister-Ford argues that outside research, strict adherence to a formula can be anti-therapeutic if it blocks responsiveness [Lister-Ford 2007, PDF p.117].

- **Adaptation.** Culturally and individually adapt content (language, examples, family involvement, pace) while preserving core therapeutic mechanisms. Monitor outcomes after adaptation. Therapist competence limits what you may deliver (APA 2.01).

**MNEMONIC — FIDELITY**

## A · C · D

**A**dherence · **C**ompetence · **D**ifferentiation. **Apply:** when asked how you know a therapy was delivered as intended, answer: adherence checks, competence ratings, and distinctness from other treatments, as in Project MATCH monitoring.

## 6. Selecting, implementing, monitoring and modifying an intervention

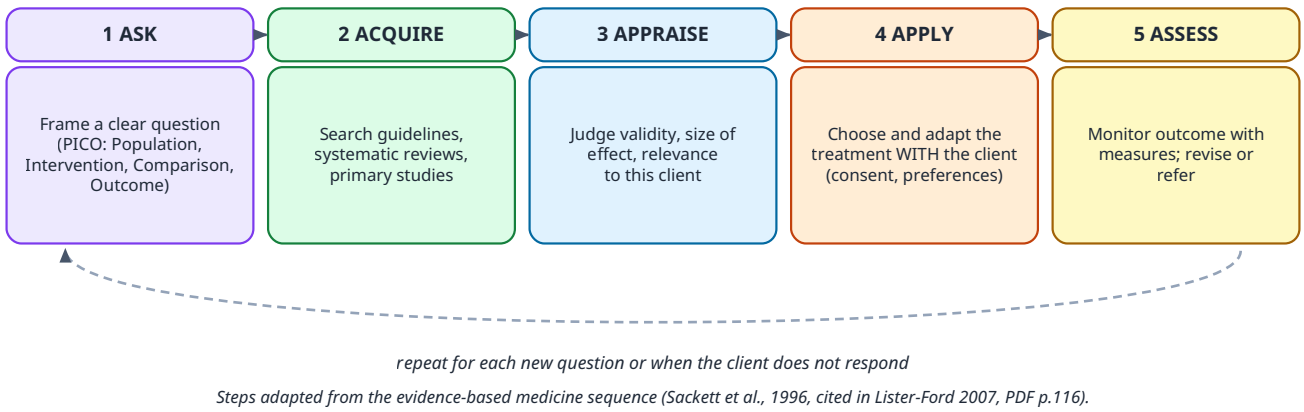



**Figure 10.3** The Ask-Acquire-Appraise-Apply-Assess cycle.
Source/basis: Lister-Ford 2007 PDF p.116 (EBM definition); standard EBM sequence

**MNEMONIC — THE 5A CYCLE**

## A · A · A · A · A

**A**sk (PICO question) · **A**cquire evidence · **A**ppraise it · **A**pply with the client · **A**ssess outcome. **PICO** = Population, Intervention, Comparison, Outcome. **Apply:** "In adolescents with panic disorder (P), is CBT (I) better than supportive therapy (C) for panic frequency (O)?"

1. **Assess and formulate** the problem (Topics 8–9).
2. **Ask a focused question** and **acquire** the best evidence (guidelines, reviews).
3. **Appraise** the evidence for validity, effect size and relevance to your client (culture, comorbidity, setting).
4. **Match to expertise:** can you deliver it competently? If not, train, supervise or refer (APA 2.01).
5. **Share the options** with the client: benefits, risks, alternatives, time and cost; obtain informed consent (Topic 6).

6. **Implement** with fidelity and responsiveness; maintain the alliance.
7. **Monitor outcomes** with agreed measures (Topic 9); review at set points.
8. **Modify** if non-response/deterioration: check delivery, alliance, formulation; adapt, add, switch or refer.
9. **Document** the evidence considered, rationale and client preferences.

## 7. Examples of evidence-based decision-making ILLUSTRATIVE EXAMPLE

**EXAMPLE A: OCD**

Rohan, 34, has contamination OCD. **Research:** ERP is a first-line psychological treatment (Foa et al. therapist guide; Clark). **Expertise:** the clinician is trained in ERP under supervision. **Client:** Rohan fears exposure and prefers a gradual approach; family accommodates rituals. **Decision:** graded ERP with MET-style motivational work first, family psychoeducation, Y-BOCS at baseline and every 4 weeks, weekly ritual-time log. **Review:** if no reliable change by week 8, check exposure adherence, ritual accommodation and comorbid depression; consider psychiatric consultation.

**EXAMPLE B: ALCOHOL USE WITH AMBIVALENCE**

Ajay, 41, drinks heavily but is unsure he wants to stop. **Evidence:** MET (four sessions) was developed to produce rapid, internally motivated change by mobilising the client's own resources [MET Manual, PDF p.11]. **Client preference:** he rejects abstinence-only goals. **Decision:** start MET with personalised feedback from assessment, negotiate goals (reduction vs abstinence), re-evaluate at midpoint and end, link to aftercare.

**EXAMPLE C: WHEN EVIDENCE DOES NOT FIT**

A client with complex trauma, learning disability and low literacy cannot complete a standard manualised protocol. The clinician adapts: shorter sessions, pictorial worksheets, caregiver involvement, and a stabilisation phase; sets simple goals; tracks function and mood with a one-item rating; and documents the departures from the manual and the reasons. This is EBPP: research informs, expertise adapts, client characteristics lead.

## 8. Ethical and legal considerations

- **Competence** (APA 2.01, 2.03): do not offer an evidence-based treatment you are not trained in.
- **Informed consent** (APA 10.01(b)): for treatments without established techniques, inform clients of the developing nature, risks, alternatives and voluntariness.
- **Honesty in claims** (APA 5.01 and RCI advertising rules SECONDARY SOURCE ): do not claim guaranteed outcomes or exaggerate the evidence.
- **Justice:** access to evidence-based care should be fair across language, region and income; consider task-sharing (WHO) with supervision.

- **Autonomy:** respect refusal or preference for another approach; guidelines should not become coercive.
- **Accountability:** document the evidence and reasoning; monitor outcomes; report results honestly.

## 9. Advantages, limitations and challenges

| Advantages | Limitations / challenges |
|---|---|
| • Improves effectiveness and safety.<br>• Supports accountability and funding decisions.<br>• Promotes transparency and learning.<br>• Encourages measurement of outcomes. | • Evidence is uneven across problems, ages and cultures; limited Indian data for some therapies.<br>• RCT samples differ from real clients; researcher allegiance.<br>• Risk of rigid, protocol-driven practice that ignores the alliance.<br>• Funding pressures can limit therapies without lists of support (Parry).<br>• Training and supervision demands; manuals may be expensive or unavailable in local languages.<br>• Evidence changes: guidelines need updating. |

Exam-oriented summary (Topic 10)

- EBPP (APA 2006) = research + clinical expertise + patient characteristics, culture, preferences (REC). Not the same as using an EST mechanically.
- Designs: RCT (efficacy), effectiveness, systematic reviews/meta-analyses (effect sizes), process and single-case; know limits (generalisability, allegiance).
- Manuals, guidelines, fidelity (A-C-D), adaptation, therapist competence.
- 5A cycle: Ask, Acquire, Appraise, Apply, Assess; modify with monitoring.
- Common factors matter: relationship about 30%, technique about 15% (Lambert estimate, Corsini).
- No therapy works equally for all: present both specific-treatment and common-factor views.

Answer skeleton: 5-mark long answer (~110–130 lines)

1. **Introduction and definitions (14 lines):** Sackett, APA 2006, EST, efficacy/effectiveness.
2. **Three components (14 lines):** REC figure.
3. **Evidence-based treatments and indications (16 lines):** Table 10.3 with cautions.
4. **Research evidence and limits (24 lines):** designs, meta-analysis, effect size, RCT limits, common factors.
5. **Manuals, fidelity, adaptation, competence (16 lines).**
6. **Implementing responsibly (14 lines):** 5A, monitoring, modifying; example.
7. **Ethics (6 lines);** limitations and conclusion (6 lines).

## 10. Practice questions (revision practice, not predictions)

**Short 3m** Define evidence-based practice in psychology (APA, 2006) and name its components.

**Short 3m** Distinguish efficacy from effectiveness.

**Short 3m** What is treatment fidelity and how can it be monitored?

**Long 5m** Discuss the practice of evidence-based therapies, including research evidence, manuals, fidelity and the role of client characteristics.

**Compare** Empirically supported treatments versus common-factors approaches.

**Application** A client with OCD refuses exposure tasks. How would you proceed in an evidence-based way?

**– Model answer framework for the application question**

1. Research: ERP is a first-line evidence-based treatment for OCD.
2. Client: explore reasons for refusal (fear, lack of understanding, family accommodation); preferences are part of EBPP.
3. Expertise: motivational strategies; psychoeducation; very graded hierarchy; involve family; start with cognitive work if acceptable.
4. Informed choice: explain options, risks of not doing exposure, alternatives (medication consult).
5. Monitor: Y-BOCS, ritual time; review at an agreed point; modify or refer.
6. Document reasoning and consent.

## 11. Sources used in Topic 10

Corey 2024 (PDF p.47, 56, 82); Lister-Ford 2007 (PDF p.62, 76, 79, 116–118); Hofmann & Weinberger 2007 (PDF p.19, 28–29); Corsini 2011 (PDF p.194, 267); MET Manual and Appendix C (PDF p.11, 138–140); Beck 2011 (PDF p.16, 369); Notes (p.4, 5, 60, 101–110); APA 2012 Resolution; APA Ethics Code 2017 (2.01, 2.04, 5.01, 10.01); WHO PM+ (PMC4592660). APA 2006 definition quoted via Corey and three secondary sources.

# Unit I Mnemonic Master Sheet

One-page revision of every mnemonic in these notes. Each letter maps to a real item from the topic; the sentence is only a memory hook. Do not use a mnemonic if you cannot unpack every letter.

**Master table of mnemonics**

| Topic | Mnemonic | Letters = meaning | Use it for |
|---|---|---|---|
| 1 | **TRAPS** | Trained person · Relationship · Approach · Purpose · Shared direction | Elements of a definition of psychotherapy |
| 1 | **HERS** | Hope · Environment (healing setting) · Relationship · Scheme (rationale) | Four common features of all psychotherapies |
| 1–2 | **R-M-P** | Remove/modify/retard symptoms · Mediate behaviour patterns · Promote growth | Wolberg's three aims |
| 2 | **H-M-A-A-A-I** | Hope · Mastery · Avoidance ended · Awareness of misconceptions · Acceptance · Insight | Kleinke's six goals |
| 2 | **SCRIBE-FM** | Symptoms · Coping · Relationships · Insight · Behaviour · Emotion regulation · Functioning · Maintenance | Eight objective domains |
| 3 | **TRAIN-ME** | Theory · Reflective practice · Assessment · Intervention · Norms (ethics/law) · Multicultural · Experience (supervised) | Building blocks of therapist training |
| 3 | **CARE** | Consult · Aware of signs · Rest/relationships · Evaluate caseload | Self-care and impairment response |
| 4 | **A-B-N-J** | Autonomy · Beneficence · Non-maleficence · Justice | Classical ethical principles |
| 4 | **B-F-I-J-R** | Beneficence/nonmaleficence · Fidelity · Integrity · Justice · Respect (APA A–E) | APA General Principles |
| 4, 6 | **C-V-I-D** | Capacity · Voluntary · Informed · Documented | Valid consent |
| 4 | **NOCAPS** | Nominated representative · Other professionals · Court/Board order · Another person at risk · Public safety · Saving life | MHCA s.23(2) exceptions |
| 4 | **ISCLCBED** | Identify · Stakeholders · Codes · Laws · Consult · Brainstorm · Evaluate · Decide | Ethical decision-making |
| 5 | **PRIDE** | Privacy · Respect · Information · Decide · Exit & Escalate | Client rights |
| 5 | **HAFT** | Honest · Attend · Fees · Take part | Client responsibilities |
| 5 | **3C + B + 3R** | Competence, Consent, Confidentiality · Boundaries · Records, Referral/continuity, Respect | Therapist responsibilities |
| 6 | **EAPP** | Explain · Assent · Preferences/best interests · Permission (guardian) | APA 3.10(b) for minors |
| 7 | **PREP** | Previous notes · Risk & measures · Evidence-based choice · Practicalities | Pre-session preparation |

| | | | |
|---|---|---|---|
| 7 | **CARTO** | Confidential · Accurate · Relevant · Timely · Objective | Qualities of a good note |
| 7 | **S-O-A-P / D-A-P** | Subjective · Objective · Assessment · Plan / Data · Assessment · Plan | Note formats |
| 8 | **IAF-PIRT** | Intake · Assessment · Formulation · Plan · Intervention · Review · Termination | Therapy phases |
| 8 | **MAUHPWSHF** | Mood check · Agenda · Update · Homework review · Prioritise · Work on problem · Summary · new Homework · Feedback | Ongoing CBT session structure |
| 8 | **SMART** | Specific · Measurable · Achievable · Relevant · Time-bound | Goal setting |
| 9 | **B-M-P-F** | Baseline · Monitor · Post · Follow-up | Assessment time points |
| 9 | **SPAN-VR** | Sensitive · Practical · Appropriate · Norms · Valid · Reliable | Choosing a measure |
| 10 | **R-E-C** | Research · Expertise · Client (characteristics, culture, preferences) | EBPP components |
| 10 | **5A / PICO** | Ask · Acquire · Appraise · Apply · Assess / Population · Intervention · Comparison · Outcome | EBP cycle and question |
| 10 | **A-C-D** | Adherence · Competence · Differentiation | Treatment fidelity |

**HOW TO USE THE SHEET IN AN ANSWER** ILLUSTRATIVE EXAMPLE

Question: “Discuss ethical issues when a 15-year-old discloses self-harm.” Open with **ISCLCBED** as your frame; use **EAPP** for assent; **NOCAPS** for the limits of confidentiality; **PRIDE** for her rights; **B-M-P-F** for follow-up. Each acronym becomes a paragraph heading, which gives the examiner a clear structure.

# Examination Preparation

All questions below are **practice and likely-revision questions**, not predictions of what will appear. Use the answer skeletons at the end of each topic for line budgets.

## 1. Key terms (quick glossary)

| Term | Meaning |
|---|---|
| Psychotherapy | Planned psychological treatment by a trained person within a deliberate professional relationship to relieve symptoms, change patterns and promote growth (Wolberg; Norcross) |

| | |
|---|---|
| Therapeutic alliance | The collaborative working relationship between therapist and client, with agreement on goals and tasks |
| Common factors | Features shared by most therapies (relationship, expectancy, rationale, setting) that account for much of the outcome |
| Case formulation | A working explanation of how problems developed and are maintained, used to plan treatment |
| Countertransference | The therapist's emotional reactions to the client, shaped by the therapist's own history |
| Supervision | Regular structured review of cases with an experienced professional |
| Competence (ethical) | Working only within the limits of one's education, training and experience |
| Informed consent | Voluntary agreement by a competent person after adequate information (MHCA s. 2) |
| Assent | A minor's agreement to take part after an age-appropriate explanation |
| Capacity | Ability to understand, appreciate consequences and communicate a decision (MHCA s.4) |
| Nominated representative | Person who supports or represents a person with mental illness; for a minor, normally the legal guardian (MHCA s.15) |
| Confidentiality | Duty not to disclose what clients reveal, subject to defined limits |
| Privileged communication | Legal protection from compelled disclosure in court |
| Boundary crossing / violation | A departure from the usual frame that may be harmless or helpful / one that is exploitative or harmful |
| Multiple (dual) relationship | A professional role plus another role with the same person (APA 3.05) |
| Process notes | The therapist's reflective notes on process and reactions, kept apart from the clinical record |
| SOAP / DAP | Note formats: Subjective-Objective-Assessment-Plan / Data-Assessment-Plan |
| SMART goal | Specific, Measurable, Achievable, Relevant, Time-bound |
| Reliable change | Change larger than measurement error |
| Clinical significance | Change that moves the client into the functional range |
| EBPP | Integration of best research with clinical expertise in the context of patient characteristics, culture and preferences |
| Efficacy / effectiveness | Results under controlled conditions / results in routine practice |

| | |
|---|---|
| Treatment fidelity | Degree to which a treatment is delivered as intended (adherence, competence, differentiation) |
| Effect size | Magnitude of a treatment effect independent of sample size |

## 2. Short-answer questions (3 marks each)

1. Define psychotherapy; name Wolberg's three aims.
2. List the four features common to all psychotherapies.
3. Distinguish psychotherapy from psychological assessment.
4. State Kleinke's six common goals.
5. Define goals and objectives with an example each.
6. Outline the pathway to become a clinical psychologist in India.
7. What are the functions of supervision?
8. Differentiate countertransference from burnout.
9. List the exceptions to confidentiality in s.23 of the MHCA 2017.
10. What is a multiple relationship? When is it unethical (APA 3.05)?
11. List five client rights.
12. State APA Standard 10.10 on termination.
13. Distinguish consent from assent.
14. Who is the nominated representative of a minor under the MHCA?
15. State the duty under POCSO s.19.
16. Explain SOAP format.
17. List four qualities of a good clinical note.
18. Outline the typical agenda of an ongoing CBT session.
19. What are SMART goals?
20. Define reliable change and clinical significance.
21. State the APA (2006) definition of EBPP.
22. Distinguish efficacy and effectiveness research.
23. What is treatment fidelity?

## 3. Long-answer and essay questions (5 marks; about 100–150 lines)

1. Define psychotherapy. Discuss common elements of definitions and distinguish it from counselling, assessment and psychiatric treatment. *(Topic 1)*
2. Discuss the objectives of psychotherapy and how they vary by client, diagnosis, approach and setting. *(Topic 2)*
3. Discuss issues in the training of professional therapists: education, supervision, personal qualities, cultural competence and burnout. *(Topic 3)*
4. Discuss ethical and legal issues in therapy, with reference to Indian law and professional codes. *(Topic 4)*
5. Present the rights and responsibilities of clients and therapists in a comparison table and discuss their handling. *(Topic 5)*

6. Discuss consent and assent in psychotherapy with minors. *(Topic 6)*
7. Describe planning and recording of therapy sessions, including documentation formats and data protection. *(Topic 7)*
8. Describe how a session is structured and how therapy goals are set, monitored and revised. *(Topic 8)*
9. Discuss pre- and post-assessment and the interpretation of change in psychotherapy. *(Topic 9)*
10. Discuss the practice of evidence-based therapies, including research evidence, manuals, fidelity and client factors. *(Topic 10)*

## 4. Compare-and-contrast questions

| Question | Key points to compare |
|---|---|
| Wolberg vs Norcross definitions | Treatment/trained person/three aims vs informed intentional application/ participants decide direction |
| Counselling vs psychotherapy | Guidance/adjustment vs treatment/deeper change; blurred boundary; Dreikurs: change within vs of lifestyle |
| Ethics vs law vs code | Ought vs must; persuasive vs binding; association vs State; conflict rule (APA 1.02) |
| Confidentiality vs privilege | Ethical-legal duty of therapist vs legal protection in court |
| Boundary crossing vs violation | Possibly benign vs exploitative; examples in Table 4.4 |
| Consent vs assent | Legal permission vs child's agreement; Table 6.1 |
| SOAP vs DAP; clinical vs process notes | Table 7.2; Table 7.1 |
| Goals vs objectives | Broad vs measurable (Jongsma) |
| Reliable change vs clinical significance | Beyond error vs into functional range |
| Efficacy vs effectiveness; EST vs EBPP | Controlled vs routine; treatment list vs decision process |

## 5. Case-based and ethical-dilemma questions with model answers

### Case 1: Threat to a third party ILLUSTRATIVE EXAMPLE

A client says he will “finish” his wife if she goes to her mother's house. **Answer framework:** ISCLCBED. Identify a possible threat to life; assess risk; consult supervisor; check MHCA s.23(2)(c),(e) and APA 4.05; options include safety planning, voluntary admission, notification of potential victim or police with minimum necessary information; document; revisit consent. See Topic 4 §5.

### Case 2: Parents and a 15-year-old disagree ILLUSTRATIVE EXAMPLE

The mother consents to therapy for her 15-year-old son; he refuses to attend and says he will not speak. **Answer framework:** explore his reasons; explain therapy and confidentiality limits to him in age-appropriate language; seek assent but do not coerce; negotiate a trial session; keep the mother informed in general terms; if refusal persists, discuss with mother and supervisor; document. If he had asked for therapy and the mother refused, legal consent rests with the guardian; explore her concerns; escalate through safeguarding only if there is risk. See Topic 6 §7.

### Case 3: Dual relationship in a small town ILLUSTRATIVE EXAMPLE

A client who is also your neighbour's cousin asks you to attend her wedding. **Answer framework:** APA 3.05 (multiple relationships) and 3.06 (conflict of interest); consider whether attendance could impair objectivity or exploit; decline politely, explain boundaries; discuss within therapy; consult supervisor; document. See Topic 4 §7.

### Case 4: Tele-therapy across states ILLUSTRATIVE EXAMPLE

A client moves to another state and wants to continue online. **Answer framework:** competence in tele-therapy (APA Telepsychology Guideline 1); informed consent specific to telepsychology (Guideline 3); security and disposal of data (Guidelines 5–6); legal and registration questions across jurisdictions (Guideline 8; Corsini PDF p.594); emergency plan; confidentiality limits in the client's location; DPDP duties. See Topic 4 §8.

### Case 5: Request for records from a client with unpaid fees ILLUSTRATIVE EXAMPLE

See Topic 5 §9 model answer: consent exists; release a clinically adequate summary (MHCA s.23(2)(b), s.25); do not withhold for non-payment where it affects care (APA 6.03); address fees separately with notice (6.04(e)).

## 6. High-yield revision tables

**Table E1 Indian legal provisions at a glance (verify before use in practice)**

| Provision | Gist | Topic |
|---|---|---|
| RCI Act 1992 s.13 | Only enrolled professionals may practise; penalty for contravention | 1, 3, 4 |
| RCI Act s.21 | Code of ethics, misconduct, removal from Register after hearing | 4, 5 |
| MHCA s.2(1)(g) | Definition of clinical psychologist (RCI-recognised; M.Phil. 2 years full-time with supervised training) | 3 |
| MHCA s.2(1)(i) | Informed consent definition | 4, 6 |
| MHCA s.4 | Capacity: understand, appreciate, communicate; unwise decision is not incapacity | 4, 5, 6 |
| MHCA s.15 | Legal guardian = nominated representative of minor unless Board orders | 6 |

| | | |
|---|---|---|
| MHCA s.20–21 | Dignity; equality/non-discrimination | 5 |
| MHCA s.22 | Right to information in understandable form | 5, 6 |
| MHCA s.23 | Confidentiality and listed exceptions | 4 |
| MHCA s.24–25 | Digital data and media; access to records | 4, 5, 7 |
| MHCA s.28 | Right to complain: professional in charge » Board » State Authority | 5 |
| MHCA s.55(1)(d) | State Authority registers clinical psychologists as MH professionals | 3 |
| MHCA s.87 | Admission of minors: safeguards | 6 |
| MHCA s.115 | Attempted suicide presumed severe stress; not punished under the IPC provision | 4 |
| POCSO s.2(d), s.19, s.21 | Child = below 18; duty to report sexual offences; penalty for failure | 4, 6 |
| DPDP Act s.9 SECONDARY SOURCE | Verifiable parental consent for children's data | 6, 7 |
| ICMR 2017 §6.5 | Research: parental consent + assent aged 7–18 | 6 |

**Table E2 APA Ethics Code standards worth remembering**

| Standard | Content |
|---|---|
| 1.02 | Conflicts between ethics and law: clarify, state commitment, resolve; may follow law if unresolvable; never to violate human rights |
| 2.01 / 2.03 / 2.06 | Boundaries of competence / maintaining competence / personal problems |
| 3.04 / 3.05 / 3.06 / 3.08 | Avoiding harm / multiple relationships / conflict of interest / exploitative relationships |
| 3.10 | Informed consent; assent; permission; court-ordered services; documentation |
| 4.01–4.05 | Confidentiality; discussing limits; recording permission; minimising intrusion; disclosures |
| 6.01–6.04 | Documentation; confidential records; no withholding for non-payment in emergencies; fees |
| 10.01 / 10.05–10.08 / 10.10 | Consent to therapy (incl. trainee status) / sexual intimacies (2-year rule for former clients) / termination |

**Table E3 Numbers and names to remember**

| Item | Value | Source |
|---|---|---|
| Approaches to therapy | >400 (Karasu 1986); >450 schools (Simon 2001) | Notes p.72, 8 |
| Meta-analysis | 475 controlled studies (Smith & Glass) | Lister-Ford PDF p. 62 |
| Variance in outcome (Lambert 1992) | 40% client, 30% relationship, 15% expectancy, 15% technique | Corsini PDF p.194 |
| APA former clients rule | At least 2 years; then only exceptional, burden on psychologist | APA 10.08 |
| Record retention (APA guideline) | 7 years adults; 3 years after majority for minors, unless superseded | APA RK Guidelines 2007 |
| RCI diploma minimum clinical work | 50 work-ups, 30 psychodiagnostic, 20 therapy cases (200 h; >= 5 children) | RCI Diploma 2011 p.9 |
| Project MATCH training | 3-day seminar; >= 2 training cases; average 26 supervised sessions to certify | MET Manual PDF p.139 |
| ICMR assent age | 7–18 years (research) | ICMR 2017 §6.5.4 |
| UKCP training | At least 4 years part-time postgraduate | Lister-Ford PDF p. 114 |

## 7. Common conceptual confusions and how to avoid them

| Confusion | Clarification |
|---|---|
| “Psychotherapy = counselling” | Overlapping; distinctions of emphasis; state the view you are quoting |
| “Confidentiality is absolute” | It has legal and ethical limits; explain at the start; release minimum necessary |
| “Assent replaces consent” | No: guardian consent is the legal default; assent is the child's agreement |
| “Guidelines are laws” | APA guidelines are aspirational; statutes are binding |
| “Evidence-based = a manualised therapy on a list” | EBPP integrates research, expertise and client characteristics |
| “Improvement on a test proves therapy worked” | Regression to mean, remission, other influences, no control |
| “APA rules apply to Indian psychologists as law” | They are a respected reference, not Indian law; RCI/MHCA bind in India |

| | |
|---|---|
| “HIPAA applies in India” | HIPAA is a US law; used here only for comparison |
| “All therapies are equal for all problems” | Broadly comparable overall, but specific treatments are better supported for specific disorders |
| “Boundary crossing is always wrong” | Some are benign or helpful; violations are exploitative |

## 8. Final rapid-revision sheet: all ten topics

One-page recall

1. **Definitions:** Wolberg (treatment; trained person; deliberate relationship; remove/mediate/promote). Norcross (informed, intentional; participants' desired direction). TRAPS; HERS. Counselling vs assessment vs psychiatry. RCI s.13; MHCA mental healthcare.
2. **Objectives:** R-M-P; Kleinke 6; SCRIBE-FM; aims»goals»objectives; vary by client/diagnosis/approach/setting; negotiate; MHCA rights; RCI short/long-term objectives.
3. **Training:** PG + 2-yr M.Phil. » RCI register » State registration; supervised practice, logbook; TRAIN-ME; qualities; supervision; personal therapy; culture; countertransference; burnout (CARE); refer; tell clients about trainee status.
4. **Ethics/law:** principles A-B-N-J; B-F-I-J-R; CVID; NOCAPS; POCSO s.19; boundaries; APA 3.05, 10.05–10.08; records, digital; ISCLCBED; APA 1.02; RCI s.21.
5. **Rights/responsibilities:** PRIDE; HAFT; 3C+B+3R; MHCA ss.18–28; records s.25; complaints s. 28; termination APA 10.10.
6. **Consent/assent:** guardian consent + assent; EAPP; MHCA s.15, s.87; POCSO; DPDP s.9; ICMR 7–18; explain in age-appropriate words; disagreements; no universal rule.
7. **Planning/recording:** PREP; Jongsma 6 steps; SOAP/DAP; CARTO; process vs clinical vs research records; recording permission; retention 7y/3y; disposal.
8. **Structure/goals:** IAF-PIRT; MAUHPWSHF; goals vs objectives; SMART; collaborative; revise; variations across approaches.
9. **Pre/post-assessment:** B-M-P-F; methods; SPAN-VR; reliable change; clinical significance; non-response/deterioration; one test is not proof.
10. **Evidence-based practice:** APA 2006; REC; 5A/PICO; RCT vs effectiveness; meta-analysis/effect size; manuals; fidelity A-C-D; adaptation; common factors; no therapy for all.

# References, Source Mapping and Source Audit

## 1. Books and documents consulted (with pages actually read)

**Table R1 Repository sources**

| Source (as found in the repository) | PDF pages read for this edition | Used in topics |
|---|---|---|

| | | |
|---|---|---|
| *notes_of_psychotherapies[1].pdf* (112 pp.; compilation of book chapters) | 1–12, 24, 49, 52, 55–58, 60, 71–72, 77–78 (and headings of all pages) | 1–10 |
| Wolberg, L. R. (2013). *The Technique of Psychotherapy* (4th ed.) | 29–32, 82, 106, 125, 130, 133–134, 160–161, 676 | 1, 2, 3, 7 |
| Wolberg, L. R. (1982; Routledge 2014). *The Practice of Psychotherapy: 506 Questions and Answers* | 31, 104 | 5, 8 |
| Corey, G. (2024). *Theory and Practice of Counseling and Psychotherapy* (11th ed.) | 31, 47–49, 56, 59–65, 74–78, 82 | 1–6, 8, 10 |
| Corsini, R. J., & Wedding, D. (2011). *Current Psychotherapies* (9th ed.) | 166, 194, 267, 459, 594–597, 602, 605 | 2, 4, 8, 10 |
| Sharf, R. S. (2010). *Theories of Psychotherapy and Counseling* (5th ed.) | 158–159 (and contents) | 1, 10 |
| Murdock, N. L. (2017). *Theories of Counseling and Psychotherapy: A Case Approach* (4th ed.) | 38–40 | 3–6 |
| Jongsma, A. E., Peterson, L. M., & Bruce, T. J. (2014). *The Complete Adult Psychotherapy Treatment Planner* (5th ed.) | 23–26 | 2, 7, 8 |
| Beck, J. S. (2011). *Cognitive Behavior Therapy: Basics and Beyond* (2nd ed.) | 16, 20, 31, 42, 78, 81–85, 90, 122–125, 143, 350, 369–370 | 2, 3, 7–10 |
| Hofmann, S. G., & Weinberger, J. (Eds.) (2007). *The Art and Science of Psychotherapy* | 19, 28–29 | 10 |
| Lister-Ford, C. (2007). *A Short Introduction to Psychotherapy* | 62, 76, 78–79, 114–118 | 1, 3, 9, 10 |
| Gabbard, G. O. (2004). *Long-Term Psychodynamic Psychotherapy: A Basic Text* | 58, 192–193 | 1, 3, 8 |
| Polnay, A. et al. (2023). *Cambridge Guide to Psychodynamic Psychotherapy* | 64, 77 | 3 |
| *Motivational Enhancement Therapy Manual* (Project MATCH), incl. Appendix C | 11, 138–140 | 2, 3, 8, 10 |
| Van Dijk, S. *DBT Made Simple* | 34, 72 | 2, 8 |
| *Psychological Rating Scales Cheat Sheet* (author: Tejas Dangodra; in repository) | 3, 78, 80 | 9 |

## 2. Online and primary documents consulted

**Table R2 Online sources and verification status**

| Source | What it supported | Status |
|---|---|---|

| | | |
|---|---|---|
| Rehabilitation Council of India Act, 1992 (No. 34 of 1992) with 2000 Amendment (Gazette text) | ss.13, 18, 19, 21, 23; definition of rehabilitation professionals | Primary text read |
| Government of India notice on RCI (Ministry of Social Justice and Empowerment) | Four statutory functions of RCI | Primary notice read |
| RCI, *Professional Diploma in Clinical Psychology* (2011), hosted on a Government of India site | Aims, supervision, centre requirements, minimum work, records, syllabus | Primary document read; diploma, not M.Phil.; may be superseded |
| Nyaaya (2022): *General Duties of a Clinical Psychologist*; *Complaining against a Clinical Psychologist*; *Code of Conduct for Psychologists* | RCI (Standards of Professional Conduct, Etiquette and Code of Ethics for Rehabilitation Professionals) Regulations 1998 numbering; complaint forums; IACP code mention | SECONDARY SOURCE Legal-literacy website; primary regulation text not obtained |
| Mental Healthcare Act, 2017 (Gazette of India Extraordinary, 7 April 2017) | Preamble; ss.2, 4, 5, 14, 15, 18–28, 55, 87, 115 | Primary text read (archive.org scan) |
| Protection of Children from Sexual Offences Act, 2012 (Act text) | ss.2(d), 19, 21 | Text read from an Act reproduction |
| Digital Personal Data Protection Act, 2023 and Rules, 2025 | s.9 children's data; exemptions | SECONDARY SOURCE Commentary only |
| ICMR (2017). *National Ethical Guidelines for Biomedical and Health Research Involving Human Participants* | §6.5 children; assent 7–18 | Primary PDF read (research context) |
| American Psychological Association (2017). *Ethical Principles of Psychologists and Code of Conduct* | Principles A–E; Standards 1.02, 1.04, 2.01–2.06, 3.04–3.10, 3.12, 4.01–4.07, 6.01–6.06, 7.04–7.07, 10.01–10.10 | Primary PDF read |
| APA (2007). *Record Keeping Guidelines*. American Psychologist | Record content, security, retention, disposition | Primary PDF read |
| APA (2013). *Guidelines for the Practice of Telepsychology* | Eight guidelines | Primary PDF read; a revised (2024) version exists, not read |
| APA (2012). *Resolution on the Recognition of Psychotherapy Effectiveness* | Conclusion of the resolution | Conclusion read via the Society for the Advancement of Psychotherapy page |
| APA Presidential Task Force on Evidence-Based Practice (2006), *American Psychologist* 61(4), 271–285 | EBPP definition | Definition quoted from Corey 2024 and three secondary sources; article itself not read |
| British Psychological Society, *Code of Ethics and Conduct* (2021) | Four principles | BPS web pages; full code not read |

| | | |
|---|---|---|
| World Health Organization: mhGAP Intervention Guide; Problem Management Plus (PM+) | Task-sharing, brief manualised programmes | PubMed Central article PMC4592660 and WHO pages (existence/purpose) |
| Indian Association of Clinical Psychologists (IACP), code of conduct | Existence of a code adopted for professional members | SECONDARY SOURCE Website unreachable; secondary excerpts only; **text not read** |
| Jacobson, N. S., & Truax, P. (1991). Clinical significance: a statistical approach to defining meaningful change in psychotherapy research. *Journal of Consulting and Clinical Psychology*, 59, 12–19 | Reliable Change Index method | Standard method cited from knowledge; not in repository; verify before quoting |

## 3. Source audit

**WHAT WAS REVIEWED**

- The repository contained **85 PDF files (80 unique by checksum; 5 exact duplicates)**. Text was extracted from all of them. **62 unique PDFs have a readable text layer; 18 are scanned images** with no text layer.
- Not every readable PDF is a psychotherapy book: many are assessment manuals, ICD, medicine and behavioural-medicine texts or personal case documents. I did not read all 62 in full; I searched the psychotherapy books for each syllabus topic and read the passages cited in Table R1 in context.
- **Psychotherapy books consulted for Unit I:** Notes PDF, Wolberg (two books), Corey, Corsini, Sharf, Murdock, Jongsma, Beck, Hofmann & Weinberger, Lister-Ford, Gabbard, Polnay, MET Manual, DBT Made Simple.
- **Searched by keyword but not used** for citations in this edition: the Handbook of Psychotherapy Integration, Holm-Hadulla, the psychodynamic introductions, McWilliams, Adlerian Psychotherapy and other modality books. They are text-readable and can be added on request.

**NOT REVIEWED: IMAGE-ONLY PDFS (NO TEXT LAYER, NO OCR AVAILABLE)**

- **Psychotherapy books:** *CBT: Keith Hawton et al.* (462 pp.) and *Transference-Focused Psychotherapy for Borderline Personality Disorder* (compressed; 432 pp.).
- **Assessment manuals and forms:** Rorschach workbook, TAT (Bellak) manual and scoring sheets, MCMI-III manuals and scoring sheet, MISIC, Picture Frustration Test, BKT, PGI battery, NIMHANS SLD battery, VSMS manuals, child proforma, and one unnamed compressed file.

These could not be text-reviewed. Topic 9 mentions assessment tests only generally and does not rely on them.

**GAPS AND LIMITATIONS**

- **IACP code** was not accessible (website unreachable); any statement about it is secondary and labelled.
- **RCI 1998 ethics regulations** and the **RCI M.Phil. curriculum** were not obtained; numbers are from secondary summaries or from the 2011 diploma document.
- **DPDP Act and Rules**: primary text not obtained.
- **Original APA 2006 EBP report** and **BPS Code** were not read in full.
- **Page numbers are PDF page indices.** Where a printed number was visible it is noted in the text.
- The earlier version of these notes (PR #87) was generated differently; this edition re-verified every legal and ethics citation against the documents above. Examples and sample notes are fictional.

## 4. Topic » source » pages mapping

**Table R3 Where each syllabus topic comes from**

| Topic | Books (PDF pages) | Bodies and statutes |
|---|---|---|
| 1 Definitions | Notes 1–4, 8, 71–72; Wolberg 29–32, 125, 130, 133–134, 676; Corey 31; Sharf 159; Gabbard 58; Lister-Ford 62 | RCI Act s.13; MHCA s.2; RCI Diploma 2011; APA 2012 Resolution |
| 2 Objectives | Wolberg 31–32; Notes 3–6, 24, 72; Jongsma 24–25; Beck 31, 78; DBT Made Simple 34, 72; MET 11; Corey 78; Corsini 459 | MHCA preamble; RCI Diploma 2011; APA 10.01; WHO PM+ |
| 3 Training | Wolberg 30, 82, 106, 160–161; Corey 47–49, 56, 59–65, 78; Gabbard 192–193; Polnay 64, 77; Murdock 40; Lister-Ford 78, 114–115, 118; MET 138–140 | MHCA s.2(1)(g), 55; RCI Act ss.13, 18, 19, 23; RCI Diploma 2011; APA 2.01–2.06, 7.04–7.07, 10.01(c); Nyaaya |
| 4 Ethics/law | Notes 5–11; Corey 60, 74–78; Corsini 594–597, 602, 605; Murdock 38–40; Beck 85 | RCI Act s.21; MHCA ss.2, 4, 22–25, 115; POCSO ss.19, 21; APA principles and standards; APA RK and Telepsychology; BPS; DPDP; Nyaaya |
| 5 Rights/ responsibilities | Corey 75–76; Murdock 39; Notes 2, 5, 7, 24; Wolberg 506Q 31; Beck 31, 78 | MHCA ss.4, 18–28; RCI Act ss.13, 21; APA 3.01, 3.08, 3.10, 3.12, 4.01–4.07, 6.01–6.06, 10.01, 10.10 |
| 6 Consent/assent | Murdock 39; Corey 75–76; Notes 3, 5, 6 | APA 3.10, 4.02; MHCA ss.2, 4, 5(4), 15, 23, 87; POCSO ss.2(d), 19, 21; DPDP s.9; ICMR §6.5 |

| | | |
|---|---|---|
| 7 Planning/ recording | Beck 20, 31, 42, 82, 85, 122–123, 143, 369; Jongsma 24–26; Notes 7–8; Wolberg 106 | APA 4.03, 4.04, 4.07, 6.01–6.03; APA RK Guidelines 2007; MHCA ss.24–25; RCI Diploma p.9; Nyaaya |
| 8 Structure/goals | Beck 42, 81–84, 90, 122–123, 125, 143; Jongsma 24–26; Notes 24, 52, 77–78; Gabbard 58, 193; MET 11; DBT Made Simple 72; Corsini 166; Wolberg 506Q 104 | RCI Diploma p.8–9 |
| 9 Assessment | Beck 85, 125, 143, 350, 369–370; Lister-Ford 62, 118; Notes 5, 49; Rating Scales Cheat Sheet 78, 80 | RCI Diploma p.8; APA 9.01–9.03; Jacobson & Truax (not in repository) |
| 10 Evidence-based therapies | Corey 47, 56, 82; Lister-Ford 62, 76, 79, 116–118; Hofmann 19, 28–29; Corsini 194, 267; MET 11, 138–140; Beck 16, 369; Notes 4, 5, 60, 101–110 | APA 2006 EBPP; APA 2012 Resolution; APA 2.01, 2.04, 10.01; WHO PM+ |

## 5. How to extend or check these notes

1. Obtain the IACP code of conduct and the RCI conduct regulations (1998) and the RCI M.Phil. curriculum; add their clause numbers to Topics 3–5.
2. Check the current text and commencement of the DPDP Act and Rules for health data.
3. If you need the two image-only psychotherapy books (Hawton CBT; TFP for BPD), an OCR step is required.
4. To add more books, tell me which topics you want deepened.

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**Psychotherapy — Unit I: Introduction to Psychotherapy**

Made by Tejas Dangodra | M.Phil. Clinical Psychology

Educational study notes. Not legal or clinical advice. Indian statutes and professional codes change: confirm against the primary text or the regulator before relying on them in practice or examinations.